

Situation Analysis Update:
CHILDREN IN PAKISTAN
AUGUST, 2020





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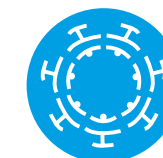
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ACRONYMS

ADP	Annual Development Plan
AJK	Azad Jammu and Kashmir
ANC	Antenatal care
ANER	Adjusted net primary enrolment ratio
ASER	Annual Status of Education Report
BISP	Benazir Income Support Programme
CC-MODA	Cross-Country Multiple Overlapping Deprivation Analysis
CRVS	Civil Registration and Vital Statistics
DFID	United Kingdom Department for International Development
DRM	Disaster risk management
DRR	Disaster risk reduction
ECE	Early childhood education
EPI	Expanded Programme on Immunization
EU	European Union
GB	Gilgit-Baltistan
GDI	Gender Development Index
GDP	Gross domestic product
GEP	Global Partnership for Education
GER	Gross enrolment rate
GII	Gender Inequality Index
GPI	Gender Parity Index
ICT	Islamabad Capital Territory
HDI	Human Development Index
IMR	Infant mortality rate
IYCF	Infant and young child feeding
KP	Khyber Pakhtunkhwa
LFS	Labour Force Survey
MD	Merged Districts of Khyber Pakhtunkhwa
MHM	Menstrual hygiene management
MICS	Multiple Indicator Cluster Survey
MMR	Maternal mortality ratio
MoCC	Ministry of Climate Change
MPI	Multidimensional Poverty Index
MPL	Minimum proficiency level

NAR	Net attendance ratio
NDC	National Determined Contribution
NER	Net enrolment ratio
NFBE	Non-formal basic education
NMR	Neonatal mortality rate
NNS	National Nutrition Survey
OOSC	Out-of-school children
O&M	Operations and Maintenance
PDHS	Pakistan Demographic and Health Survey
PKR	Pakistani Rupee
PSLM	Pakistan Social and Living Standards Measurement
SDGs	Sustainable Development Goals
SitAn Update	Situation Analysis Update of Children in Pakistan 2020
U5MR	Under-five mortality rate
UNDP	United Nations Development Programme
UNICEF	United Nations Children’s Fund
US\$	United States Dollar
WASH	Water, sanitation and hygiene
WHO	World Health Organization



EXECUTIVE SUMMARY

Of Pakistan’s estimated population of 220 million,¹ 51 percent are under 19 years old.² This means that 113 million Pakistanis are between the ages of 0 and 19, and 13 percent of them are between 0 and 4 years old. A cycle of high population growth, coupled with poor health and education outcomes, contributes to persistent socioeconomic, gender and geographic inequalities. These challenges are reflected in the global Human Development Index (HDI), where Pakistan’s ranking fell to 152nd of 189 countries in 2019.³ Its HDI value of 0.56 is below average for countries in the ‘medium’ human development group, as well as in South Asia. Nevertheless, Pakistan has made progress on a number of fronts – most notably on reducing child and maternal mortality rates, improving food security, expanding vaccination coverage, increasing the prevalence of skilled birth attendance and antenatal care, boosting gross enrolment rates (GER) for pre-primary (katchi), accelerating birth registration, enhancing access to improved drinking water, and reducing open defecation.

This Situation Analysis Update 2020 (SitAn Update) provides an essential analysis of new and updated statistics on key child-related indicators, national policies, legislation, current trends and research produced since the inception of UNICEF’s Pakistan Country Programme 2018–2022. It

1 United Nations Department of Economic and Social Affairs, World Population Prospects 2019, UNDESA, New York, 2019, <<https://population.un.org/wpp/Download/Standard/Population>>, accessed 4 March 2020.
2 Pakistan Bureau of Statistics, Labour Force Survey 2017–18, Government of Pakistan, Islamabad, 2018.
3 United Nations Development Programme, Human Development Report 2016, UNDP, New York, 2016; United Nations Development Programme, Human Development Report 2019, UNDP, New York, 2019.

aims to gauge the current situation of children, adolescents and women in Pakistan, in order to guide programme revisions and policy-making processes in the coming years. The SitAn Update also emphasizes the impact of the global COVID-19 pandemic on the realisation of child rights' outcomes for children in Pakistan. The contextual framework used for the analysis of developments in the past three years is based on the Sustainable Development Goals (SDGs) that are especially relevant for child rights. The analysis follows a rights-based, equity-focused approach, building on the 2017 Situation Analysis of Children in Pakistan. It focuses on selected child rights in line with UNICEF's key areas of work in Pakistan:

- every child survives and thrives;
- every child learns;
- every child is protected from violence and exploitation;
- every child lives in a safe and clean environment; and
- every child has an equitable chance in life.

Every child survives and thrives: Stunting and underweight have declined only moderately between 2011 and 2018 with 40.2 percent of the children still suffering from stunting (43.7 percent in 2011) and 28.9 percent are underweight (31.5 percent in 2011). This highlights the need for a greater focus on these issues, as well as for increased resource allocations. Public sector expenditure on the health and nutrition sector accounts for only 1.1 percent of Pakistan's GDP.⁴

Two out of five children under the age of 5 suffer from moderate or severe stunting, 17.7 percent suffer from wasting, almost one in three children are underweight, and 53.7



⁴ Ministry of Finance, *Pakistan Economic Survey 2019–20*, Government of Pakistan, Islamabad, 2020

percent suffer from anaemia.⁵ Nearly half of Pakistan's children are deficient in critical vitamins and minerals, more than one-third of households are food insecure, and with 18.3 percent of households are affected by severe food insecurity.⁶ Exclusive breastfeeding in the first six months of a child's life has increased to 48 percent in 2018 from 37.6 percent in 2011, while age-appropriate complementary feed alongside breastmilk for infants after 6 months of age was 35.9 percent. Nevertheless, minimum dietary diversity, minimum meal frequency and minimum acceptable diets are below acceptable levels. One in eight adolescent girls and one in five adolescent boys is underweight. 56.6 percent of adolescent girls in Pakistan are anaemic.⁷ Factors that contribute to poor nutritional indicators include inadequate systems for monitoring and evaluation; difficulties in tracking public expenditure on nutrition through financial reporting systems; and delays in the implementation of provincial multi-sectoral nutrition strategies.

Child mortality rates have declined, reflecting important improvements in child survival. Pakistan's neonatal mortality rate (NMR) fell to 42 deaths for every 1,000 live births,⁸ the infant mortality rate (IMR) to 62 per 1,000 live births, and under-five mortality rate (U5MR) to 74 per 1,000 live births. Despite this, one in every 14 Pakistani children does not survive to his or her fifth birthday. Improvements are apparent in maternal health, as the maternal mortality rate (MMR) fell to 178 maternal deaths for every 100,000 live births,⁹ and other key indicators improved – including the provision of antenatal care by skilled providers and skilled birth attendants assisting deliveries.¹⁰ Inequity – across geographic (urban/rural) location and wealth quintile – and the poor quality of care in the perinatal period contributes to high rates of still birth, neonatal deaths and maternal deaths. UNICEF is working with government counterparts and other partners to address both fronts through its current country programme.

Immunization has witnessed considerable improvement, as two-thirds of children between 12 and 23 months old have received all eight basic vaccinations.¹¹ However, Pakistan still has long way to reach the global target on immunization, and supply and demand sides need to be addressed. Despite progress on combatting polio by 2017, polio remained endemic in Pakistan in 2019–2020. It is one of only two countries in the world with new reported cases of polio virus. Quality of care has been highlighted in the 10-point agenda of Pakistan's National Vision 2016–2025, demonstrating the Government's commitment to accelerating improvement in new-born, child and maternal survival, with a focus on reducing morbidity and mortality linked to common preventable causes. Urban-rural disparities suggest a lack of available health services, limited

⁵ Nutrition Wing, Ministry of National Health Services, Regulation and Coordination, United Nations Children's Fund, Aga Khan University, and UK Aid, *National Nutrition Survey 2018*, Government of Pakistan, Islamabad, 2019.

⁶ Ibid.

⁷ Nutrition Wing, Ministry of National Health Services, Regulation and Coordination, United Nations Children's Fund, Aga Khan University, and UK Aid, *National Nutrition Survey 2018*, Government of Pakistan, Islamabad, 2019.

⁸ National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

⁹ World Bank, 'Pakistan Maternal Mortality Rate 1990–2020', World Bank, Washington D.C., <<https://www.macrotrends.net/countries/PAK/pakistan/maternal-mortality-rate>>, accessed 4 May 2020.

¹⁰ National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

¹¹ Ibid.

access to services due to difficult terrain and poor infrastructure, the poor quality of health services, and high rates of poverty in rural areas.

Every child learns: Pakistan's progress on education outcome indicators remains insufficient to support sustainable economic and social development. Public expenditure on education hovers between 2 and 3 percent of GDP (2.3 percent in 2018-19).¹² Despite steady improvements in key education indicators in recent years, the country still has 22.8 million out-of-school children (OOSC) and more girls are out of school than boys.¹³ Overall, Pakistan has the second highest number of out-of-school children in the world. A high population growth rate makes raising enrolment rates particularly challenging as providing adequate numbers of good quality facilities and teachers for the growing population of children has been major issue in the country. Additionally, inadequate quality of education has led to high dropout rates in recent years. Lack of access to middle schools, particularly for girls, has also undermined the efforts made at primary level and improving enrolments. Inequities in the education outcomes present themselves across genders, different socioeconomic groups, geographic areas, and within provinces and tend to compound with girls from poor rural families least likely to go to school. External factors, notably poverty, constraining social norms, and ethnic/ language issues also contribute to drop-out and inequities in education and learning outcomes.

There are clear disparities in the proportion of out-of-school children between provinces and socioeconomic status as well as geographical location. Within the country, the highest percentages of OOSC live in Balochistan, Khyber Pakhtunkhwa's Merged Districts and Sindh. Although Pakistan's gross enrolment ratio (GER) at the pre-primary level has increased considerably, its primary net attendance ratio (NAR) declined to 58.5 percent¹⁴ – a downward trend evident across all of the country's provinces. Secondary education fares better, with slight improvements in the NAR for middle and secondary education. Nevertheless, the gender gap in retention and enrolment continues to be a major concern with primary GPI at 0.92, middle and secondary GPI at 0.89 at the school levels and youth literacy GPI at only 0.81. Moreover, despite investments to improve the learning environment, Pakistan still lacks critical inputs. Both low school participation rates and the high number of out-of-school children are directly linked with a poor learning environments and poor learning outcomes in schools. As alternative forms of learning offer children a 'second chance' at education, the non-formal basic education (NFBE) sector can help to address the critical challenges posed by a huge number of OOSC. However, Pakistan's non-formal education sector remains largely neglected and underfunded.

12 Ministry of Finance, *Pakistan Economic Survey 2019–20*, Government of Pakistan, Islamabad, 2019.

13 National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, *Pakistan Education Statistics, 2016–17*, Government of Pakistan, Islamabad, 2018, <<http://library.aepam.edu.pk/Books/Pakistan%20Education%20Statistics%202016-17.pdf>>, accessed 4 May 2020.

14 National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

Every child is protected from violence and exploitation: Several SDG targets affirm every child's right to live free from fear, neglect, abuse and exploitation, as they address specific forms of violence and harm experienced by children. In line with these targets, birth registration of children under the age of five improved considerably – rising 42.2 percent.¹⁵ Statistics on violence remained constant over time. Most children (81 percent in both Punjab and Khyber Pakhtunkhwa) between 1 and 14 years old have experienced psychological aggression, physical punishment or violent behaviour as a form of discipline.¹⁶ The current labour force participation rate for children between the ages of 10 and 14 reflects a downward trend in recent years. In Punjab, 13.4 percent of children aged 5 to 17 are involved in child labour, down from 16 percent in 2014.¹⁷ In Khyber Pakhtunkhwa, the current rate is 14.4 percent for children in the same age group.¹⁸ Pakistan's high incidence of child marriage – determined by analysing the proportion of adolescents and young women aged 15 to 19 who are currently married – has remained at 14 percent since 2013 and over 48 percent of women aged 20 to 24 are married.¹⁹

Every child lives in a safe and clean environment: Pakistan still lacks universal and equitable access to safe drinking water, especially in rural areas where improper water treatment practices are rife. Nevertheless, progress is evident on water, sanitation and hygiene (WASH) indicators, including the greater availability, accessibility and quality of drinking water, and the growing use of improved sanitation facilities. As per the Joint Monitoring Programme for Water Supply and Sanitation (JMP) 2019 report, the vast majority (92 percent) of households now have access to improved drinking water, and over two-thirds (70 percent) to improved toilet facilities. Open defecation remains fairly widespread and practiced by 10 percent of households in Pakistan, largely practiced by poor in rural areas. WASH and climate change are inextricably linked. Disasters can destroy water supplies and toilet, or leaving behind contaminated water and putting the lives of millions of children at risk.

Every child has an equitable chance in life: Gender issues cut across all thematic areas and gender disparities are pronounced for children in Pakistan. The country ranks low both on the Gender Development Index (GDI) and the Gender Inequality Index (GII), although its GDI and GII values have improved marginally in recent years. Examples of gender inequality include the fact that more girls are out of school than boys, a low Gender Parity Index (GPI) value at the primary school level (0.92) which declines further at the middle and secondary education level (0.89).²⁰ Child marriage persists, rooted in deeply entrenched gender norms and expectations about

15 National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

16 Punjab Bureau of Statistics and United Nations Children's Fund, *Multiple Indicator Cluster Survey 2017–2018, Punjab*, Government of Punjab, Lahore, 2018; Khyber Pakhtunkhwa Bureau of Statistics, Planning & Development Department, and United Nations Children's Fund, *Khyber Pakhtunkhwa Multiple Indicator Cluster Survey 2016–17*, Government of Khyber Pakhtunkhwa, Peshawar, 2017.

17 Punjab Bureau of Statistics and United Nations Children's Fund, *Multiple Indicator Cluster Survey 2017–2018, Punjab*, Government of Punjab, Lahore, 2018.

18 Khyber Pakhtunkhwa Bureau of Statistics, Planning & Development Department, and United Nations Children's Fund, *Khyber Pakhtunkhwa Multiple Indicator Cluster Survey 2016–17*, Government of Khyber Pakhtunkhwa, Peshawar, 2017.

19 National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2012–13*, NIPS, Islamabad, 2013; National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

20 Pakistan Bureau of Statistics, *Pakistan Social & Living Standards Measurement Survey 2018–19*, Ministry of Planning Development & Special Initiatives, Government of Pakistan, Islamabad, <http://www.pbs.gov.pk/sites/default/files/pplm/publications/pplm_hies_2018_19_provincial/key_findings_report_of_pplm_hies_2018_19.pdf> accessed 22 June 2020.

girls' value and roles. Women's participation in the labour market is extremely low and is decreasing with the passage of time.

Pakistan's national poverty alleviation programme, *Ehsaas* (literally 'compassion') was launched in 2019. It aims to expand social protection, safety nets and to support human capital development across the country. With the broad mandate of bringing all public poverty alleviation programmes under one umbrella, *Ehsaas* complements and expands on ongoing social protection initiatives. In the context of COVID-19, the programme has initiated emergency cash transfers with the aim of providing relief to 12 million poor households.

Despite the proven impacts of social protection programmes, coverage of children and families remains generally low and there is insufficient data about children who have access to social protection. Moreover, while some provinces have established social protection authorities and a number of social protection initiatives is growing, these initiatives are still fragmented. In addition, determining the child-sensitivity of these structures and programmes is limited. Insufficient financing is a major barrier to comprehensive child-sensitive social protection systems in Pakistan. Building a comprehensive approach to integrated social protection systems across the life course in Pakistan is extremely essential.

Children and COVID-19 in Pakistan: The COVID-19 pandemic poses immense challenges for all countries, including Pakistan. Public health services, including continuation of essential health services has been negatively



impacted. The unprecedented crisis will wreak a heavy toll on the poorest and most vulnerable. In Pakistan, impoverished, vulnerable workers will likely see their sources of income and basic needs disrupted. The devastating long-term impacts of COVID-19 will be felt and seen more keenly among children and women in the poorest segments of society.

COVID-19 has influenced all domains of multidimensional poverty – intensifying its incidence and intensity. As a result of the pandemic, 56.6 percent of Pakistan's population has become susceptible to becoming multi-dimensionally vulnerable, with an estimated 126 million people likely to be pushed into multidimensional poverty.²¹ Pakistan's already fragile health system will struggle to maintain the delivery of essential health services, particularly vital reproductive, maternal, newborn and child health services. There may be more budget allocations on health, however, these are more focused on clinical response to COVID-19 rather than Public Health Response. Pakistan's most food insecure households, who belong to the country's lowest economic strata, will suffer the most from increased food insecurity. Interruptions in food and nutrition services will negatively impact the groups with the highest nutrition needs, especially children under-five, pregnant and lactating women, and the elderly. Nationwide school closures have affected 42 million learners in the country putting school children, especially girls, at high risk of not returning to schools when they reopen. Closures are poised to widen learning inequalities and disproportionately harm vulnerable children and youths. Restrictions on

21 United Nations Development Programme, *Multidimensional Vulnerability Index*, UNDP, Islamabad, 2020.

movement, the loss of income, isolation, and high levels of stress are increasing the likelihood that children will experience physical, psychological and/or sexual abuse at home. An increase in gender-based violence, sexual exploitation and abuse are also expected based on experiences of past epidemics. Economic distress is likely to force more children into child labour and child marriage. In tandem, the disruption of water, sanitation and hygiene interventions and services are poised to undermine infection prevention and control efforts, while feeding into a growing risk of other diseases.

There are indications of strong political will to move away from a 'business as usual' approach and to invest in solutions for marginalized groups. However, with allocations diverted to fight COVID-19's impact, little is left for development budgets. This will push back provincial growth trajectories and may, if unchecked, derail progress considerably. To stop this happening, and cement the well-being of children across Pakistan, this SitAn Update proposes a number of recommendations based on its findings.

OVERALL RECOMMENDATIONS:

- Ensure policy support for higher, sustained and inclusive growth, balanced regional development, and increased access to financial resources to support the livelihoods of Pakistan's most vulnerable populations.
- Support the Government to scale up gender and child-sensitive social protection measures, strengthening integrated social protection systems, and to assessing the impact of social protection support during the pandemic crisis in order to inform adjustments in future interventions.
- Institutionalize the routine measurement of child poverty as distinct from general household poverty.
- Recognize children as a unit of allocation and expenditure with adequate differentiation of the needs of younger children and adolescents in order to incorporate them into national and provincial growth strategies, annual budgeting and planning across different sectors. This is vital for securing sustained improvements on key child development indicators, while reducing multidimensional child poverty and deprivation. To achieve this, it will be important to measure vulnerability and poverty taking the child as a unit of analysis and planning.
- Strengthen partnerships with civil society and the private sector to deliver interventions for children in emergencies, such as COVID-19.

RECOMMENDATIONS TO ENSURE THAT EVERY CHILD SURVIVES AND THRIVES:

- Support government priorities, outlined in the National Action Plan for Preparedness and Response to Coronavirus Disease (NAP), to contain and respond to the outbreak in a timely and efficient manner. Prioritize financial resources for emergency preparedness and implement emergency preparedness actions by strengthening inter-sectoral collaboration

with government sectors, the private sector and civil society at the provincial level.

- Conduct a thorough mapping of those most at risk of being left behind due to COVID-19. Formulate mechanisms during and after the pandemic to incorporate provisions that meet the needs of those at most risk.
- Prioritize newborn health and stillbirth reduction through multiple approaches – addressing health system issues, HR, quality of services and community based newborn programme, collaborate with other agencies for service utilization.
- Adopt a systems approach to nutrition that makes multiple systems accountable for nutrition results beyond sectoral objectives – including the food, health, WASH, education and social protection systems.
- Provide a comprehensive package of maternal and newborn care and develop an investment plan and contribute for universal health care (UHC) implementation within the framework of primary health care acceleration. Strengthen interaction between public and private healthcare sector.
- Adopt public health measures with resilient health system approach for the continuation of essential health services, and mainstreaming COVID 19 responses within UNICEF's current country programme. The public health response should focus on Risk Communication and Community Engagement (RCCE); infection prevention and control in health facilities and communities; the protection of health workers; and maintaining essential services by strengthening primary health care services and supporting the provision of supplies.
- Deliver an integrated package of services using a system strengthening approach for primary health care.
- Ensure the continuation of essential health services including routine immunisation and the treatment of severe acute malnutrition, alongside advocacy and the dissemination of information on infant and young child feeding (IYCF) practices through the print and electronic media. Implement effective communication strategies to engage communities, ensure supply and demand for vaccination as an essential means of saving lives. Emphasize the broader effects of the pandemic on child and maternal health, in addition to the direct impacts of COVID-19.

RECOMMENDATIONS FOR EDUCATION CONTINUITY AND LEARNING:

- Develop strategies to address weaknesses observed during the pandemic, such as in terms of the education delivery chain's capacity from top to bottom.
- Support government education efforts in terms of response coordination, efficient delivery, and real-time monitoring to provide up-to-date information on children in order to better understand the scale of current problems, beyond aggregate numbers.
- Support provincial education departments to explore a mix of no tech/offline, low tech and high tech/online modalities across the technology spectrum to reach maximum number of

children as well as respond to learning needs and of children at different levels. Create and develop content for different platforms and modalities according to levels, subjects and grades.

- Expand demand side education interventions, including additional efforts to bring girls back to school through targeted messages on participation and by using incentives.
- Support the non-formal basic education by promulgating policies that enables more children to access education so as to create a level of parity between formal and non-formal education.

RECOMMENDATIONS TO ENSURE THAT EVERY CHILD IS PROTECTED FROM VIOLENCE AND EXPLOITATION:

- Support the Government to strengthen its Civil Registration and Vital Statistics (CRVS) capacities and continued civil registration as an essential service. Collaborate with health sector stakeholders for notifications of births and deaths through hospital/ health facilities and other means.
- Support government efforts to ensure that COVID-19 prevention and response plans integrate age-appropriate, gender-sensitive measures to protect all children from violence, neglect, abuse and exploitation. Ensure that all children and women have sustained access to social services including mental health and psychosocial support.
- Advocate with the government to recognize the social service workforce as an essential service.
- Prioritize the development of a child-sensitive social protection policy framework and its use at national and in provincial levels including strengthening social service workforce. Particularly, support government capacity for planning and delivery of shock responsive social protection.
- Advocate with the government to recognize the social service workforce as an essential service.

COMMUNICATE WITH, AND ENGAGE, COMMUNITIES, PARENTS, CAREGIVERS AND CHILDREN/ADOLESCENTS THEMSELVES WITH EVIDENCE-BASED INFORMATION AND ADVICE ON CHILD PROTECTION AND PREVENTING VIOLENCE AND ABUSE AND EXPLOITATION. DEVELOP AND LAUNCH A NATIONWIDE CAMPAIGN THAT RAISES AWARENESS ABOUT THE NEGATIVE EFFECTS OF THE PANDEMIC ON WOMEN AND CHILDREN. RECOMMENDATIONS TO ENSURE THAT EVERY CHILD LIVES IN A SAFE AND CLEAN ENVIRONMENT:

- Safe WASH services in health care facilities (HCFs) to deliver quality health services; protect patients, health workers, and staff; and to prevent further transmission.
- Support government and competent authorities to ensure the continuity and quality of water, and sanitation services – which will be highly affected by reduced workforce, disrupted supply chains, and payment challenges – through close collaboration with national and local WASH authorities.
- Hygiene promotion that focuses on behavioural change is important as well as challenging in the context of COVID as it requires community interaction.
- Scale up the WASH intervention in schools and hand washing initiatives for safe re-opening of schools after lockdown is lifted.
- Integrate climate resilience and WASH as integral components of future interventions.
- Provision of capacity development of decentralized tiers.
- Strengthen partnerships with civil society and private sector in the delivery of interventions for children including during emergencies such as COVID-19.





CHAPTER 1: INTRODUCTION

INTRODUCTION



Where do Pakistan's children stand today? How far have we come since 2017, when UNICEF published its last *Situation Analysis of Children in Pakistan*, to realize the rights of children in Pakistan? This *Situation Analysis Update 2020* (SitAn Update) aims to answer these questions. It analyses key developments between 2017 and 2020 and assesses progress on children rights, child related indicators, and programming environment since the development of the Situation Analysis in 2017. It also highlights gaps and pinpoint disparities including initial impact of Covid-19 and associated containment measures including lock-downs. Based on this analysis, it offers recommendations to ensure that every child in Pakistan survives and thrives, every child learns, every child is protected from violence and exploitation, and every child lives in a safe and clean environment. The evolving COVID-19 pandemic makes this update especially timely and important. To address the current crisis, this report includes an analysis of COVID-19's impact on specific sectors, which enhances the value and significance of the findings presented under each thematic area.

The ongoing UNICEF *Pakistan Country Programme 2018–2022* was informed by the comprehensive *Situation Analysis of Children in Pakistan* in 2017. UNICEF's Country Office in Pakistan is conducting a Mid-Term Review (MTR) to review, revise and refine the current Country Programme, in order to suggest course corrections to improve its alignment with the priorities of the Government of Pakistan and

their national development framework, Vision 2025, as well as UNICEF's Strategic Plan 2018–2021 and the joint United Nations Sustainable Development Framework (UNSDF) for Pakistan. This SitAn Update provides an analysis of new and updated statistics on key child-related indicators, national policies, legislation, current trends and research generated since the inception of UNICEF's current Country Programme in 2018. It aims to gauge the current situation of children, adolescents and women in Pakistan, in order to guide programme revisions and policy-making processes in the remaining years of the current country programme.

CONTEXTUAL FRAMEWORK

The focus of this SitAn Update is to build on new and emerging information to assess progress made on achieving outcomes, take stocking of key developments and changes in the situation of children, adolescents and women in Pakistan. It also adds a new dimension in view of the current COVID-19 pandemic and its impacts. The United Nations framework for immediate socioeconomic response to COVID-19 was used to assess impacts and response across this report's thematic areas.²² However, since the outbreak of the epidemic is still evolving and numbers are still increasing, the current report thus cannot provide a conclusive analysis around the impact of COVID19 on various sectors. Nevertheless, it does attempt to provide an indicative analysis for each sector and respective child rights. A limitation in this SitAn Update is the analysis of private sector and relevance under thematic areas covered in this report.

The SitAn Update follows a rights-based, equity-focused approach, building on the 2017 Situation Analysis around selected child rights. It undertakes a disaggregated assessment of the status of children's and women's rights, and highlights trends towards their realization. In tandem, it analyses the underlying causes of shortfalls and disparities evident between children of different age groups, between girls and boys, between children in different parts of Pakistan, between urban and rural areas, and between different wealth quintiles. This updated situation analysis of children and women is based on a combination of new statistics, national policies, laws and trends, and new research and analysis undertaken since UNICEF's last Situation Analysis in 2017.

The SitAn Update adopts the following contextual framework for analysing developments over the past three years, and for presenting key findings:

- The guiding principle of this SitAn Update is to conduct an analysis around the Sustainable Development Goals (SDGs) which are especially relevant for child rights: SDG 1 ('no poverty'), SDG 2 ('zero hunger'), SDG 3 ('good health and well-being'), SDG 4 ('quality education'), SDG 5 ('gender equality'), SDG 6 ('clean water and sanitation'), SDG 8 ('economic growth and decent work'), the cross-cutting SDG 10 ('reduced inequalities'), and SDG 16 ('peace, justice and strong institutions').

²² The UN framework for an immediate socioeconomic response to COVID-19 of April 2020 prioritizes health, social protection and basic services, safeguarding food supply chains, protecting jobs and livelihoods, and addressing debt in developing countries. See: <<https://unsdg.un.org/resources/un-framework-immediate-socio-economic-response-covid-19>>

- The analysis is structured around four key thematic areas: health and nutrition, education, child protection, and water and sanitation. It also integrates a focus on cross-cutting topics in the context of recent developments in Pakistan, including gender, inequality, climate change, social protection and the impacts of the COVID-19 outbreak. As such, this SitAn update makes the most disadvantaged and vulnerable children more visible for the purposes of decision-making, policy-making, legislation, budgeting and national research. An analysis of early childhood development (ECD) and adolescents is also covered across the four thematic areas.
- The SitAn Update entails an institutional, legislative and contextual analysis, including analysis of community systems for demand, accountability and feedback that captures developments between 2017 and 2020, including changes in government and development partners' financing and priorities across the report's thematic and cross-cutting areas.
- This report highlights variability and trends where considerable changes have occurred across Pakistan and its provinces, supported by analytical rigour and factual evidence.

METHODOLOGY

The situation analysis update consisted of a desk-based review of data currently available that fit the purpose of the analysis. Primary research was not conducted; however, the analysis was based on a mix of quantitative and qualitative evidence. The consultation part of the research process was limited due to the extended lockdown in Pakistan. As such, virtual meetings were set up with UNICEF programme teams and their feedback was incorporated to further strengthen the analysis.²³

Data sources: The data sources used to inform this SitAn Update include the *Pakistan Demographic Health Survey* (PDHS 2012-13 and 2017-18); the latest *Multiple Indicator Cluster Surveys* (MICS) for Punjab (2017-18), Khyber Pakhtunkhwa (2016-17) and Gilgit-Baltistan (2016-17); the *Pakistan Education Statistics* (PES 2014-15, 2015-16 and 2016-17); the *Pakistan Social and Living Standards Measurement Survey* (PSLM 2013-14 and key findings of 2018-19); the *National Nutrition Survey* (NNS 2011 and 2018); the *Pakistan Labour Force Survey* (LFS 2014-15 and 2017-18); and the *National Census 2017*. Regional and global evidence was also used for the purpose of comparison, as were national and provincial government strategies, development and sectoral plans and other documents, and new research on COVID-19 that had emerged when the draft report was submitted. A review of UNICEF's Country Programme document and Programme Strategy Notes on Health, Nutrition, Education, WASH, Child Protection, and Polio Eradication was also undertaken as part of the secondary research.

Structure of the report: The report is structured in six chapters. The main body of the report (Chapter 3) offers a comprehensive review and analysis around four themes included in UNICEF's

²³ Meetings will be set up after the draft report has been reviewed by Chiefs of Sections.

Country Programme and the 2017 Situation Analysis:

- every child survives and thrives²⁴;
- every child learns;
- every child is protected from violence and exploitation; and
- every child lives in a safe and clean environment.

Separate short chapters are included on: cross-cutting themes, such as gender, inequality, climate change and social protection acknowledging the right of every child of an equitable chance in life; a forward looking development partners and institutions mapping exercise to inform potential engagements by UNICEF; and an analysis of the impact of COVID-19. The analysis culminates in a final chapter (Chapter 6) on the SitAn Update's key findings, conclusions and recommendations for UNICEF for the next two and a half years. National and provincial infographics are included as annexes to the final report.

²⁴ The analysis also includes maternal health indicators.



CHAPTER 2: KEY CONTEXTUAL DEVELOPMENTS IN PAKISTAN (2017–2020)

KEY CONTEXTUAL DEVELOPMENTS IN PAKISTAN (2017–2020)

POPULATION DYNAMICS

Pakistan has an estimated population of 220 million,²⁵ having increased from 208 million reported in Pakistan's National Census in 2017. Over 51 percent²⁶ of the population is under 19 years of age, translating into 113 million people. The proportion of the population in this age group has declined from 52.74 percent²⁷ in 2015. Over 13 percent of the total population belongs to the 0–4 age group (early childhood), and 23 percent to the 10–19 age group (adolescents). Despite a slight decline, Pakistan's population growth rate²⁸ – estimated at 2.04 percent in 2017 – remains high. This poses major challenges to many sectors, including education, where the public sector struggles to increase facilities and services to keep pace with the growing population.

Table 1: Pakistan's key demographic indicators

DEMOGRAPHIC INDICATORS*		2017	2019	2022
Total population (millions)		207.77	211.17	229.49
Urban population (millions)		75.58	76.82	79.92
Rural population (millions)		132.19	134.35	136.63
Total fertility rate		3.60	3.30	3.24
Crude birth rate (per thousand)		28.60	–	26.00
Crude death rate (per thousand)		6.99	–	–
Population growth rate (percentage)		2.04	1.90	1.85

* All figures for 2019–2022 are projected figures.

Source: Government of Pakistan, Pakistan Economic Survey 2019–20, National Census 2017, Pakistan Demographic and Health Survey (PDHS) 2017–18, and United Nations Population Division, World Population Prospects: 2019 Revision.

²⁵ United Nations Department of Economic and Social Affairs, World Population Prospects 2019, UNDESA, New York, 2019, <<https://population.un.org/wpp/Download/Standard/Population>>, accessed 4 March 2020.

²⁶ Pakistan Bureau of Statistics, *Labour Force Survey 2017–18*, Government of Pakistan, Islamabad, 2018.

²⁷ Pakistan Bureau of Statistics, *Labour Force Survey 2014–15*, Government of Pakistan, Islamabad, 2015.

²⁸ Inter-censal growth rate



Pakistan continues to have very poor human development outcome indicators, especially in terms of education and health, compared with other lower middle-income countries and its neighbours in South Asia. The cycle of high population growth, coupled with poor health and education outcomes, contributes to persistent socioeconomic, gender and geographic inequalities. Pakistan's ranking on the Human Development Index (HDI) fell from 147th of 188 countries in 2016, to 152nd of 189 countries in 2019.²⁹ Its HDI value of 0.560 (compared to 0.550 in 2016) is below the average for countries in the medium human development group (0.634) and below the average for South Asia (0.642).³⁰ Pakistan is also among the lowest ranked countries on the Human Capital Index (HCI) – ranking 134 of 157 countries³¹ – which gauges the productivity of the next generation of workers, compared to benchmarks in education and health.

Pakistan has the highest rate of urbanization in South Asia.³² According to the 2017 Population Census, 36.4 percent of the population lives in urban areas compared to 32.5 percent in 1998. The UN Population Division estimates that nearly half of Pakistan's population will live in cities by 2025. One of the main drivers of Pakistan's urban growth is migration from rural areas. Migrations from rural to urban areas have largely been due to better jobs and improved access to basic services in the cities, which has inflated Pakistan's biggest cities rapidly.³³ The immense size of the urban population has important implications for public service delivery, particularly for children. In the absence of sufficient urban planning and physical and social infrastructure, this represents a tremendous challenge for employment, social indicators and environmental sustainability – all of which will have a bearing on child rights, particularly residing in inhabiting peri-urban, marginal and at-risk areas. It also worsens their vulnerability to the impacts of disasters, placing more lives and livelihoods at risk.

ECONOMIC OUTLOOK AND COVID-19

In 2020, the COVID-19 outbreak prompted a serious health emergency and an especially intense economic emergency in Pakistan. The International Monetary Fund (IMF) predicts that Pakistan's GDP will contract by -1.5 percent in 2020, compared to a growth of over 5 percent in 2017 and 2018, and of 3.2 percent in 2019. Earlier estimates suggested that the economy would contract by up to -4.6 percent, or at least by -1.3 percent, indicating that Pakistan's economy could fall into recession.³⁴ IMF latest forecasts for Pakistan are to restore GDP growth by 1 percent in 2021 compared to 1.9 percent projected earlier, driven by weaknesses in the second half of 2020.³⁵ As the economy enters a downward spiral, the socioeconomic impact of COVID-19 will be felt most keenly by the world's most vulnerable people – especially women and children.

²⁹ United Nations Development Programme, *Human Development Report 2016*, UNDP, New York, 2016; United Nations Development Programme, *Human Development Report 2019*, UNDP, New York, 2019.

³⁰ United Nations Development Programme, *Human Development Report 2019*, UNDP, New York, 2019.

³¹ Ibid.

³² UNDP Pakistan <https://www.pk.undp.org/content/pakistan/en/home/library/development_policy/dap-vol5-iss4-sustainable-urbanization.html>

³³ <https://www.theigc.org/blog/the-six-biggest-challenges-facing-pakistans-urban-future/>

³⁴ World Bank, *South Asia Economic Focus, Spring 2020: The Cursed Blessing of Public Banks*, World Bank, Washington D.C., 2020, <<https://openknowledge.worldbank.org/handle/10986/33478>>, accessed 4 May 2020.

³⁵ Regional Economic Outlook July 2020 Update. Middle east and Central Asia, IMF, <<https://www.imf.org/en/Publications/REO/MECA/Issues/2020/07/13/regional-economic-outlook-update-menap-cca#report>>

These projections compound ongoing challenges, including growing and predominant informal sector, women's low labour force participation, and high levels of youth unemployment and disenfranchisement. The layoffs as a result of economic recession will have severe consequences for Pakistan's poorest households. Adolescents and young people entering the job market will be the most vulnerable. According to recent estimates by the National Command and Operation Centre (NOCC),³⁶ 18 million people in Pakistan could lose their jobs.³⁷ The number of confirmed COVID-19 cases in Pakistan rose to over 290,000 as of 19 Aug 2020, with the highest number of cases in the province of Sindh followed by Punjab.³⁸ The overarching challenges that the pandemic poses for the Government are threefold: (i) there is a need to implement measures to save lives by containing the spread of COVID-19, particularly interventions to protect the poorest and most vulnerable from negative shocks caused by its spread and resultant containment policy actions; (ii) there is a need to put in place immediate measures that support early, rapid economic recovery,³⁹ and safeguarding essential services for women and children.

The stimulus package of PKR 1.2 trillion presented by the Federal Government focuses on the economic survival of the most vulnerable members of society. The package's emergency cash initiative is the largest ever social protection initiative in the country's history. Provincial governments have also come forward, with Sindh's COVID-19 Emergency Relief Ordinance 2020 and Punjab's Post-COVID-19 Public Investment Strategy (the Responsive Investment in Social Protection and Economic Stimulus (RISE) Framework).

MULTIDIMENSIONAL POVERTY, CHILD POVERTY AND VULNERABILITY

The first goal of the Agenda 2030 for sustainable development (SDG 1) calls for an end to poverty in all its forms, everywhere. Pakistan's national baseline for poverty indicates that 29.5 percent of the population lived below the national poverty line in 2015 (US\$ 1.90 per day), while its target is to reduce this figure to 9 percent by 2030. With regard to SDG target 1.2.2 – on the proportion of the population living in poverty in all its dimensions, according to national definitions – multidimensional poverty in Pakistan stood at 38.8 percent in 2015 (baseline), with a target set to reduce this proportion to 19 percent by 2030. On SDG target 1.3.1 – on the proportion of the population covered by social protection systems – Pakistan's baseline in 2015 was 29.9 percent, against an ambitious target of ensuring that 70 percent of Pakistanis⁴⁰ will be covered by social protection programmes by 2030⁴¹ (see Annex).

³⁶ The National Core Committee (NCC), chaired by Prime Minister and with representatives from all of Pakistan's provinces, is the Government's lead agency to address the COVID-19 pandemic. The National Command and Operation Centre (NCOC) has been set up to act as the implementation arm of the NCC.

³⁷ Pakistan Institute of Development Economics, *COVID-19 Bulletin*, No. 13, Islamabad, PIDE, 2020, <<https://www.pide.org.pk/pdf/PIDE-COVID-Bulletin-13.pdf>>, accessed 4 May 2020; The Express Tribune, 'Lockdown may render 18 million jobless in Pakistan,' 3 May 2020, <<https://tribune.com.pk/story/2213164/1-pakistans-virus-fatality-rate-much-lower-countries-asad-umar>>, accessed 4 May 2020.

³⁸ See: <<http://covid.gov.pk>>

³⁹ Sub-committee of the National Coordination Committee for COVID-19 on Economic Analysis, *Preliminary Macroeconomic and Socioeconomic Assessments*, April 2020, Government of Pakistan, Islamabad, 2020.

⁴⁰ Data on this indicator needs to be disaggregated by sex, while distinguishing children, unemployed persons, older persons, persons with disabilities, pregnant women, newborns, victims of work injuries, the poor, and the vulnerable.

⁴¹ Planning Commission, Ministry of Planning, Development & Reform, *Summary for the National Economic Council (NEC): Sustainable Development Goals (SDGs) National Framework*, March 2018, Government of Pakistan, Islamabad, 2018.

The Government of Pakistan has used different techniques to measure poverty, including 'food and energy intake', the 'cost of basic needs', and most recently the Multidimensional Poverty Index (MPI) in 2015 which focuses on three dimensions: health, education and living standards (see the 2017 Situation Analysis Report for a detailed discussion of the MPI).⁴² As COVID-19 stands to influence all of the dimensions of multidimensional poverty in different ways, data from the 2015 MPI is no longer sufficient for identifying vulnerable segments of the population. Here, the analysis is informed by recent estimates⁴³ of how the vulnerability of Pakistan's population will be affected by the pandemic. For the first time in Pakistan, UNDP has formulated a COVID-19 Responsive Multidimensional Vulnerability Index (CRMVI) based on dimensions and indicators related to education, health, living standards, employment, land and livestock, and modes of communication. In light of Pakistan's overall MPI value of 0.197 in 2015, the socio-economic vulnerability of poor households is projected to increase due to health emergencies such as the COVID-19 pandemic, resulting in a CRMVI value of 0.264. This means that 56.6 percent of the population – roughly 126 million people – may fall into the category of 'vulnerable' persons. The greatest contributors to vulnerability are: having less than five years of schooling, earning less than the minimum wage, dependency ratios, the ownership of livestock, and overcrowding. An estimated 92 million people in rural areas and 32 million people in urban areas are susceptible to becoming multi-dimensionally vulnerable as a result of COVID-19.⁴⁴ Sindh and Punjab are projected to experience the highest increase in multidimensional vulnerability given their high population density. Balochistan and Khyber Pakhtunkhwa may experience a lower increase in the vulnerable population, despite their higher MPI and CRMVI values. Similarly, stark projections are offered by the World Bank, which predicts that the pandemic will cause Pakistan's national poverty ratio of 31.3 percent, as recorded in June 2018 (69 million people), to jump sharply to over 40 percent by June 2020 (87 million).⁴⁵

The routine measurement of child poverty is a requirement of SDG reporting, and a foundation for reducing poverty among children. It is imperative that child poverty is measured as a distinct phenomenon, rather than merely within the scope of general household poverty, given its far-reaching, intergenerational impact.

A recent study⁴⁶ provides us with the MPI, Child-disaggregated MPI and Child MPI for Punjab. With 26 percent of Punjab's population multidimensionally poor in 2017-18, the MPI value for Punjab is 0.124. The MPI values ranged from 0.055 in Gujranwala division and 0.056 in Rawalpindi to 0.209

⁴² United Nations Development Programme, Oxford Poverty and Human Development Initiative and Ministry of Planning, Development & Reform, *Multidimensional Poverty in Pakistan*, UNDP, Islamabad, 2015.

⁴³ United Nations Development Programme, *Multidimensional Vulnerability Index*, UNDP, Islamabad, 2020.

⁴⁴ *ibid.*

⁴⁵ World Bank, *Poverty & Equity Data Portal*: Pakistan, World Bank, Washington D.C., 2017, <<http://povertydata.worldbank.org/poverty/country/PAK>>, accessed 4 May 2020.

⁴⁶ UNICEF and Oxford Poverty and Human Development Initiative (OPHI)-University of Oxford, *Study of Multidimensional Poverty and Children in Punjab*, Pakistan, May 2020.

in Bahawalpur and 0.279 in DG Khan. The Child MPI is constructed with dimensions: education (years of schooling, child school attendance), health (immunization, ante-natal care, assisted delivery), living standard (water, sanitation, walls, overcrowding, electricity, cooking fuel, assets, land and livestock, child nutrition and labour), and child conditions (cognitive development). The age disaggregation is performed also for the new 2017-2018 Punjab MPI, which shows that children are disproportionately heavily and more frequently affected by multidimensional poverty in Punjab. Age disaggregation shows that multidimensional poverty afflicts just over one in five adults (21.5 percent) but it's one in three children (32.2 percent). The Child MPI, which is the product of the incidence and the intensity, has a value of 0.220 for Punjab and the incidence of multidimensional poverty for children is 50.7 percent with intensity of poverty at 43.5 percent. The results reveal that the incidence of rural poverty is practically twice larger than the incidence in urban areas. In addition, the Child MPI is far larger in rural areas than in urban areas. Regional disparities exist with Child MPI highest in Rajanpur (0.478) and DG Khan (0.410) and lowest in Gujrat (0.098) and Lahore (0.099). Incidence of multidimensional poverty is 62.4 percent among children of 0-5 years of age compared to 35.2 percent for children of 6-17 years age, and Child MPI of 0.281 and 0.140, respectively for these age cohorts.



Table 2: Child MPI, Incidence and Intensity of Poverty in Punjab⁴⁷

		CHILD MPI	INCIDENCE (%)	INTENSITY (%)
Punjab		0.220	50.7	43.5
Rural		0.274	61.2	44.8
Urban		0.119	31.0	38.5
Boys		0.219	50.9	43.1
Girls		0.221	50.5	43.9
Age 0-5		0.281	62.4	45.1
Age 0-17		0.140	35.2	39.7

⁴⁷ Incidence – the percentage of people who are poor, or the poverty rate or headcount ratio & intensity – the average percentage of dimensions in which poor people are deprived, or the average deprivation score of poor persons.

Unfortunately, poverty, inequity, injustice and the devastating long-term impacts of COVID-19 will be felt most among children and women from the poorest segments of society in low and middle-income countries. The pandemic may exacerbate existing forms of poverty, cause new forms of poverty, and render more people becoming poor.

To achieve SDG 1, significant policy support is required, including higher, sustained and inclusive growth; social protection for at least 70 percent of the population living below the poverty line; balanced development across Pakistan's provinces and regions; and increased access to credit to support livelihoods.

GOVERNANCE AND PUBLIC EXPENDITURE

Since the 18th Amendment to the Constitution (2010) devolved administrative responsibility for 17 key social subjects to Pakistan's provinces, provincial authorities have developed as administrative units – replete with new departments, laws and oversight frameworks. These subjects include education and special education, health, labour and manpower, local governance, rural development, women's development, youth development, population welfare, and social welfare. The establishment of local governments within the provinces has been the cornerstone of the devolution process. While all four provinces have passed legislative acts,⁴⁸ local governments are not fully functional. This is problematic, as local governments have a significant role to play in providing services at the grassroots level. Moreover, Pakistan's provinces remain ill-equipped to handle administrative burdens, especially in terms of education and health services, resulting in a governance gap. By removing these subjects from the federal umbrella, and financing provinces without sufficient policy frameworks, checks and accountability, the devolution process may be regarded as weakening the federation without necessarily strengthening the provinces.

Another important development to highlight is the 25th Constitutional Amendment (2018), which sanctioned the merger of the Federally Administered Tribal Areas (FATA) into the neighbouring province of Khyber Pakhtunkhwa in 2019. The Government considers the merger one of the most significant political reforms in Pakistan's history, paving the way for an unprecedented extension of constitutional rights and governance structures to 5 million of the poorest people in Pakistan.⁴⁹ While local government elections in the region were planned for 2019, these were postponed indefinitely. At the time of writing in May 2020, local government elections in the Merged Districts had yet to be held.

Pakistan was one of the first countries to commit to implementing the 2030 Agenda for Sustainable Development and its 17 SDGs in October 2015. In 2018, the National Economic Council approved Pakistan's National SDG Framework. Effective implementation of the 2030 Agenda in Pakistan

⁴⁸ The Punjab Local Government Act (PLGA) 2019 (repealing the previous 2013 Act), the Punjab Village Panchayats and Neighbourhood Councils Act 2019, the Sindh Local Government (Third Amendment) Act 2016, the Khyber Pakhtunkhwa Local Government (Amendment) Act 2019, and the Balochistan Local Government (Amendment) Act 2019.

⁴⁹ United Nations Development Programme, FATA Governance Project, UNDP, Islamabad, 2020, <<https://www.pk.undp.org/content/pakistan/en/home/projects/fata-governance-project.html>>, accessed 4 May 2020.

hinges on the effectiveness of the local government system in embedding the SDGs at the grassroots level. Achieving the goals ultimately depends on the ability of local and provincial governments to promote integrated, inclusive and sustainable development.⁵⁰

Public expenditure:⁵¹ To determine public expenditure on children in Pakistan, this report analyses spending on education, health and WASH, given that Pakistan’s current budgetary system does not recognize children as a separate ‘unit of allocation’ or ‘unit of expenditure’. Public expenditure (current and development expenditure) on education was estimated at 2.3 percent of GDP in 2019 – totalling PKR 868 billion. It has remained at similar levels in recent years, rising only slightly from 2.2 percent in 2016–17. Development expenditure represents only 10 percent of total expenditure on education, remaining being current expenditure that primarily funds salaries. This scenario contrasts the huge deficit in availability of capital educational infrastructure such as schools and classrooms in most provinces. Public expenditure on health and nutrition is extremely low – representing 1.1 percent of GDP in 2019. Of Pakistan’s total health expenditure of PKR 421 billion, only 13.7 percent was allocated to development schemes in the sector, while the rest was given over to current expenditure. Overall public sector expenditure on water, sanitation and hygiene (WASH) in 2018-19 totalled PKR 62 billion, against Pakistan’s calculated annual investment needs of PKR 450 billion for the WASH sector.

Table 3: Pakistan Public Expenditure

EXPENDITURE	CURRENT (RS. BILLION)		DEVELOPMENT (RS. BILLION)		TOTAL (RS. BILLION)		% OF GDP	
	2016-17	2018-19	2016-17	2018-19	2016-17	2018-19	2016-17	2018-19
Education	597	778	102	90	699	868	2.2	2.3
Health & Nutrition	230	363	99	58	329	421	1.0	1.1
WASH	24	22	48	40	72	62	0.22	-

Source: Ministry of Finance, Pakistan Economic Survey 2018–19, Government of Pakistan, Islamabad, 2019. Pakistan WASH Budget Analysis (2017–2018, 2018–2019 and 2019–2020). UNICEF. March 2020 Joint Sector Review, Drinking Water, Sanitation and Hygiene (WASH), Pakistan. January 2019 Pakistan WASH budget Analysis (2017–2018, 2018–2019 and 2019–2020)

50 Government of Pakistan, *Pakistan’s Implementation of the 2030 Agenda for Sustainable Development: Voluntary National Review 2019*, Government of Pakistan, Islamabad, 2019.

51 Ministry of Finance, *Pakistan Economic Survey 2018–19 & 2019–20*, Government of Pakistan.



The Government has also made efforts in the past to re-prioritize Public Sector Development Programme (PSDP) allocations to focus more on pro-poor projects, and on projects in less developed areas. Currently, pro-poor expenditures stand totals PKR 3,099 billion, an increase from PKR 2,694.6 billion in 2015–16. Pro-poor expenditure accounts for 8 percent of GDP and has declined from 9.5 percent in 2016-17. In this context, the highest spending has been on health, education and infrastructure.

Although, the exact expenditure is on dealing with the impacts of COVID-19 cannot be articulated, development budgets have been under pressure as public sector financial allocations are diverted towards addressing health and economic impacts of the pandemic. This may derail provincial trajectories, highlighting the critical importance of addressing the high risk of decreasing budgets for the most vulnerable, including children. Every child survives and thrives

52 The Government has prioritized 17 pro-poor sectors through its *Medium-Term Expenditure Framework (MTEF)* in the *Second Pakistan Poverty Reduction Strategy Paper (PRSP II)*. These include the environment, water and sanitation, education, health, population planning, social security and welfare, natural disasters, agriculture and rural development, among others.





**CHAPTER 3:
RIGHTS-BASED,
EQUITY-FOCUSED THEMATIC
ANALYSES**

RIGHTS-BASED, EQUITY-FOCUSED THEMATIC ANALYSES



EVERY CHILD SURVIVES AND THRIVES NUTRITIONAL STATUS OF CHILDREN IN PAKISTAN

Malnutrition is one of the most serious public health concerns in Pakistan. It not only takes a heavy toll on the health of the population – especially of infants, children and women of reproductive age in the form of contribution to high mortality and morbidity rates – but also poses a major developmental challenge. High levels of malnutrition costs Pakistan US\$7.6 billion or 3 percent of GDP every year.⁵³ They threaten the achievement of the SDGs, particularly SDG 2 (‘zero hunger’) and SDG 3 (‘good health and well-being’). Worldwide, poor nutrition is associated with almost half of all child deaths,⁵⁴ causing an estimated 45 percent of the deaths in children under five (approximately 3.1 million child deaths each year).⁵⁵ As stunting, wasting and children being overweight are key indicators for quantifying the prevalence of malnutrition, SDG target 2.2 focuses on these indicators to improve nutrition and end all forms of malnutrition. This includes achieving internationally agreed targets on stunting, wasting and overweight in children under-five, as well as addressing the nutritional needs of adolescent girls, and pregnant and lactating women by 2025.⁵⁶

⁵³ World Food Programme, ‘Malnutrition Costs Pakistan US\$7.6 Billion Annually, New Study Reveals’, WFP, Islamabad, 28 February 2017, <<https://www.wfp.org/news/malnutrition-costs-pakistan-us76-billion-annually-new-study-reveals>>, accessed 4 May 2020.

⁵⁴ World Health Organization, ‘Children: reducing mortality’, WHO, Geneva, 2019, <<https://www.who.int/news-room/fact-sheets/detail/children-reducing-mortality>>, accessed 4 May 2020.

⁵⁵ Black, Robert E., et al., ‘Maternal and Child Undernutrition and Overweight in Low-and Middle-Income Countries: Prevalence and Consequences’, *The Lancet Launch Symposium*, London, 6 June 2013.

⁵⁶ United Nations, *Sustainable Development Goals: Goal 2 Zero Hunger*, UN, New York, 2015, <<https://www.un.org/>

It is important to note that factors associated with stunting in children under the age of five include levels of maternal education, as well as wealth quintiles. Significant predictors of stunting in Pakistan include poor maternal nutrition (height<145 cm, BMI<18.5), no maternal education, adolescent marriage, unsafe drinking water and unimproved sanitary facility, and low wealth quintile.⁵⁷

According to the NNS 2018, 40.2 percent of the children in Pakistan are stunted – that is, two in every five children under the age of 5 suffer from moderate or severe stunting. This is considerably higher than the global average of 21.3 percent and the South Asian average of 33.2. Moreover, 17.7 percent of Pakistani children in this age group suffer from wasting, compared to 6.9 percent globally and 14.8 percent in South Asia.⁵⁸ Boys appear to be more affected by all forms of malnutrition than girls. Almost one in three Pakistani children is underweight (28.9 percent). However, these findings reveal improvements in stunting and underweight rates compared to data from the NNS 2011, which identified rates of stunting and underweight of 43.7 percent and 31.5 percent (wasting rate deteriorated from 15.1 percent in 2011). It also identified marked disparities between urban and rural populations and found that the highest prevalence of stunting (48.3 percent) exists among children in the Merged Districts of Khyber Pakhtunkhwa (NMD), followed by Balochistan and GB (both at 46.6 percent), Sindh (45.5. percent) Khyber Pakhtunkhwa (40 percent) and Punjab (36.4 percent). Prevalence of stunting, wasting and underweight children remains extremely high in Pakistan, especially among poor and rural households – reflecting how closely malnutrition is tied to poverty. The estimated annual average reduction rate of 0.5 percent point is too slow to reduce stunting considerably in Pakistan by 2030 – the deadline for achieving the SDGs.⁵⁹

According to the NNS 2018, wasting is most prevalent in Sindh (23.3 percent) closely followed by Khyber Pakhtunkhwa’s Merged Districts (23.1 percent). Data from Balochistan reveals a prevalence of wasting of 18.9 percent. Gilgit-Baltistan and Islamabad Capital Territory have the lowest prevalence of wasting at 9.4 percent and 12.1 percent, respectively. Data trends indicate that wasting in Pakistan has been increasing in recent years, rising from 15.1 percent in 2011 to 17.7 percent in 2018, according to the NNS 2018. As such, the prevalence of wasting among children has exceeds the emergency threshold of 15 percent. Significant predictors of wasting in Pakistan include poor maternal nutrition (BMI<18.5), male child and unimproved sanitation (rural areas only).⁶⁰

⁵⁷ <<https://www.un.org/sustainabledevelopment/>>, accessed 6 August 2019.

⁵⁸ Relative importance of 13 correlates of child stunting in South Asia: Insights from nationally representative data from Afghanistan, Bangladesh, India, Nepal, and Pakistan, *Social Science & Medicine* 187 (2017) <<http://dx.doi.org/10.1016/j.socscimed.2017.06.017>>

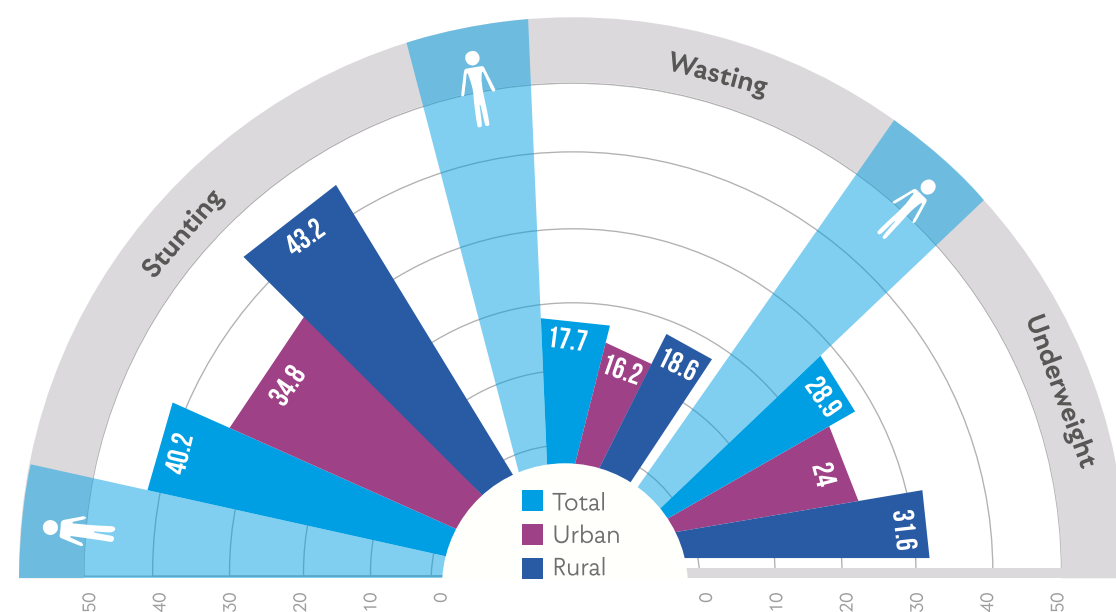
⁵⁹ UNICEF, WHO and WB (2020), *Joint Malnutrition Estimates 2020 Edition*. Geneva: World Health Organization.

⁶⁰ Nutrition Wing, Ministry of National Health Services, Regulation and Coordination, United Nations Children’s Fund, Aga Khan University, and UK Aid, *National Nutrition Survey 2018*, Government of Pakistan, Islamabad, 2019.

⁶¹ Relative importance of 13 correlates of child stunting in South Asia: Insights from nationally representative data from Afghanistan, Bangladesh, India, Nepal, and Pakistan, *Social Science & Medicine* 187 (2017) <<http://dx.doi.org/10.1016/j.socscimed.2017.06.017>>



Figure 1: Stunting, wasting and underweight in children under 5



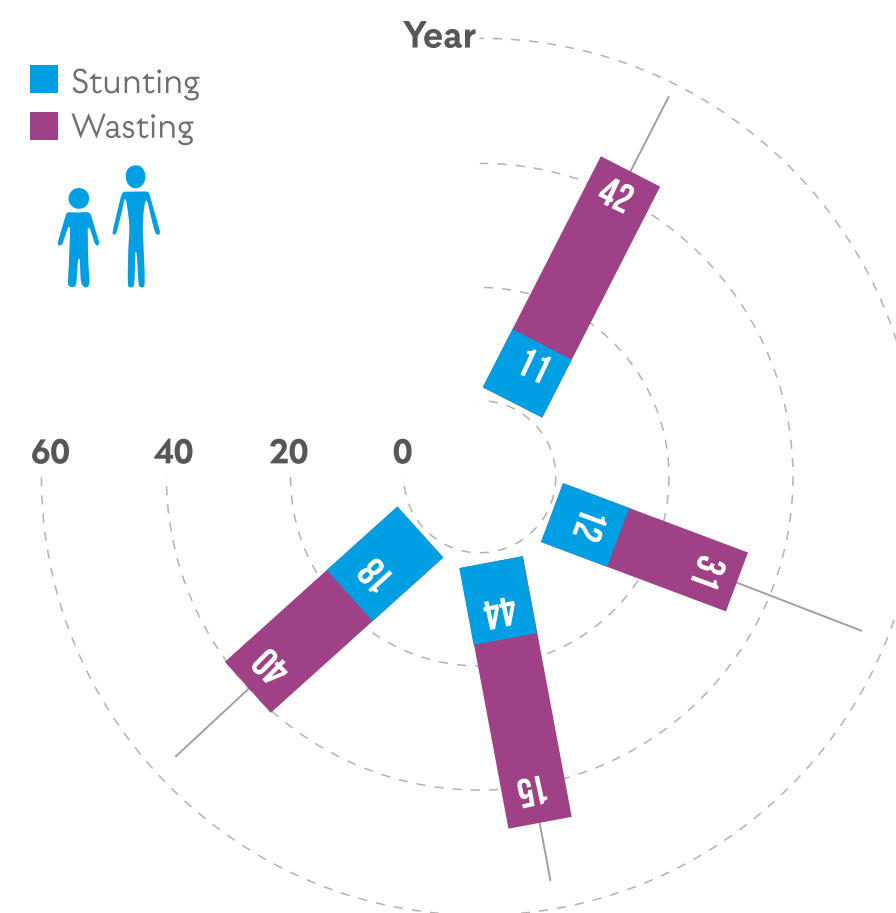
Source: National Nutrition Survey 2018

Stunting levels are generally high in all provinces of Pakistan. According to the NNS 2018, stunting appears especially widespread in the Merged Districts of Khyber Pakhtunkhwa (where it affects 48.3 percent of children), closely followed by Balochistan and Gilgit-Baltistan (both 46.6 percent), and Sindh (45.5 percent). The MICS 2016–17 in Gilgit-Baltistan also revealed the high prevalence of moderate to severe stunting (46.2 percent), and of severe stunting (22 percent), among children under-five. Khyber Pakhtunkhwa's MICS 2016–17 identified high levels of stunting in the province, with 41.4 percent of children suffering from moderate to severe stunting, and 20.7 percent severely stunted. MICS 2018 indicators for Punjab found a decline in stunting – which fell to 31.5 percent in 2018, from 33.5 percent in 2014.⁶¹

⁶¹ Punjab Bureau of Statistics and United Nations Children's Fund, *Multiple Indicator Cluster Survey 2014, Punjab*, Government of Punjab, Lahore, 2014.



Figure 2: Trends in stunting and wasting in children under 5 (%)

Source: National Nutrition Survey 1985, 2001, 2011 & 2018⁶²

⁶² The growth standard/reference changed from NCHS growth references (pre-2006) to 2006 WHO growth standards –the data for 1985 and 2001 has been stated on as is basis from NNS for relevant years and has not been reanalysed using WHO (2006) growth standards

The reasons for high stunting rates are manifold. They are linked to poor infant and young child feeding practices, a lack of exclusive breastfeeding, insufficient awareness of good eating practices and the effects of poverty and geographic (urban/rural) location.

MICRONUTRIENT DEFICIENCIES IN CHILDREN UNDER-FIVE

Nearly half of Pakistan's children are deficient in critical vitamins and minerals, including iron, vitamins A and D, zinc and calcium. Iron Deficiency Anaemia (IDA) in children under-five is a recognized public health problem with an adverse impact on child morbidity, mortality and cognitive development. More than half (53.7 percent) of Pakistani children are anaemic, of whom 5.7 percent have severe anaemia.⁶³ Children in rural areas are more likely to be anaemic (56.5 percent) than in urban centres (48.9 percent). While the prevalence of anaemia remains high among children under-five, it has declined from 61.9 percent in 2011. According to the PDHS 2017–18, nearly 38 percent of children aged 6–23 months eat iron-rich food once a day.

Over half (51.5 percent) of Pakistani children suffer from vitamin A deficiency, one of the leading causes of child blindness. While 12.1 percent of children in the country have severe vitamin A deficiency, 39.4 percent are affected by moderate deficiency.⁶⁴ According to the MICS 2016–17 in Khyber Pakhtunkhwa, only 66.8 percent of children in the province received vitamin A supplementation six months before the survey, compared to 76.9 percent in Gilgit-Baltistan and 64.1 percent in Punjab. The prevalence of vitamin D deficiency is especially high, affecting 62.7 percent of children under-five in Pakistan.⁶⁵ This is a major concern as vitamin D plays an essential role in maintaining bone health by regulating calcium concentrations in the body.



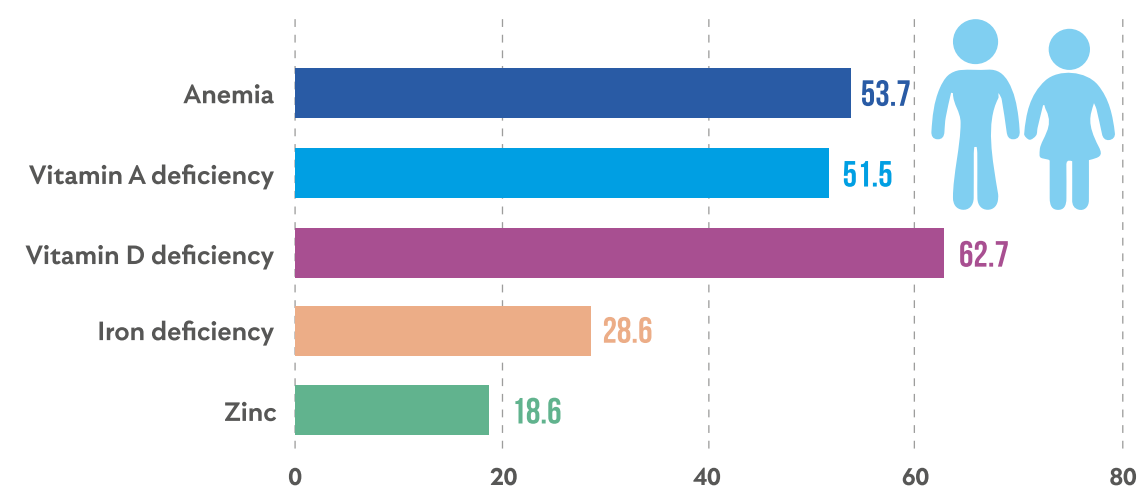
⁶³ Nutrition Wing, Ministry of National Health Services, Regulation and Coordination, United Nations Children's Fund, Aga Khan University, and UK Aid, *National Nutrition Survey 2018*, Government of Pakistan, Islamabad, 2019.

⁶⁴ Ibid.

⁶⁵ Ibid.

Zinc deficiency affects 18.6 percent of children in Pakistan. Zinc deficiency can occur in infants and children as a result of inadequate dietary intake or a disturbed zinc metabolism due to disease or inherent defects, and can lead to growth retardation and impaired immune functions. A slightly higher (19.5 percent) prevalence of zinc deficiency is evident among children in rural areas, compared to urban children (17.1 percent).⁶⁶ There has been a marked improvement in curbing zinc deficiency since 2011, when it affected 39.2 percent of children in the country.

Figure 3: Micronutrient malnutrition in children (%)



Source: National Nutrition Survey 2018

Iodine is essential for physical and mental development, as well as the normal functioning of the thyroid gland. Regarding Universal Salt Iodization (USI) in Pakistan, the NNS 2018 found that iodized salt consumption is more prevalent among urban households (84.4 percent) than their rural counterparts (76.7 percent), while the national average is 79.6 percent. Regional comparison shows that the iodized salt consumption is lowest in Khyber Pakhtunkhwa's Merged Districts (31.6 percent), whereas in Punjab, Islamabad Capital Territory, Gilgit-Baltistan, and Azad Jammu and Kashmir, the consumption of iodized salt exceeds 85 percent.

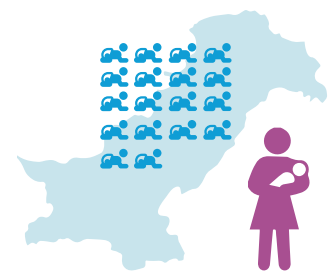
Pakistan's rates of maternal anaemia – a leading cause of preterm delivery, low birth weight and perinatal mortality – are high, affecting 41.7 percent of mothers.⁶⁷ Around 18.2 percent women of reproductive age suffer from iron deficiency anaemia, which is a leading cause of anaemia in infants and young children. This is more pronounced among women in rural areas (18.7 percent) than urban settings (17.4 percent).⁶⁸

⁶⁶ Ibid.

⁶⁷ Ibid.

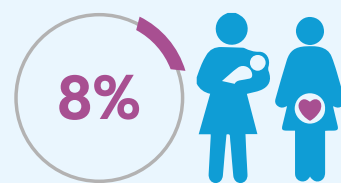
⁶⁸ Ibid.

Box 1: Adolescent mothers



PDHS 2017–18 data shows that the average birth rate is 46 births per 1,000 women (aged 15–19),

while 8 percent of adolescent women (aged 15–19) are already mothers or pregnant with their first child. The percentage of childbearing among women age 15–19 has not changed since 2012–13 (8 percent).



However, the percentage of women age 19 who have begun childbearing has increased from 17% to 19% between 2012–13 and 2017–18.

Early marriages and repeated pregnancies have a negative impact on adolescent women's health, and are a key determinant of maternal health outcomes. Adolescent mothers are inexperienced in childcare and their children are at greater risk of mortality.



BREASTFEEDING AND COMPLEMENTARY FEEDING PRACTICES

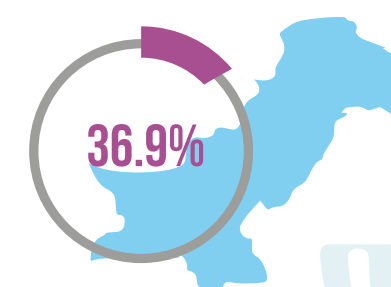
Pakistan has a strong culture of breastfeeding. The World Health Organization (WHO) recommends that children should be exclusively breastfed for the first six months of their lives, and that age-appropriate complementary feed should be introduced after this to prevent malnutrition. There has been a steady increase in the proportion of children receiving breastmilk during the first hour after birth between 2011 and 2018, however the trend for exclusive breastfeeding is not linear. From 50 percent in 2001, it declined to 37.7 percent in 2011 and then improved again to 48 percent in 2018. Exclusive breastfeeding in the first six months of life is highest in Khyber Pakhtunkhwa (60.7 percent) and Merged Districts (59.0 percent), and lowest in Azad Jammu and Kashmir (42.1 percent) and Balochistan (43.9 percent). Rates of age-appropriate complementary feeding alongside breastmilk for infants after 6 months of age was 35.9 percent. According to NNS data, Sindh (43.5 percent) and the Merged Districts of Khyber Pakhtunkhwa (45.5 percent) perform better than other areas on age-appropriate complementary feeding. The timely introduction of complementary feeding requires significant improvement in Pakistan overall, especially in Islamabad Capital Territory (21.9 percent) and Balochistan (22.3 percent).⁶⁹ The latest MICS data shows similar trends, identifying rates of exclusive breastfeeding among infants under 6 months in Khyber Pakhtunkhwa, Gilgit-Baltistan and Punjab of 57.2 percent, 63 percent and 42.1 percent, respectively.

⁶⁹ Nutrition Wing, Ministry of National Health Services, Regulation and Coordination, United Nations Children's Fund, Aga Khan University, and UK Aid, *National Nutrition Survey 2018*, Government of Pakistan, Islamabad, 2019.

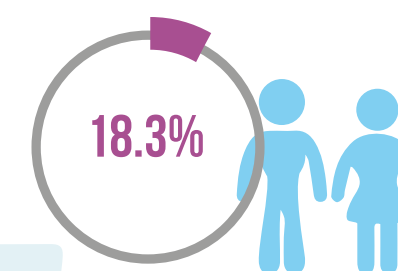
Quality complementary feeding is assessed using three indicators: minimum dietary diversity, minimum meal frequency and minimum acceptable diet. The NNS 2018 that Pakistan's performance on all three indicators is below acceptable levels. It is also important to note that, over time, performance on these three complementary indicators has declined rapidly. Only 14.2 percent of children between the 6 and 23 months old receive meals that meet minimum dietary diversity standards, only 18.2 percent receive the minimum number of meals per day, and only 3.6 percent receive a minimum acceptable diet to ensure optimal growth. For Pakistan to meet SDG target 2.2, it needs to invest in the robust, large-scale promotion of adequate complementary feeding practices to reduce stunting and micronutrient deficiencies among children.

Box 2: Food security

Although Pakistan is a food surplus country, 36.9 percent of its households are food insecure and 18.3 percent are affected by severe food insecurity (hunger). Overall, the country's food insecurity situation has improved since 2011, when food insecurity affected 58.1 percent of its households.

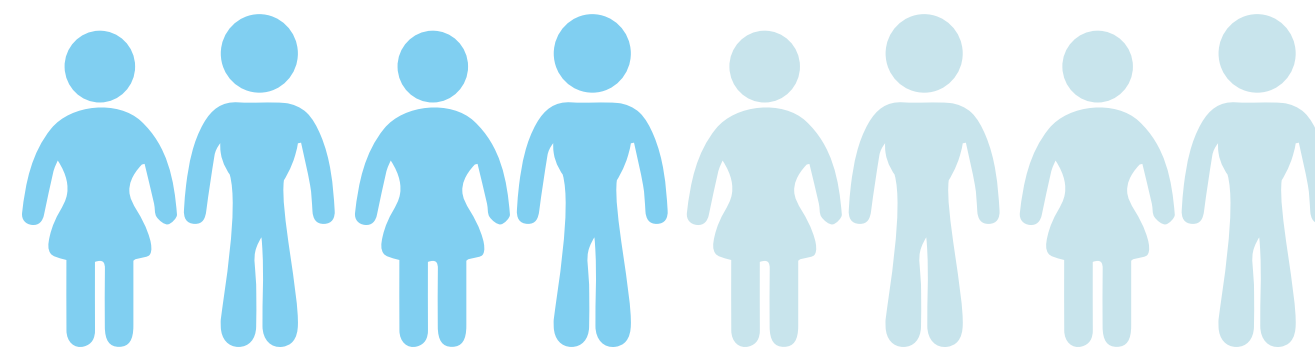
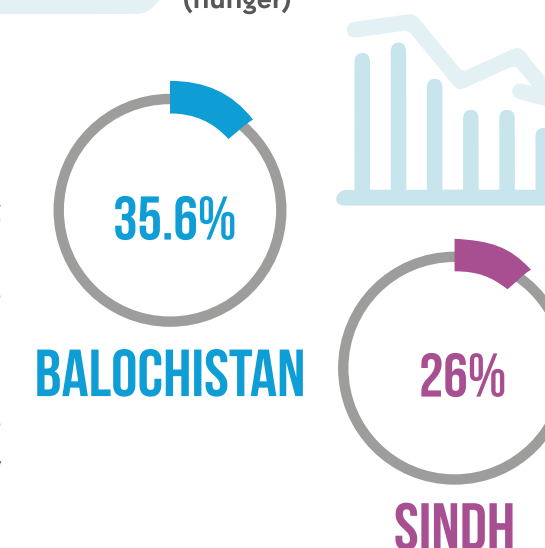


36.9 per cent of Pakistan households are food insecure



18.3 per cent are affected by severe food insecurity (hunger)

However, severely insecure households afflicted by hunger have nearly doubled from 9.8 percent in 2011. Pakistan ranking on the Global Hunger Index has improved – rising to 94th of 117 countries in 2019 compared to 106th in 2017 – and the proportion of the population that is undernourished declined to 20.3 percent in 2019, down from 21.1 percent in 2010. Balochistan and Sindh have highest percentage of people affected by severe food insecurity – 35.3 percent and 26 percent, respectively.



NUTRITION SERVICES

Structural factors that contribute to poor nutritional indicators include limited coverage of nutrition services; difficulties in tracking public expenditure on nutrition through financial reporting systems; inadequate systems for monitoring and evaluation; and delays in the implementation of provincial multi-sectoral nutrition strategies. To date, most nutrition initiatives in Pakistan have focused on treating acutely malnourished children, which alleviates the symptoms of malnutrition without addressing its root causes. Pakistan clearly needs strong institutional mechanisms and capacities, particularly at the sub-national levels. It requires greater ownership of nutrition interventions at the district level, and a greater focus on preventing malnutrition by tackling its root causes. One of the key barriers is the project-based approach to nutrition within MoH – and thus lack of full integration into the health system. The nutrition sector receives relatively small, short term and unpredictable budgetary allocations, making it heavily dependent on donors. Public sector interventions tend to be oriented towards treatment, rather than prevention. A lack of investment in nutrition overall means a lack of investment in building the capacity of stakeholders and trained professionals involved in nutrition interventions. Thus, not only are nutrition initiatives few and far between, they also do not focus sufficiently on strengthening capacity and improving quality in a cost-effective manner – such by as taking a preventive, rather than curative, approach to malnutrition and undernutrition.

Improving adolescents' nutrition, moreover, is not prioritized in government planning and programming. Although, Pakistan has a strategy on adolescent nutrition, maternal and adolescent nutrition represents the weak link of the lifecycle approach as implemented in Pakistan.

NEONATAL, INFANT AND CHILD MORTALITY

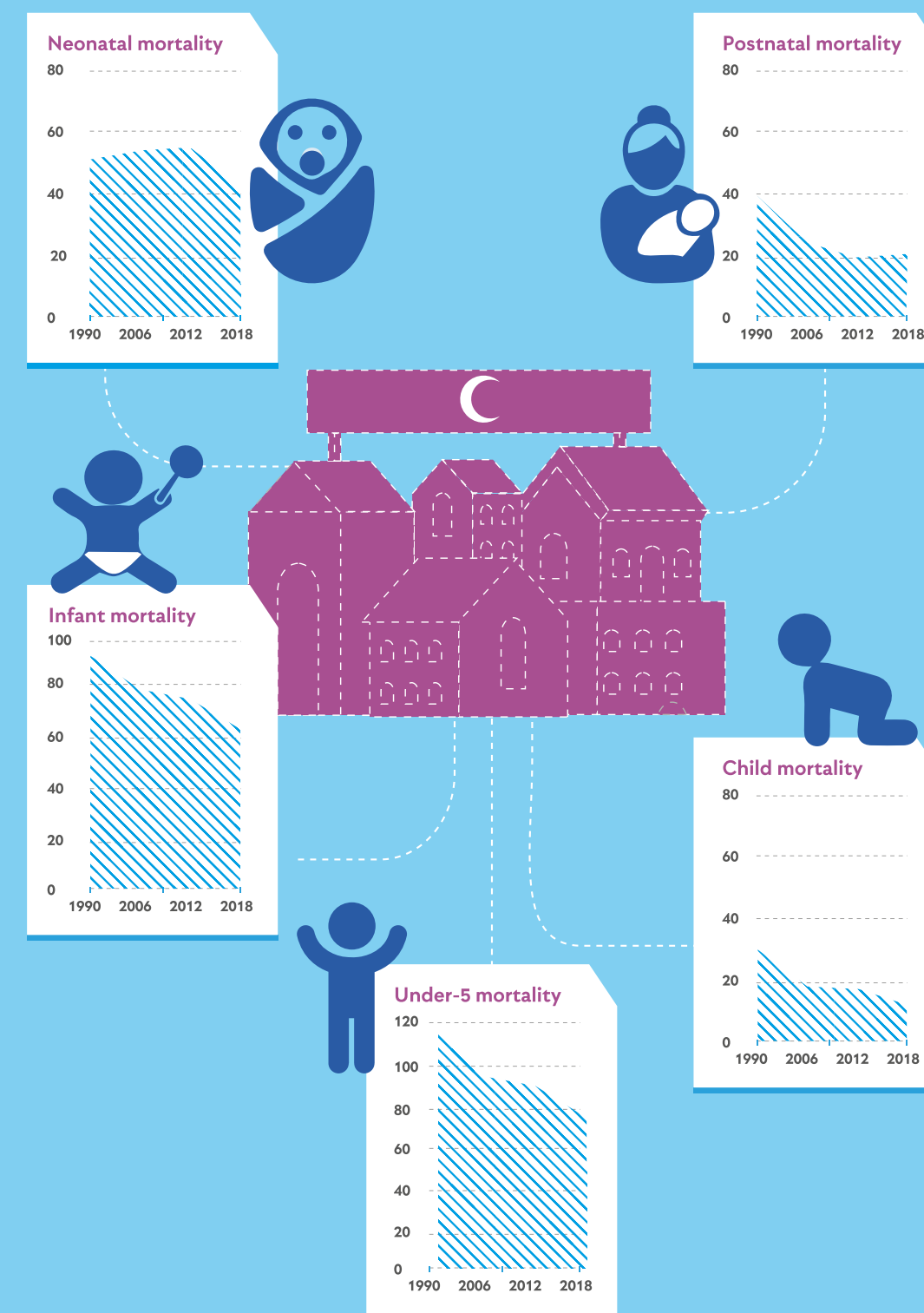
SDG target 3.2 aims to end preventable deaths among newborns and children under five years old. The global target calls on countries to reduce neonatal mortality to as low as 12 deaths per 1,000 live births, and under-five mortality rates to as low as 25 deaths per 1,000 live births. However, reducing child mortality remains highly challenging for Pakistan, given the nexus between poverty, inequity, inequality, quality of care, social injustice and food insecurity.

According to the PDHS 2017–18, child mortality rates have declined in the five years since the last demographic and health survey in 2012–13. The neonatal mortality rate (NMR) declined from 55 to 42 deaths for every 1,000 live births, while the infant mortality rate (IMR) declined from 74 to 62 deaths per 1,000 live births. Moreover, under-five mortality (U5M) fell from 89 to 74 per 1,000 live births. Despite progress, the current rates still mean that one in every 14 Pakistani children does not survive to their fifth birthday⁷⁰ and that the death of newborns within the first week of life accounts for half of all infant mortality.⁷¹

⁷⁰ National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

⁷¹ Jalil F, et al, 'Early child health in Lahore, Pakistan: IX. Perinatal events', *Acta Paediatrica Supplement*, vol. 82, no. 390, 1993, pp. 95–107.

Figure 4: Trends in early childhood mortality rates (per 1,000 live births)



Source: Pakistan Demographic and Health Survey (PDHS) 1990, 2006, 2012 & 2018

The U5MR differs greatly across urban and rural settings, as well as across regions, provinces, wealth quintiles, and maternal education levels. In terms of provinces and regions, U5MR is 85 deaths per 1,000 live births in Punjab, 77 per 1,000 live births in Sindh, 64 per 1,000 live births in Khyber Pakhtunkhwa, 33 deaths per 1,000 live births in Khyber Pakhtunkhwa's Merged Districts (known as the Federally Administered Tribal Areas (FATA) at the time of data collection. There have been data quality challenges due to access and security issues in FATA during the fieldwork as noted in PDHS 2017-18) and 78 per 1,000 live births in Balochistan.⁷² According to PDHS, U5MR in rural areas is 83 deaths per 1,000 live births compared to 56 deaths per 1,000 live births in urban areas of Pakistan. Urban-rural disparities suggest a lack of available health services, limited access to services due to difficult terrain and poor infrastructure, the poor quality of health services, and high rates of poverty in rural areas.

Childhood mortality rates decrease uniformly as mother's education increases. For example, U5MR is 91 deaths per 1,000 live births among children whose mothers have no education and 48 and 38 deaths per 1,000 live births among children whose mothers have secondary and higher education, respectively.⁷³ In addition, the U5MR is higher where birth spacing is not optimal – that is, when a child is born less than two years after a previous birth (122 deaths per 1,000 live births), compared to children born three years after a previous birth (41 deaths per 1,000 live births). Modern contraceptive services and maternal and newborn health

care are essential for promoting the well-being of women, their families and communities. According to the PDHS 2017-18, the contraceptive prevalence rate (CPR) in Pakistan is only 34 percent among women age 15-49. Socio-cultural barriers create bottlenecks that prevent young people from seeking support and services to meet their physical, mental, emotional and psychological needs.

Analysis of Punjab's MICS 2018 data also indicates that maternal age has an impact on mortality rates. This data reveals that Punjab's IMR (85 deaths per 1,000 live births) and U5MR (97 deaths per 1,000 live births) are higher when mothers are under 20 years old.

Around the world, of the 6.2 million deaths among children and young adolescents under 15 years of age, 85 percent (5.3 million) occurred in the first five years of life as recorded in 2018, and nearly half (47 percent) of deaths among children under-five occurred in the neonatal period (2.5 million).⁷⁴ Globally, the leading cause of death among children under-five in 2018 was pre-term birth complications (16 percent), followed by pneumonia (12 percent), inter-partum events (11 percent), diarrhoea (8 percent), neonatal sepsis (7 percent), and malaria (5 percent). Infectious diseases remain the leading cause of under-five mortality in Pakistan: diarrhoea and pneumonia are the prime causes of child mortality in the country. Each year, approximately 91,000 child deaths are attributed to pneumonia and 53,300 to diarrhoea.⁷⁵

⁷⁴ Levels and trends in child mortality. Report 2019. Estimates developed by the UN inter-agency group for child mortality estimation.

⁷⁵ United Nations Children's Fund, *Mid-Term Evaluation of Project for Accelerating Policy Change, Translation and Implementation for Pneumonia and Diarrhoea Commodities 2016–2020: Pakistan*, UNICEF, Islamabad, 2016.

⁷² National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

⁷³ National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.



BASIC VACCINATION COVERAGE

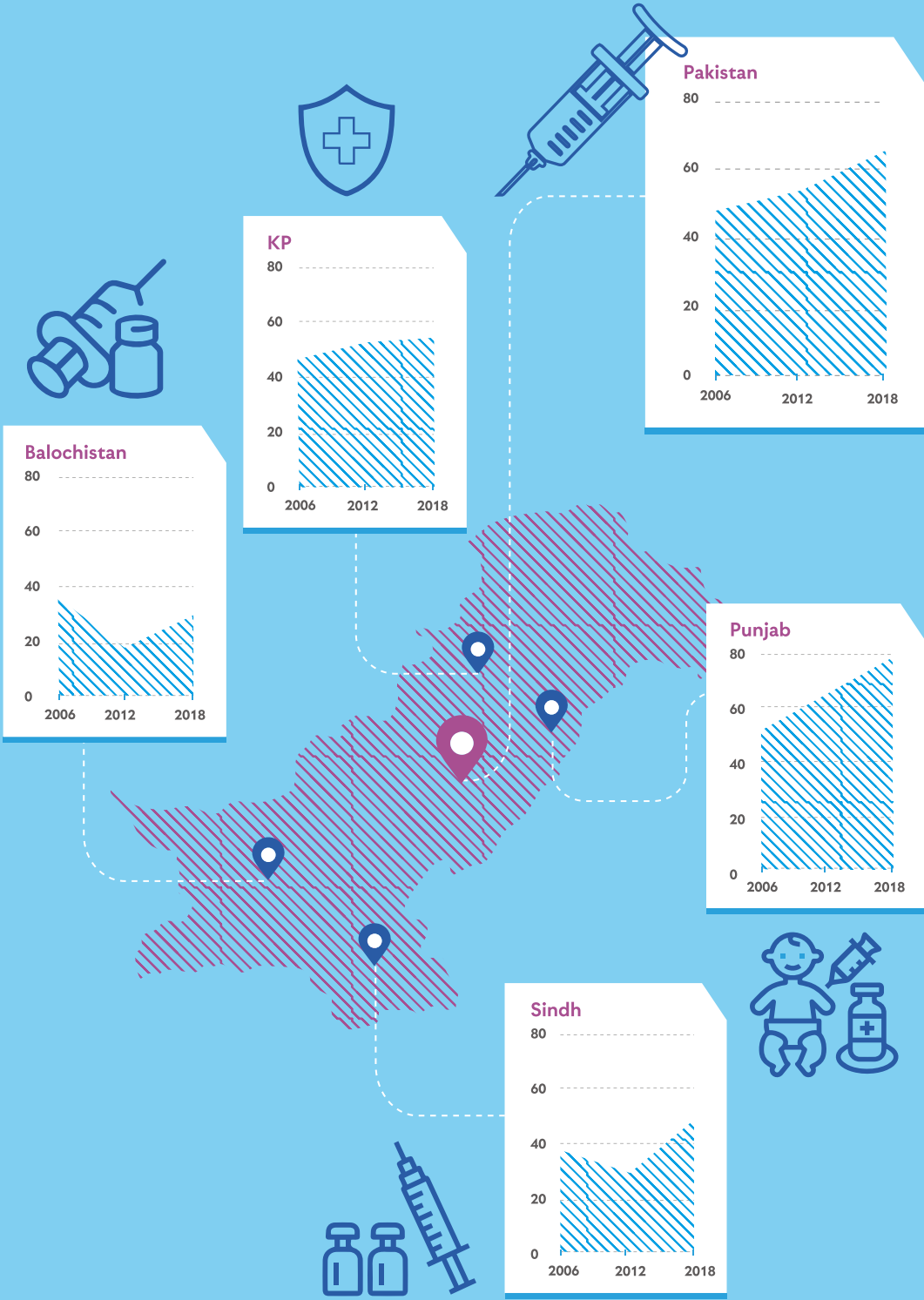
The universal immunization of children against six common vaccine-preventable diseases – tuberculosis, diphtheria, pertussis, tetanus, polio and measles – is crucial to reducing infant and child mortality. Pakistan has achieved major milestones in basic vaccination coverage. Two-thirds⁷⁶ of children (66 percent) between 12 and 23 months old have received all eight basic vaccinations⁷⁷ under the Expanded Programme for Immunization (EPI), a considerable improvement from 54 percent in 2012–13. Overall, only 4 percent of children in this age group have not received any basic vaccinations compared to over 5 percent in 2012–13.⁷⁸ Despite improvement in immunization coverage, Pakistan is home to around 1.4 million un- and under-immunized children residing primarily in high risk urban, remote rural and conflict areas. The focus on zero dose children through cross sectoral approach is crucial to address the subnational disparities in immunization outcomes and hence the child health outcomes.

Basic vaccination coverage is lowest in Balochistan (29 percent), although rates in the province have improved in the past five years (up from 16 percent). Vaccination coverage is highest in Punjab (80 percent), followed by Azad Jammu and Kashmir (75 percent) and Islamabad Capital Territory (68 percent). Rates are considerably lower in Gilgit-Baltistan (57 percent), Khyber Pakhtunkhwa (55 percent), Sindh (49 percent), and Khyber Pakhtunkhwa’s Merged Districts (30 percent), where coverage is only one percentage point higher than in Balochistan.

76 National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.
77 To have received all basic vaccinations, a child must receive at least: One dose of BCG vaccine, which protects against tuberculosis; three doses of DPT vaccine, which protects against diphtheria, pertussis (whooping cough), and tetanus; three doses of polio vaccine; and one dose of measles vaccine
78 National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2012–13*, NIPS, Islamabad, 2013.



Figure 5: Immunization coverage across Pakistan



Source: Pakistan Demographic and Health Survey (PDHS) 2006, 2012 & 2018

Measles remains a leading cause of death among children, with survivors often left with life-long disabilities such as blindness, deafness or brain damage. The Government of Pakistan has shown a strong political commitment to regional measles elimination target and kick-started a nationwide measles campaign to vaccinate more than 32 million children with support from WHO and UNICEF in 2018.⁷⁹ The total number of suspected measles cases in 2019 were 9,031 (2,699 reported to-date in 2020). This translates into a reduced incidence rate of 6.04 per million people in 2020, down from 9.67 per million in 2019. However, in the current context of COVID-19, monthly Penta vaccinations have declined since January 2020, due to the disruption in service delivery and preventive services. Although the Government of Punjab continues providing preventative services, these have been completely curtailed in Balochistan and Gilgit-Baltistan. A lack of protective personal equipment (PPE) prevented health workers and vaccinators from delivering immunization services. Moreover, a number of vaccines – including Penta – could not be procured once Pakistan closed its airspace in 2020. As a result of the lockdown, some 80,000 children missed their most recent dose of Penta vaccine.⁸⁰

Infectious diseases in Pakistan are one of the main contributors to the burden of disease. Incidence of communicable diseases – tuberculosis, malaria, dengue fever, typhoid, viral hepatitis and a number of other infections caused by bacteria, viruses, fungi and parasites

remained on the rise in 2019.⁸¹ In 2019, Pakistan faced the worst ever outbreak of dengue fever with more than 45,000 people infected with the dengue virus between July and November 2019.⁸² Ministry of National Health Services Regulations and Coordination (MoNHSRC) has established Dengue Control & Operational Cell at the National Institute of Health (NIH) and launched an extensive community awareness campaign, particularly Rawalpindi and Islamabad, where dengue cases are most concentrated. Extensive social mobilization, community engagement, and vector control activities need to be rolled out in all the affected areas.

Polio remains a challenge in Pakistan and Afghanistan, the countries in the world where transmission of the poliovirus has never ceased. Progress was apparent between 2014 and 2017, when wild poliovirus cases dropped from 306 cases to eight. Since then, however, the situation has worsened, with cases steadily increasing in 2018 (12 cases), 2019 (147 cases) and 2020 (49 as of May 2020). This situation is further complicated by the circulation of vaccine-derived polio virus type 2 circulating since the middle of 2019. As of June 2020, 69 such cases had been detected, reflecting low essential immunization coverage rates across Pakistan. While the virus is extensively spread across the country, most cases are identified in Khyber Pakhtunkhwa (63 percent). Of these cases, 71 percent are among Pashto-speaking households, and 61 percent have never received any doses of OPV in routine vaccinations. Although the entire country

79 WHO, Regional Office for the Eastern Mediterranean <<http://www.emro.who.int/pak/pakistan-news/who-and-unicef-join-forces-to-support-pakistans-mass-measles-immunization-campaign.html>>

80 UNICEF Pakistan, Health Team

81 <https://www.thenews.com.pk/print/600776-pakistan-continues-to-face-double-burden-of-diseases-in-2019>

82 Government of Pakistan, Pakistan Economic Survey 2019–20. <http://www.finance.gov.pk/survey/chapter_20/11_Health_and_Nutrition.pdf>

remains at risk of polio's spread, a key focus remains on core reservoirs and 40 selected 'super high-risk' union councils within these core reservoirs. These include Karachi, Peshawar, Quetta, Pishin and Killa Abdullah. Serious hurdles facing the polio eradication programme include structural and operational challenges, limited delivery of other basic services, misconceptions and mistrust of the vaccine and the programme, and intricate tribal and cultural norms.⁸³ In its 16th report, the Independent Monitoring Board (IMB) of the Global Polio Eradication Initiative recognized that communities most at-risk of polio are largely those with limited access to fresh water, poor sanitation, and a lack of public service infrastructure, including proper health care provision. These are also the communities most likely to be hostile to offers of the oral polio vaccine.

Poor infrastructure and access constraints make it difficult to achieve vaccination coverage in far flung parts of the country. As a result, immunization rates are especially low in regions with remote, scattered populations, including the Merged Districts of Khyber Pakhtunkhwa, Balochistan and some districts in Gilgit-Baltistan. The poorest segments of society suffer from particular inequities in terms of their utilization of immunization services. Alongside poverty, low literacy rates in remote areas are another factor that contributes to low vaccination coverage.

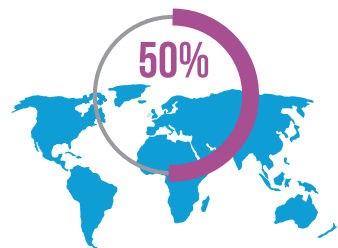
The Expanded Programme on Immunization's (EPI) team has collectively identified priority districts in the country, based on the number of children 'missed' by vaccination drives,

and low immunization coverage rates. These districts exist in some of the most difficult areas in the country. As such, they require customized strategies for service delivery and demand generation. Factors that contribute to low immunization coverage in these districts include caregivers' literacy levels, especially of mothers; socioeconomic status; religious and social pressures; the quality and availability of fixed site and outreach services; security threats to vaccinators and social mobilizers; and the limited capacity of frontline health workers. To overcome such challenges and bottlenecks, there is a need to allocate human resources to remote and underserved areas, to incentivize these workers, to train and mobilize community health workers (CHWs) and LHWs who hail from these communities, and to spearhead awareness raising programmes to mitigate social taboos surrounding vaccination.



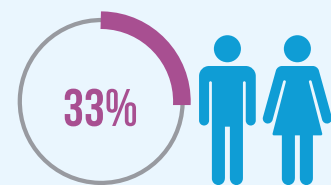
83 UNICEF Pakistan, Polio Team.

Box 2: Urban slums



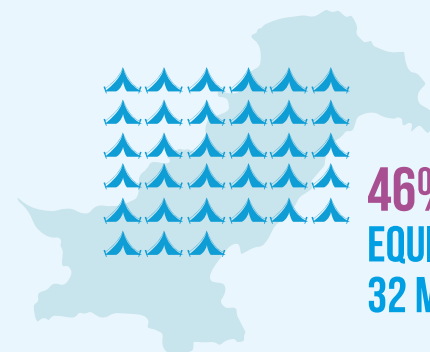
Urbanization is a demographic trend in both the developed and developing world. Rates of urbanization exceed 50 per cent globally and over 1 billion people live in urban slum environments.

These trends are evident across South Asia, including Pakistan, where the absolute numbers of populations in slums are on the rise. Some 33 per cent of the 40.9 million people in Pakistan's 10 megacities (Karachi, Hyderabad, Faisalabad, Gujranwala, Lahore, Multan, Rawalpindi, Quetta, Islamabad and Peshawar) live in 3,096 slum areas.



Approximately 36 per cent of these areas are not registered by local authorities in eight cities. Slum growth is driven in part by accelerating rates of urbanization across these 10 megacities.

In 2014, estimates suggested that 46 per cent of Pakistan's urban population lived in slum areas (referred to as 'katchi abadis'), equivalent to over 32 million people.



LOW IMMUNIZATION COVERAGE

Political, socio-economic, environmental and health system determinants shape patterns of low health care access and utilization in slums and underserved areas. As discussed above, the immense size of the urban population has important implications for service delivery, particularly for children. Low immunization coverage and the proportion of children have missed all of their vaccines (14 per cent) – also called 'zero-dose children' – are associated with socio-economic factors, including:



Poor housing and environmental standards



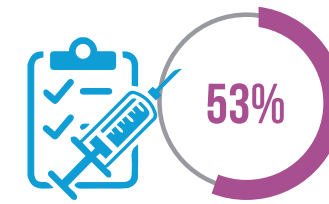
under-employment



low income status

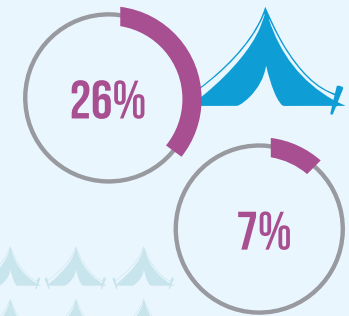


low literacy levels



While 53 per cent children in Pakistan are fully immunized, there remains an inadequate supply of health facilities, outreach services and human resources to provide essential immunization, environmental, maternal and child health services.

Overall, 26 per cent of union councils in slums and underserved areas are without EPI facilities, and just 7 per cent of EPI facilities are within a 2 kilometres radius of slum residents.



Low levels of social capital, community organization and trust in modern health care systems have been witnessed in urban slums. To safeguard the health of children and women in slums, there is a need to improve social and environmental conditions, including



safe water



sanitation



waste removal



housing

In essence, major investments are required in social and public health infrastructure. Action on environmental health is critical, given that at least 23 per cent of all mortality worldwide is attributable to environmental factors.

Source: United Nations Children's Fund, Health and Immunisation Services for the Urban Poor in Pakistan: A summary report of health profiles, health service assessments and immunisation coverage surveys in the 10 largest cities of Pakistan, UNICEF, Islamabad, 2020.

CHILDHOOD ILLNESSES

SDG 3 defines targets for key newborn and child health indicators, which Pakistan has committed to. SDGs 3.2's targets include ending the preventable deaths of newborns and children under-five by preventing diseases such as diarrhoea, pneumonia and malaria – among the leading killers of children under-five. According to WHO, early childhood diseases and infections are the leading causes of morbidity and mortality in developing countries, including acute respiratory infections (ARI), fever and dehydration from diarrhoea.

Proper care for newborns is essential to reduce neonatal problems and death. The Government of Pakistan has taken many measures to decrease neonatal mortality and morbidity rates with the

help of UNICEF, WHO, and other partners, but desirable goals are yet to be achieved. There is broad consensus that, in order to achieve the global targets outlined in Every Newborn Action Plan (ENAP) for neonatal deaths and stillbirths (< 10/1000 births), the country will need to make significant improvements in in-patient care for Newborns and Young Infants (NYIs). According to PDHS 2017-18, Pakistan has a high perinatal mortality rate of 57/1000 total births. Maternal factors for stillbirths have been identified as extreme maternal ages (below 18 and above 35), parity of zero or more than 3 (nulliparous and multiparous), lack of education and poor socio-economic status, poor health-seeking behaviour, under-nutrition, anaemia, lack of autonomy and inadequate number of ANC visits. Obstetric causes included; antepartum haemorrhage, obstructed labour, placental disorders, and pregnancy-induced hypertension as factors leading to stillbirth.⁸⁴

According to PDHS 2017-18, only 64 percent of newborns received a postnatal check within the first 2 days after birth and 41 percent of newborns had a postnatal check within the first hour of life, while 35 percent of newborns did not receive any postnatal check and only 19 percent of newborns were weighed during the first 2 days after their birth. The National Maternal, Neonatal, and Child Health (MNCH) program has launched the Integrated Management of Newborn and Childhood Illness (IMNCI) strategy as an integrated approach to management of infectious diseases such as pneumonia, diarrhoea, malaria, and measles, and also chronic malnutrition, among children age 2 months to 5 years.

In the past five years, the prevalence of diarrhoea in children under the age of five has declined in Pakistan, falling from 23 percent⁸⁵ in 2012–2013 to 19.1 percent in 2017–18.⁸⁶ The lowest prevalence of diarrhoea in children is evident in Sindh (14.4 percent) and Azad Jammu and Kashmir (14.2 percent). However, the problem continues to loom large in the other parts of the country, most notably in Khyber Pakhtunkhwa (21.3 percent), Punjab (20.5 percent), Khyber Pakhtunkhwa's Merged Districts (19.9 percent), Balochistan (18.6 percent) and Gilgit-Baltistan (16 percent).

According to PDHS data, 14 percent of children under-five showed symptoms of acute respiratory infection, and 84.2 percent of children with symptoms sought treatment or advice from a health service provider. Overall, 71 percent of children with diarrhoea in this age group received treatment or advice in 2017–18, up from 61 percent in 2012–13. Oral rehydration therapy (ORT) is essential for children with diarrhoea, including oral rehydration salts (ORS) and zinc supplements. The survey's results show that, on average, 37 percent of children with diarrhoea in Pakistan received ORS, only 8 percent received both ORS and zinc, and 14 percent received no treatment whatsoever.

84 UNICEF, Aga Khan University and Government of Pakistan. A Situational Analysis on Stillbirths, Newborn Deaths and Small and Sick Newborn Care. Findings from Pakistan - 2019.

85 National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2012–13*, NIPS, Islamabad, 2013.

86 National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

Regional data sheds further light on these trends. In Punjab, for example, diarrhoea is most prevalent seen among children between 36 and 47 months old.⁸⁷ While treatment from a health facility or provider is sought for 65.7 percent of children under-five with diarrhoea, only 34.6 percent of cases are treated with ORS and 12.8 percent with both ORS and zinc. In Khyber Pakhtunkhwa, diarrhoea is most prevalent among children between 12 and 23 months old – that is, during the weaning period.⁸⁸ Overall, a health facility or provider was consulted on diarrhoea among children in only half (51 percent) of such cases. Care-seeking to treat diarrhoea is highest in the district of Dera Ismail Khan (60 percent) in the province, and lowest in the district of Malakand (42 percent). Overall in Khyber Pakhtunkhwa, 32 percent of children with diarrhoea received ORS, while 7 percent received both ORS and zinc. The treatment of diarrhoea is more prevalent in urban parts of the province (43 percent) than in rural areas (37 percent). This is also true for acute respiratory infections, with more children under-five in urban areas receiving treatment (70 percent) than in rural settings (57 percent). The data also reveals a positive relation between a mother's level of education and health care-seeking behaviour, as well as between household wealth and treatment for childhood illnesses. For instance, health facilities or providers were consulted in 51 percent cases when children aged 0 to 59 months had diarrhoea in Khyber Pakhtunkhwa, with levels ranging from 48 percent when mothers only had a primary education, to 62 percent when mothers had a higher level of education.

Encouraging trends are evident in terms of child illnesses in the latest PDHS. Advice and treatment is sought for a roughly equal proportion of boys (84.6 percent) and girls (84.4 percent) suffering from diarrhoea, fever and acute respiratory infections. Regional disparities persist, however, and treatment lags behind for children in Khyber Pakhtunkhwa's Merged Districts and Balochistan. This appears linked to the underlying causes of low levels of development in these regions, high poverty rates and inequity, as well as poor infrastructure, inadequate WASH services, a lack of access to facilities, and challenging geographic terrain.

IMPACT OF HIV/AIDS ON CHILDREN

April 2019 witnessed a surge of HIV cases among children in Ratodero taluka (administrative sub-division), in the district of Larkana, Sindh.⁸⁹ The vast majority (82 percent, or 719 of 876 cases) were detected in children under the age of 15, and 79 percent (604 cases) were among children under the age of five. Of the 31,239 individuals tested between April and July 2019, 3 percent tested positive for HIV; however, HIV prevalence was higher in children between 0 and 2 years old (7 percent) and those between 3 and 5 years old (6 percent).⁹⁰

87 Punjab Bureau of Statistics and United Nations Children's Fund, *Multiple Indicator Cluster Survey 2017-2018, Punjab*, Government of Punjab, Lahore, 2018.

88 Khyber Pakhtunkhwa Bureau of Statistics, Planning & Development Department, and United Nations Children's Fund, *Khyber Pakhtunkhwa Multiple Indicator Cluster Survey 2016-17*, Government of Khyber Pakhtunkhwa, Peshawar, 2017.

89 World Health Organization, 'HIV cases–Pakistan', WHO, Islamabad, 3 July 2019, <<https://www.who.int/csr/don/03-july-2019-hiv-cases-pakistan/en>>, accessed 4 May 2020.

90 Mir, Fatima, et al., 'HIV infection predominantly affecting children in Sindh, Pakistan, 2019: a cross-sectional study of an outbreak', *The Lancet*, vol. 20, no. 3, 2020, pp. 362–370, <[https://www.thelancet.com/journals/laninf/article/PIIS1473-3099\(19\)30743-1/fulltext](https://www.thelancet.com/journals/laninf/article/PIIS1473-3099(19)30743-1/fulltext)>, accessed 4 May 2020, <<https://www.ncbi.nlm.nih.gov/pubmed/31866326>>, accessed 4 May 2020.

Parent-to-child transmission of HIV is prevalent in Pakistan,⁹¹ yet the country has a very low (less than 6 percent) rate of coverage of services for the prevention of parent-to-child transmission (PPTCT).⁹² A concentrated epidemic scenario – as in Ratodero – makes it especially difficult to identify HIV positive pregnant women, due to the absence of an effective mechanism to actively detect, trace and find parents who are HIV positive, and to link them with available services. In April 2019, an HIV screening camp for all ages was established in response to a report of an unusually large number of paediatric HIV diagnoses in Larkana. Of children whose mothers were tested for HIV, only 11 percent had HIV-positive mothers. Most children who tested positive for HIV in Sindh in 2019 had a history of multiple injections (89 percent), and some of blood transfusions (9 percent).⁹³ During the screening, several risk factors were identified, including: unsafe intravenous injections during medical procedures, unsafe child delivery practices, unsafe practices at blood banks, poorly implemented infection control programmes, and the improper collection, storage, segregation and disposal of hospital waste.



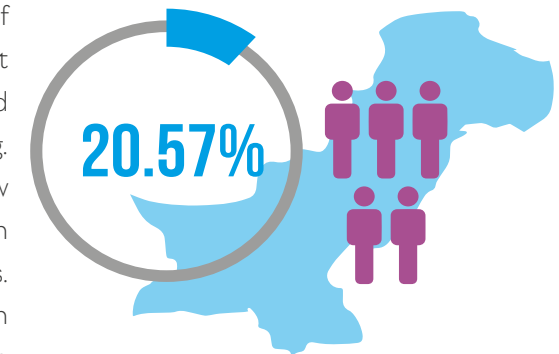
⁹¹ World Health Organization, 'HIV cases–Pakistan', WHO, Islamabad, 3 July 2019, <<https://www.who.int/csr/don/03-july-2019-hiv-cases-pakistan/en>>, accessed 4 May 2020.

⁹² United Nations Children's Fund, *Study on Review of HIV Treatment, PPTCT & Paediatric Services*, UNICEF, Islamabad, 2019.

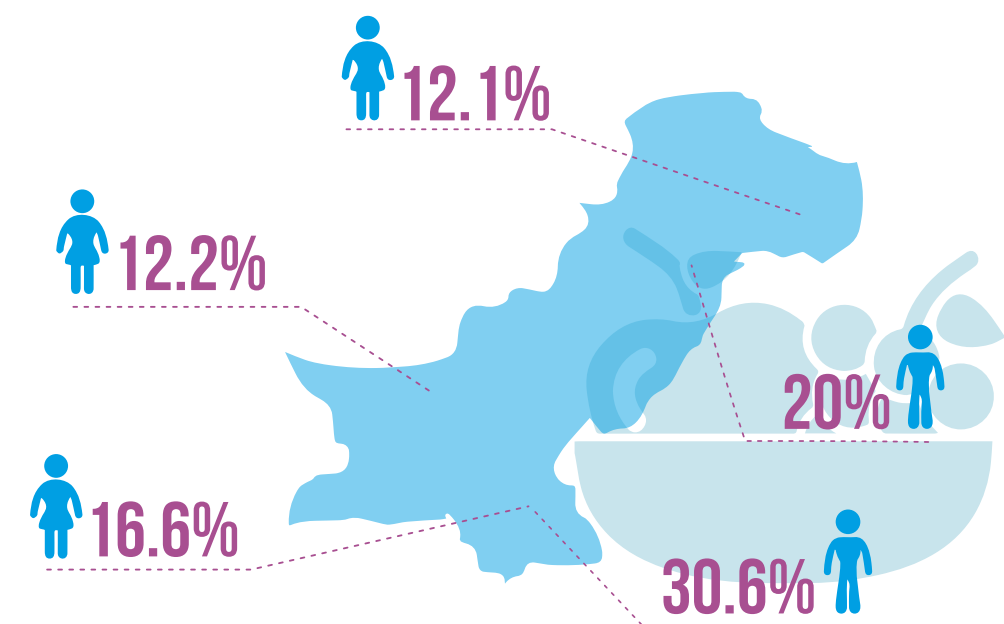
⁹³ Mir, Fatima, et al., 'HIV infection predominantly affecting children in Sindh, Pakistan, 2019: a cross-sectional study of an outbreak', *The Lancet*, vol. 20, no. 3, 2020, pp. 362–370, <[https://www.thelancet.com/journals/laninf/article/PIIS1473-3099\(19\)30743-1/fulltext](https://www.thelancet.com/journals/laninf/article/PIIS1473-3099(19)30743-1/fulltext)>, accessed 4 May 2020, <<https://www.ncbi.nlm.nih.gov/pubmed/31866326>>, accessed 4 May 2020.

Box 3: Adolescent health and nutrition status (aged 10–19)

Adolescents account for 20.57 per cent of Pakistan's population. Despite this, adolescent health and nutrition has not been a prioritized in government planning or programming. Adolescence is a period that offers a window of opportunity to invest in the health, nutrition and family well-being of future adults. Nationwide, the double burden of malnutrition among adolescents is a serious concern, with a considerable proportion either underweight or overweight/obese. There is a clear need to focus on mitigating the risk of non-communicable diseases (NCDs) among them.



The NNS 2018 reveals that one in eight adolescent girls is underweight, as is one in five adolescent boys is underweight. Thus, adolescent boys are more affected (21.1 per cent) than girls (11.8 per cent), with similar trends in urban and rural settings. Provincial analysis indicates that there are more underweight girls in Sindh (16.6 per cent) Balochistan (12.2 per cent) and Azad Jammu and Kashmir (12.1 per cent) than in other parts of the country. In tandem, 30.6 per cent of adolescent



boys in Sindh and 20 per cent in Islamabad Capital Territory are underweight. The NNS 2018 highlights that more than half (56.6 per cent) of adolescent girls in Pakistan are anaemic, although the prevalence of severe anaemia is low (0.9 per cent) among girls. Obesity is a public health issue of concern in Pakistan, both in urban and rural areas, affecting 7.7 per cent of boys and 5.5 per cent of girls. As noted above, social-cultural barriers create bottlenecks that prevent young people from seeking support or services to meet their physical, mental, emotional and psychological needs.

MATERNAL MORTALITY, MORBIDITY AND NUTRITIONAL STATUS

Pakistan's national development framework, Vision 2025, commits the country to reducing its maternal mortality ratio (MMR) to less than 40 maternal deaths per 100,000 live births. Pakistan's current MMR is far higher (178 deaths per 100,000 live births), although this marks a considerable improvement from the 276 deaths per 100,000 live births recorded in PDHS 2006–07.⁹⁴ Most maternal deaths in Pakistan are preventable. Post-partum haemorrhage is the foremost cause of MMR in the country, accounting for 47.76 percent of deaths, followed by pregnancy induced hypertension, which causes 25.37 percent of deaths. An increased risk of maternal mortality is evident among women between 25 and 29 years old (35.1 percent), followed by women aged 20 to 24 (26.11 percent) and women over the age of 30 (23.88 percent).⁹⁵ In developed countries, thromboembolism is responsible for 15 percent of maternal deaths; however, exact data on this cause of maternal mortality is not available for developing countries, including Pakistan.

According to the PDHS 2017–18, the number of women who received antenatal care from a skilled provider (doctor, nurse, midwife, or lady health visitor) has increased to 86.2 percent – that is, nine in 10 women aged 15 to 49 – up from 73 percent in 2012–13. However, antenatal care remains very low in Balochistan (55.5 percent). Punjab has the highest rate of antenatal care in the country (92.3 percent), with more pregnant women seeking health care in urban areas (96.1 percent) than in rural settings (90.4 percent). More than half of the women in Pakistan (55 percent) receive pre-natal care in the first trimester of their pregnancy. The proportion of deliveries assisted by skilled birth attendance rose to 69 percent in 2017–18, while deliveries in health care facilities increased to 66.2 percent, up from 48 percent in 2012–13.

Women of reproductive age in Pakistan bear a double burden of malnutrition. One in every seven women (14 percent) is undernourished a decline from 18 percent since 2011, while overweight and obesity are increasing as over 37 percent of women were reported to be overweight or obese in 2018 compared to 28 percent in 2011. Urban-rural disparities are glaringly in this context, as considerably more rural women are underweight (15.8 percent) than their urban counterparts (12 percent). Women in rural areas are more malnourished, while overweight and obesity are higher in urban women.⁹⁶ Regional disparities also abound, with malnourishment among women most pronounced in Sindh (22.6 percent), Balochistan (14.5 percent) and Azad Jammu and Kashmir (12.9 percent). Micronutrient deficiencies among women of reproductive age are a leading cause of such deficiencies in children, resulting in stunting, wasting and children being underweight. The NNS 2018 found that 41.7 percent of women of reproductive age in Pakistan are anaemic, down from 48.9 percent in 2011. Once again, more women in rural areas are anaemic (44.3 percent) than in urban centres (40.2 percent).

⁹⁴ World Bank, 'Pakistan Maternal Mortality Rate 1990-2020', World Bank, Washington D.C., <<https://www.macrotrends.net/countries/PAK/pakistan/maternal-mortality-rate>>, accessed 4 May 2020.

⁹⁵ Rafiq, Sonia, Wajeeha Syed and Simi Fayyaz Ghaffar, 'Trends and causes of maternal mortality in a tertiary care hospital over five years: 2013-2017', *Pakistan Journal of Medical Sciences*, vol. 35, no. 4, 2019, pp. 1128–1131, <<https://www.ncbi.nlm.nih.gov/pmc/articles/PMC6659062/>>, accessed 4 May 2020.

⁹⁶ Nutrition Wing, Ministry of National Health Services, Regulation and Coordination, United Nations Children's Fund, Aga Khan University, and UK Aid, *National Nutrition Survey 2018*, Government of Pakistan, Islamabad, 2019.

The PDHS 2017–18 found that only 59 percent of women take iron tablets or syrup during pregnancy. Iron deficiency anaemia is most pronounced in Sindh (23.8 percent), followed by Balochistan (19 percent). Overall, country level data suggests that 18.2 percent of women of reproductive age in Pakistan suffer from iron deficiency anaemia.

Inequity, poverty, a lack of access to health care facilities, unskilled birth attendants and unskilled midwives are among the most important determining factors contributing towards preventable maternal deaths. Early marriage, high fertility, inadequate birth spacing and illiteracy are other factors that underlie Pakistan's high rates of maternal mortality. A study⁹⁷ on the causes of the high MMR, especially in Khyber Pakhtunkhwa and Balochistan, highlights two key areas in dire need of improvement. First, staffing patterns at peripheral facilities (the number of doctors available) and, second, the accessibility of essential obstetric care (the distance from a woman's area of residence to a health facility and the availability of emergency obstetric care facilities). Financial constraints, sociocultural barriers and inadequate maternal health facilities are also key reasons that Pakistan remains far from achieving the SDG targets on reducing maternal mortality.

MATERNAL HEALTH CARE, FERTILITY RATES AND FAMILY PLANNING

Extremely high fertility rates directly impact child and maternal mortality rates, as well as the nutritional status of women and children. Pakistan's total fertility rates have dropped since 1990–91 – from 4.9 children per woman to 3.6, a decline of 1.3 children per woman. However, fertility rates are not considerably different at present than in 2012–13 (3.8 children per woman). Factors that influence these levels include area of residence, region or province, household wealth, and a woman's level of education. Women in urban areas have an average of 2.9 children, compared to an average 3.9 children per woman in rural settings. Regional variations are also apparent, from as low as 3.0 children per woman in Islamabad Capital Territory, to as high as 4.8 children per woman in the Merged Districts of Khyber Pakhtunkhwa. Household wealth and income also has a direct relation with the total fertility rates, ranging from 4.9 children per woman among lowest wealth quintile to 2.8 children among women from the highest wealth quintile.

Poor quality of care in the perinatal period contributes to high rates of still birth, neonatal deaths and maternal deaths. Quality of care has been highlighted in the 10-point agenda of Pakistan's National Vision 2016-2025, demonstrating the Government's commitment to accelerating improvement in new-born, child and maternal survival, with a focus on reducing morbidity and mortality linked to common preventable causes.

⁹⁷ Agha, Sohail, 'A profile of women at the highest risk of maternal death in Pakistan', *Health Policy and Planning*, vol. 30, no. 7, 2015, pp. 830–836, <<https://doi.org/10.1093/heapol/czu066>> <<https://academic.oup.com/heapol/article/30/7/830/825238>>, accessed 4 May 2020.

LEGISLATIVE AND POLICY ANALYSIS

Since 2017, several policy and legislative frameworks have been developed to enable children in Pakistan to survive and thrive:

- Vision 2025 highlights nutrition as a key issue of concern. In response, the Pakistan Multi-Sectoral Nutrition Strategy (PMNS) was developed in 2018, in order to create new opportunities and change the way nutrition is approached at the national, provincial and local levels.⁹⁸ The strategy aims to guide nutrition-sensitive and nutrition-specific intervention to address the immense challenge of malnutrition among women and children in Pakistan. The strategy recommends adopting a 'nutrition lens' as a necessary component of annual programme reviews, planning and budgeting across all sectoral development schemes, and all of Pakistan's Annual Development Programmes.
- Pakistan's Infant and Young Child Feeding Strategy (IYCF 2016–2020) provides a framework of action to protect, promote and support optimal infant and young child feeding practices across the country. A provincial IYCF Strategy 2018–22 was launched in Balochistan to improve the nutritional status, growth, development and survival of infants and children. In response to the alarmingly high malnutrition rates in the province, the strategy aims to foster effective nutrition-related initiatives across Balochistan.
- In the federal capital, the Islamabad Compulsory Vaccination and Protection of Health Workers Act 2019 was passed, as was the Vaccination (amended) Act, 2017 in Khyber Pakhtunkhwa. The latter gives the superintendent of vaccination jurisdiction to issue a notice to persons refusing to vaccinate their children.
- Policy level documents in place now include Newborn Strategy and Action Plan for all Provinces, Standard of Quality of Care and Kangaroo Mother Care Protocol.
- The Comprehensive Multi-Year National Immunization Strategic Plan (2011–15) was extended to cover the 2014–2018 period, in order to provide essential, safe and effective immunization to all eligible children and women in their areas of residence.
- The government approved National Emergency Action Plan for Polio Eradication 2018-19 that calls for improved coordination & collaboration mechanism with Government's flagship Ehsaas (literally 'compassion') programme, WASH, Nutrition and Health programs (like Polio Plus) for support from Polio program in implementation, monitoring and reporting of delivery of integrated interventions in Super High-Risk Union Council (SHRUCs).

HEALTH SERVICES AND DELIVERY SYSTEM

Key issues facing Pakistan's health care delivery system include high population growth, gaps in health infrastructure, insufficient funding and budgetary allocations, the uneven distribution of health care professionals, and limited access to quality health services. Challenges facing the health delivery system also include limited institutional capacity – particularly among MNCH

⁹⁸ Nutrition Wing, Ministry of National Health Services, Regulation and Coordination, United Nations Children's Fund, Aga Khan University, and UK Aid, *National Nutrition Survey 2018*, Government of Pakistan, Islamabad, 2019.

Units in Provincial Health Departments – a lack of transparency and accountability and limited coordination between federal and provincial Departments of Health. Capacity challenges also encompass a lack of qualified staff in neonatology, including surgery and nursing staff, in district hospitals with variations across provinces, and the absence of organized pre-service and in-service training programmes for nurses and physicians on clinical care and communications. A weak information system impedes efforts to capture important information across district and tertiary health care settings. In tandem, the continuity of care is hampered by the absence of functional referrals between primary, secondary and tertiary health facilities.

Millions of women in Pakistan do not receive the maternal and newborn care they need to prevent and manage health complications that may arise during pregnancy, delivery and the postpartum period. Providing a comprehensive package of maternal and newborn care – that includes antenatal, delivery, postpartum and post-abortion services – is costly and can pose a challenge for low income families. The unmet need for modern contraception is considerable in every province and region of Pakistan. It is highest in Balochistan and Khyber Pakhtunkhwa's Merged Districts, where about two-thirds of married women's needs for contraceptives are not met, and lowest in Islamabad Capital Territory, where this proportion is nearly half of married women.

Pakistan continues to have pockets of low health coverage in densely populated, impoverished urban areas. In geographically remote and underserved areas, alongside urban slums, pockets persist where children miss out on routine immunization services. These high-risk areas hold back improvements in the country's overall immunization coverage rate. To overcome these challenges, as discussed above, it is important to allocate and incentivize human resources, to train and mobilize community health workers and LHWs within communities, and to undertake awareness raising programmes to mitigate social taboos surrounding vaccination. The lack of adolescent-friendly sexual and reproductive health services at the primary health care level hampers improved health-seeking behaviour and prevents adolescents from making informed choices about their well-being. Other forms of health care for adolescents, including those with chronic illnesses and disability needing medical care need to be put in place. Legislative and policy provision for issues of consent, parental consent and adolescent autonomy needs to be addressed.

Several policy frameworks and programmes are relevant for health services and their delivery system, including:

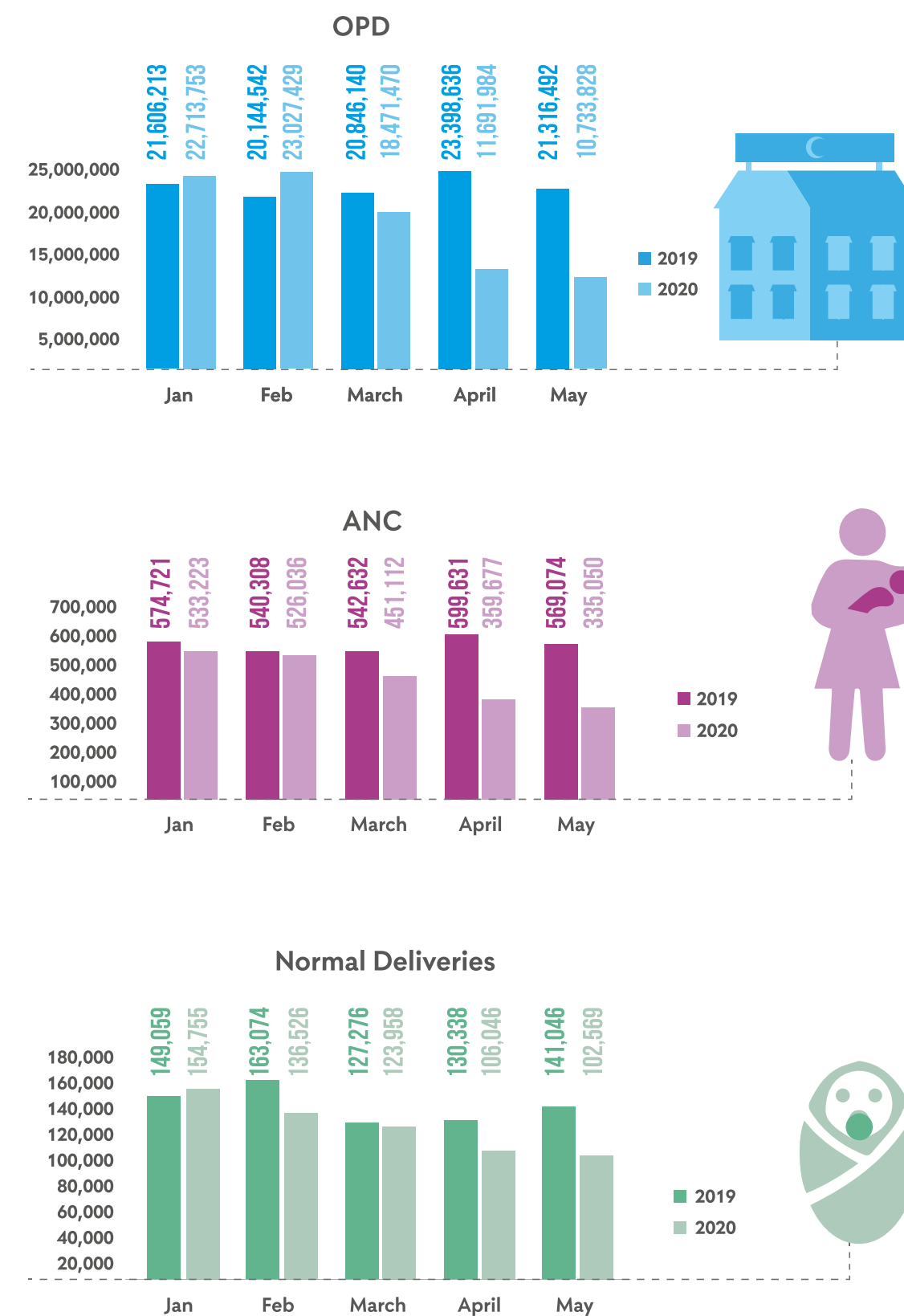
- A National Action Plan (2019-2023) was launched to advance Universal Health Coverage – an important contributor to sustainable development – as well as to address health emergencies and disease outbreaks, while strengthening the effectiveness and efficiency of organizations to better support the health system.

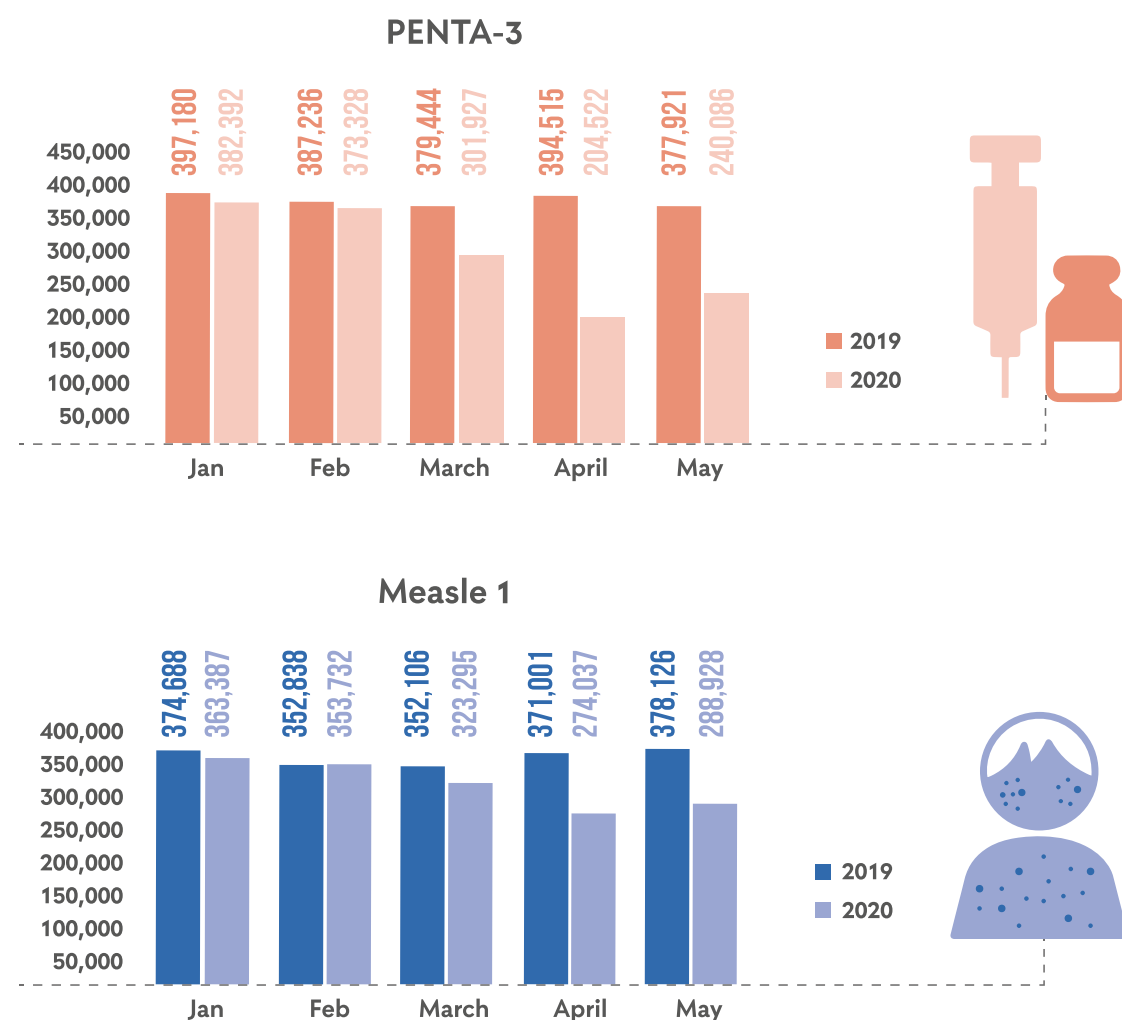
- The Government launched the Sehat Sahulat Programme to overcome equity issues in access to quality health care. Its overarching objective is to lead towards Universal Health Coverage (UHC) in Pakistan, with a special focus on those living below the poverty line. The programme is being implemented in a phased manner, with its first phase implemented in 38 districts covering 3.2 million families.
- The Lady Health Worker (LHW) Programme plays a central role in reproductive, maternal, newborn, and child health policy in Pakistan. However, the extent to which it addresses the needs of marginalized and vulnerable women and children is compromised across all regions by: (i) the lack of an explicit focus on geographical areas and socio-economic groups with the greatest needs; (ii) an increasing focus on immunization relative to other health, education and nutrition needs; and (iii) management and resourcing issues. The selection of areas is currently found to be determined by proximity to where LHWs have already operated, where qualified LHWs can be recruited, or where access is relatively easy. There is also a decline in the number of LHWs in almost all parts of Pakistan, decreasing coverage in some regions. To date, no region has met its population coverage target. Key challenges include: a freeze on recruitment; increased LHW responsibilities beyond core functions, in particular for polio programming; considerable funding deficits, which have created shortages of supplies and equipment; and a reduction in the regularity of training received by LHWs.

IMPACT OF COVID-19 ON THE HEALTH AND NUTRITIONAL STATUS OF CHILDREN AND WOMEN

COVID-19 is expected to impact children and women in multiple ways, threatening their survival, health and nutritional status. COVID-19 is likely to reduce the uptake and provision of essential health care services. This is exacerbated by people's fear of seeking services during a pandemic, a decreased health workforce able to deliver essential services, and the unmet need for protective personal equipment needed to keep health workers safe. As the sheer numbers of COVID-19 patients inundate health services, children and pregnant women are less able to access essential reproductive, maternal, newborn and child health services, such as antenatal care, skilled birth attendants, and treatment for acute respiratory infections, diarrhoea and fever. Due to lockdowns, immunization services have experienced severe disruptions, causing major setbacks for polio vaccination campaigns that had been gaining traction, and increasing the likelihood of the continued spread of polioviruses.

Figure 6: Impact of COVID-19 on essential health services





Source: Ministry of National Health Services, Regulations and Coordination. HMIS Data as of July 2020

A drop of 50 percent was witnessed in OPD consultations in May 2020 when compared to May 2019. Similarly, considerable reduction in ANC visits, normal deliveries, and Penta 3 and measles 1 vaccination were recorded on account of the country wide lockdown.

Health workers are particularly vulnerable as they are consistently and repeatedly exposed to the virus. As of July 2020, more than 5,000 health workers were infected and 58 healthcare providers had lost their lives to COVID-19.⁹⁹

Disruption in nutrition and WASH services increases the risk of waterborne diseases and may reverse hard-won gains achieved to meet SDG targets. Milestones achieved in NMR, IMR and U5MR are likely to experience a reversal due to pressure on health systems, and hundreds of

⁹⁹ Government of Pakistan and media reports. <https://www.thenews.com.pk/print/680655-pakistan-has-lost-42-doctors-among-58-healthcare-providers-to-covid-19>. <https://www.amnesty.org/download/Documents/ASA3326332020ENGLISH.pdf>

thousands of additional child deaths could occur in 2020 compared to pre-pandemic years.¹⁰⁰ High rates of child deaths from malnutrition are expected to rise as a result of the pandemic, with poor households, people in geographically remote and crisis-affected areas, and vulnerable groups bearing the brunt of this burden. Access to an adequate diet is being curtailed for those at high risk due to multiple factors, ranging from limited access (financial constraints, unemployment, and the loss of steady income) to disruption in supply chains. Interruptions in food and nutrition services have already negatively impacted groups¹⁰¹ with the greatest nutrition needs – especially children under-five, pregnant and lactating women, and the elderly. The sharp rise in poverty expected in the wake of the pandemic (see chapter 2) is likely to exacerbate marked disparities that are already clearly visible between wealth quintiles in terms of health service coverage. While Pakistan's lowest wealth quintiles spend 6.6 percent of their household income on health, households in the highest wealth quintiles spend 1.3 percent.

If the COVID-19 pandemic results in continued and widespread disruption to health systems and reduced access to food, low- and middle-income countries, including Pakistan, can expect to see large increases in maternal and child deaths.¹⁰² Therefore, it is important to address the broader effects of the pandemic on child and maternal health, in addition to the direct impacts of COVID-19. A broader public health response is needed, in the form of RCCE, IPC and PHC acceleration for essential health services – including immunization and MNCH service delivery – to save vulnerable children and women. The double burden of COVID-19 on health issues directly caused by the virus, and the simultaneous disruption of essential health services, need to be addressed. The ability of Pakistan's already fragile health system to maintain essential health services will depend on its baseline capacity and burden of disease, coupled with the increased burden of COVID-19 cases.

Response: The Ministry of National Health Services, Regulations and Coordination was quick to respond to the COVID-19 outbreak and developed a *National Action Plan for Preparedness and Response to Coronavirus Disease* (NAP) on 12 February, 2020. This was further refined as a policy document on 13 March, 2020, to ensure Pakistan's preparedness, containment and mitigation of the threat posed by COVID-19. The Ministry aimed to develop a National Preparedness and Response Plan for COVID-19, based on the framework of Pandemic Preparedness for Pakistan, under the Global Health Security Agenda. It also sought to provide a policy framework for federal, provincial and regional stakeholders so as to build their capacities to respond to the pandemic. The main objectives of the NAP are to:¹⁰³ (i) contain and respond to the outbreak in a timely

¹⁰⁰ United Nations, *Policy Brief: The Impact of COVID-19 on Children*, UN, New York, 15 April 2020, <https://www.un.org/sites/un2.un.org/files/policy_brief_on_covid_impact_on_children_16_april_2020.pdf>, accessed 4 May 2020.

¹⁰¹ High Level Panel of Experts on Food Security and Nutrition, *Impact of COVID-19 on Food Security and Nutrition*, Committee on World Food Security (CFS) and HLPE, Rome, 19 March 2020, <http://www.fao.org/fileadmin/templates/cfs/Docs1920/Chair/HLPE_English.pdf>, accessed 4 May 2020.

¹⁰² Lancet Global Health, 'Early estimates of the indirect effects of the COVID-19 pandemic on maternal and child mortality in low-income and middle-income countries: a modelling study', 2020, <[https://doi.org/10.1016/S2214-109X\(20\)30229-1](https://doi.org/10.1016/S2214-109X(20)30229-1)>, accessed 12 May 2020.

¹⁰³ Ministry of National Health Services, Regulation, and Coordination, *National Action Plan for Coronavirus disease (COVID-19) Pakistan*, Government of Pakistan, Islamabad, 2020.

and efficient manner; (ii) prioritize financial resources and increase domestic and international investment for the country's emergency preparedness; and (iii) implement emergency preparedness actions by strengthening intersectoral collaboration between government sectors, the private sector and civil society at the provincial level. The plan covers planning and coordination mechanisms, laboratory support, food security, logistics, communication, infection prevention and control at points of entry (POE) and health facilities, human resource training and management, surveillance, quarantine and isolation preparedness, reducing community exposure, and monitoring and evaluation.

In response to the pandemic, WHO announced its Strategic and Response Plan, outlining the public health measures that the international community stands ready to provide to support all of the world's countries to prepare for, and respond to, COVID19. The global response to the pandemic requires both public health and multi-sectoral socioeconomic responses, involving measures to ensure the continuity of existing essential health services, maintain supply chains, and address the impact of the disease on the most vulnerable – such as women and children – through social protection and other initiatives.¹⁰⁴ UN's response to the COVID-19 pandemic centres on designing programmes to sustain women- and child-centric services, as well as to mitigate the negative impacts of the pandemic on women and children. Its policy guidelines and programmes aim to raise the awareness and enhance the capacities of policy-makers to secure the rights and wellbeing of all women and children.

The UN Development System (UNDS) has mobilized around the movement for universal health coverage (UHC), primary health care (PHC), and the SDG Global Action Plan for Healthy Lives and Well-being for All (SDG 3). UNDS plans to bring these efforts to the foreground using the infrastructure developed to ensure the provision of essential services to vulnerable populations. This will include covering immunization efforts (including polio eradication measures), in addition to strengthening health systems while the pandemic rages worldwide. UNDS also plans to provide guidance on maintaining essential preventive outreach services.¹⁰⁵

As discussed above (see chapter 2), the World Bank's approved Pandemic Response Effectiveness Project – worth US\$200 million – seeks to support Pakistan to take effective and timely action to respond to COVID-19 by strengthening the country's health care systems to prevent, detect and respond to the pandemic. In tandem, it will be used to mitigate socioeconomic disruption¹⁰⁶ at the federal and provincial levels. The project, grounded on 'One Health',¹⁰⁷ aims to provide an integrated approach across sectors and disciplines. It will also draw an extra US\$38 million from

¹⁰⁴ United Nations, *Shared Responsibility, Global Solidarity: Responding to the socio-economic impacts of COVID-19. Report of the Secretary-General*, United Nations New York, March 2019, <<https://unsdg.un.org/sites/default/files/2020-03/SG-Report-Socio-Economic-Impact-of-Covid19.pdf>>, accessed 4 May 2020.

¹⁰⁵ United Nations, *Shared Responsibility, Global Solidarity: Responding to the socio-economic impacts of COVID-19. Report of the Secretary-General*, United Nations New York, 3 April 2020 (draft).

¹⁰⁶ World Bank Report #: PAD3826

¹⁰⁷ 'One Health' is an approach to designing and implementing programmes, policies, legislation and research in which multiple sectors communicate and work together to achieve better public health outcomes. For more information, see: <www.who.int/features/qa/one-health/en>

eight existing projects for urgently needed medical equipment and supplies. Moreover, it will support the poor and vulnerable to cope with the immediate impact of the pandemic through social protection measures, food rations and remote learning programmes, alongside establishing quarantine facilities with public and private sector hospitals.

MoNHSRC launched 'We CARE', a national campaign targeting over 100,000 frontline health workers with the aim to protect and support health workers with the aim to provide adequate Personal Protective Equipment (PPE) to health workers, orienting them on usage of various PPE items as per international standards, and creating an overall psycho-social environment of care and support.

RECOMMENDED RESPONSE

Now, there is a need to focus on three priority areas for health in the context of COVID-19:

1. The public health response to COVID-19;
2. The continuity of essential health, immunization and nutrition services; and
3. Making the existing health care system adaptive to COVID-19 ensuring that younger children and adolescents are heard on health care delivery and continuing the focus on child-friendly health services.

In the context of the COVID-19 pandemic, it is imperative that Pakistan prioritizes maternal and child-focused services, most importantly essential newborn and maternal health care services, immunization programs, nutrition programmes and WASH services. Synergies are especially – between governments, international organizations, donors, non-governmental organizations (NGOs), civil society and the private sector. It is vital that all of these partners work together to analyse and finance urgent measures to provide relief, especially to the most marginalized, the poorest, and those at high risk of the social and economic impacts of COVID-19.

SHORT-TERM RECOMMENDATIONS:

- Prioritizing newborn health and stillbirth reduction through multiple approaches – addressing health system issues, HR, quality of services and community based newborn programme, collaborate with other agencies for service utilization.
- Emphasize the Public Health Response to COVID-19 with focus on risk communication and community engagement, IPC, protection of health workers and continuation of essential MNCH, Immunization and HIV services.
- Undertake a rapid, yet comprehensive, mapping exercise to identify those who are most at risk of being left behind in Pakistan. Prioritize the most vulnerable groups with critical age and gender lenses, and formulate mechanisms during and after the pandemic which incorporate provisions on the right to food,¹⁰⁸ both in terms of quantity and nutritional quality.

¹⁰⁸ High Level Panel of Experts on Food Security and Nutrition, *Impact of COVID-19 on Food Security and Nutrition*, Committee on World Food Security (CFS) and HLPE, Rome, 19 March 2020, <http://www.fao.org/fileadmin/templates/cfs/Docs1920/Chair/HLPE_English.pdf>, accessed 4

- Include the analysis of quantitative data on pre- and intra-COVID-19 trends on vaccination rates, antenatal care visits, hospital deliveries, and other key indicators in secondary data collection methods. Strive to undertake primary mixed-method data collection through interviews and/or surveys with health care staff, the caregivers of children under-five, and pregnant and lactating women.
- Ensure the continuation of essential health services for the treatment of severe acute malnutrition (SAM), as well as advocacy and the dissemination of information on infant and young child feeding (IYCF) practices through print and electronic media.
- Prioritize routine immunization of children in essential service delivery.
- Continue to deliver all immunization services, as this is vital for saving lives that would otherwise be lost to vaccine-preventable diseases. Implement effective communication strategies to engage communities and maintain strong demand for vaccination remains strong. Ensure that immunization services must consider the importance of protecting children against preventable diseases, while simultaneously guaranteeing the safety of health care workers.
- Deliver an integrated package of services using a system strengthening approach for primary health care. It is important to be fully aware of the impact on the epidemiology of wild poliovirus and vaccine-derived polioviruses, as well as of other vaccine-preventable diseases. To this end, the programme should work towards integrated campaign approaches, especially for measles and vitamin A supplementation.
- Use telemedicine as a possible solution to address the health needs of women of reproductive age, and pregnant and lactating women, including the provision of online medical consultations, wherever possible.

MEDIUM-TERM RECOMMENDATIONS:

- Ensure universal health coverage to make health care accessible, affordable and available to all.
- Implement and monitor the nutrition sector's COVID-19 Response Plan in consultation with federal and provincial health departments, international organizations, donors and NGO partners.
- Provide fixed-site immunization services to communities where health system capacities are intact and essential health services are operational. Ensure constant surveillance of outbreaks of vaccine-preventable diseases. If an outbreak occurs, undertake a mass vaccination campaign after a risk-benefit assessment.
- Establish a network of maternal and neonatal child health hospitals.
- Support national efforts to ensure food security for all of the people in Pakistan, including in terms of economic access, availability and the use of nutritious diets and supplements.

- Work with partners for evidence based advocacy and increasing the health budget with prioritization to PHC services.
- Expanding delivery of quality services at community level.
- Prioritize climate change and environmental health within the domain of public health system with effective cross sectoral collaboration.
- Develop investment plan and contribute for UHC implementation within the framework of Primary Health Care Acceleration.

EVERY CHILD LEARNS

Children of school-going age (5 to 14 years old) represent over one-quarter (27.14 percent) of Pakistan's overall population.¹⁰⁹ While steady improvements are apparent in key education indicators, 22.84 million children are still out of school, including 12.16 million girls. Among children between 5 and 9 years old, 5.06 million children are out of school. The same is true for 11.5 million adolescents between the ages of 10 and 14.¹¹⁰ Literacy rates in Pakistan remain low, at 72 percent, a slight improvement from 71 percent in 2013–14.¹¹¹ Pakistan faces a 'learning crisis' with 75 percent of children of late primary school age not proficient in reading, and 65 percent not being able to achieve minimum proficiency levels (MPL) by the end of their primary education.¹¹²

Considerable gains have been made by reform programmes across Pakistan's provinces, including notable improvements in the gross enrolment rates (GER) for pre-primary/early childhood education (ECE), learning outcomes,¹¹³ and relatively high investments in primary education. Despite this, increasing efforts by provincial governments since 2010 to expand access to education and improve core education indicators have not yielded the desired results. Challenges persist, impeding accessible and high-quality school education for all of the children in Pakistan. These include the need to: do more to get children into schools at pre-primary and primary level; focus on retention rates beyond primary schooling; ensure sustainable financing of the school reform agenda; significantly improve education quality and learning outcomes in both public and private schools through systemically addressing issues in learning pathways, textbooks, teaching and learning in classrooms and overall management of quality education; maintain and upgrade

109 Pakistan Bureau of Statistics, 'Percentage Distribution of the Population by Age, Sex and Area', *Labour Force Survey 2017–18*, Government of Pakistan, Islamabad, 2018, <[http://www.pbs.gov.pk/sites/default/files/Labourper cent20Force/publications/lfs2017_18/TABLE_1_perc_R.pdf](http://www.pbs.gov.pk/sites/default/files/Labourper%20Force/publications/lfs2017_18/TABLE_1_perc_R.pdf)>, accessed 4 May 2020.

110 National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, *Pakistan Education Statistics, 2016–17*, Government of Pakistan, Islamabad, 2018, <<http://library.aepam.edu.pk/Books/Pakistan%20Education%20Statistics%202016-17.pdf>>, accessed 4 May 2020.

111 Pakistan Bureau of Statistics, *Pakistan Social & Living Standards Measurement Survey 2018-19*, Ministry of Planning Development & Special Initiatives, Government of Pakistan, Islamabad, <http://www.pbs.gov.pk/sites/default/files/plsm/publications/plsm_hies_2018_19_provincial/key_findings_report_of_plsm_hies_2018_19.pdf> accessed 22 June 2020.

112 World Bank, *Pakistan Learning Poverty Brief: EduAnalytics*, World Bank, Washington D.C., 2019.

113 National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, *Pakistan Education Statistics, 2016–17*, Government of Pakistan, Islamabad, 2018, <<http://library.aepam.edu.pk/Books/Pakistan%20Education%20Statistics%202016-17.pdf>>, accessed 4 May 2020; ASER Pakistan, *Annual Status of Education Report: ASER-Pakistan 2016*, South Asian Forum for Education Development (SAFED), Lahore, 2017, <http://www.aserpakistan.org/documents/Report_Final_2016.pdf>, accessed 4 May 2020; ASER Pakistan, *Annual Status of Education Report: ASER-Pakistan 2019*, ASER Pakistan Secretariat, Lahore, 2020, <http://aserpakistan.org/document/aser/2019/reports/national/ASER_National_2019.pdf>, accessed 4 May 2020.

a robust data collection and evaluation regime; and revisit the overall priorities of education reform to pay more attention to middle and secondary schooling, as well as to achieve greater coherence and clarity on the role of public schools in the education sector. For Pakistan to achieve universal pre-primary, primary, and secondary education access by 2030, the total number of teachers will need to expand from 1.2 million to almost 1.7 million and increase in spending and sustained higher domestic allocations to the education sector with the greatest investments needed at the secondary and post-secondary levels.¹¹⁴

Pakistan's current education outcomes, discussed below, are insufficient to support the country's economic and social development. They are equally insufficient the SDG 4 targets – ensuring that all children have access to quality early childhood development, ensuring that all children have complete free, equitable and quality primary and secondary education, and to eliminate gender disparities in education by 2030.

OUT-OF-SCHOOL CHILDREN

A worrying statistic for a country, whose current workforce is young, mostly unskilled, and poorly prepared for productive employment,¹¹⁵ is the 22.84 million out of school children aged 5 to 16 as of 2016-17. While 5.06 million children of primary school age are out of school, the number of OOSC drastically increases among the age group of 10 to 16-year-olds – rising over threefold to 17.8 million OOSC (6.51 million among age group 10-12 years and 11.26



¹¹⁴ UNICEF Education Team and

¹¹⁵ World Bank, *Tracing the Flow of Public Money, Punjab: Expenditure and Quantity of Service Delivery Survey in Primary School Sector*, World Bank, Washington D.C., 2015.

million among age group of 13-16 years).¹¹⁶ Recently published Pakistan Social & Living Standards Measurement Survey 2018-19 state that 30 percent of children aged 5 to 16 are out of school in Pakistan compared to 33 percent in 2013-14.¹¹⁷ Pakistan has the second highest number of out of school children in the world. Regionally, Balochistan has the highest percentage of children between the ages of 5 and 16 who are not in school, followed by Sindh, Khyber Pakhtunkhwa and Punjab (see Annex).

More girls are out of school than boys – 12.16 million girls, compared to 10.68 million boys from the primary to the higher secondary level.¹¹⁸ OOSC include children who have never attended school, as well as children who have dropped out. Girls are more likely to drop out than boys, with 21.8 percent of girls dropping out of school in Class 6, compared to 18.4 percent of boys. Average dropout rates are lowest in early primary grades (0.2 percent in Class 1, 0.6 percent in Class 2, and 2 percent in Class 3) but increase considerably once children complete their primary education –

¹¹⁶ National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, *Pakistan Education Statistics, 2015-16*, Government of Pakistan <<http://library.aepam.edu.pk/Books/Pakistan%20Education%20Statistics%202015-16.pdf>>, accessed 4 May 2020.

¹¹⁷ Pakistan Bureau of Statistics, *Pakistan Social & Living Standards Measurement Survey 2018-19*, Ministry of Planning Development & Special Initiatives, Government of Pakistan, Islamabad, <http://www.pbs.gov.pk/sites/default/files/pplm/publications/pplm_hies_2018_19_provincial/key_findings_report_of_pplm_hies_2018_19.pdf> accessed 22 June 2020.

¹¹⁸ National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, *Pakistan Education Statistics, 2016-17*, Government of Pakistan, Islamabad, 2018, <<http://library.aepam.edu.pk/Books/Pakistan%20Education%20Statistics%202016-17.pdf>>, accessed 4 May 2020; National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, *Pakistan Education Statistics, 2015-16*, NEMIS, AEPAM, Government of Pakistan, <<http://library.aepam.edu.pk/Books/Pakistan%20Education%20Statistics%202015-16.pdf>>, accessed 4 May 2020.

7.7 percent and 19.8 percent in Class 6.¹¹⁹ More children in rural areas are likely to be out of school compared to their urban counterparts. Dropout rate is very high at 26 percent in the transition between *katchi* and grade-1.¹²⁰

MICS data from 2017¹²¹ reveals that 13 percent of children of primary school age in Punjab are not attending pre-primary, primary or lower secondary school, compared to 26.7 percent in Khyber Pakhtunkhwa and 23.8 percent in Gilgit-Baltistan. The percentage is even higher for adolescents of lower secondary school age who are out of school and not attending primary, secondary or higher education: 20 percent in Punjab, 34.4 percent in Khyber Pakhtunkhwa and 18.7 percent in Gilgit-Baltistan. Lack of access to schools is due to inadequate supply of schools as well as limited demand for education due to social norms that put less value on education especially for girls, particularly in rural areas, which also partly contributes to the phenomenon of out of school children.

EARLY CHILDHOOD EDUCATION (ECE)/PRE-PRIMARY EDUCATION

Pre-primary level in Pakistan largely consist of *katchi* classes which are not necessarily quality ECE classrooms. In Pakistan, many children are enrolled at pre-primary levels, but few schools offer an ECE program. Most of these children do not have any specific resources allocated to them e.g. teacher, room or supplies. Although this is an informal space, in practice it serves as an entry class for public sector primary schools. Reliable data does not exist on ECE and a large number of children reported as pre-primary students are actually in *katchi* class. The pre-primary or *katchi* enrolment levels of boys and girls increased by 8 percent and 7 percent, respectively, compared to 2015–16. Overall, 9.78 million children in Pakistan are enrolled in pre-primary, 46 percent of whom are girls.¹²² This marks an improvement from 8.7 million children enrolled in 2015. The gross enrolment ratio (GER) at the pre-primary level increased to 84 percent, compared to 74 percent in 2015–16. However, the gender parity index (GPI) at the pre-primary level is 0.87, as girls' GER is only 78 percent, compared to 89 percent for boys.

Trends in Punjab reveal that pre-primary attendance is declining. While 34.4 percent of children under-five attended pre-primary classes in 2018, this was a decline from 37.3 percent in 2014.¹²³ Disparities between urban and rural regions, and between boys and girls do not appear

119 Pakistan Bureau of Statistics, *Pakistan Social and Living Standards Measurement Survey 2013–14*, Government of Pakistan, Islamabad, 2015.

120 Calculations based on data from National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, *Pakistan Education Statistics, 2016–17*, Government of Pakistan, Islamabad, 2018, <<http://library.aepam.edu.pk/Books/Pakistan%20Education%20Statistics%202016-17.pdf>>, accessed 4 May 2020.

121 Punjab Bureau of Statistics and United Nations Children's Fund, *Multiple Indicator Cluster Survey 2017-2018, Punjab*, Government of Punjab, Lahore, 2018; Khyber Pakhtunkhwa Bureau of Statistics, Planning & Development Department, and United Nations Children's Fund, *Khyber Pakhtunkhwa Multiple Indicator Cluster Survey 2016-17*, Government of Khyber Pakhtunkhwa, Peshawar, 2017; Gilgit-Baltistan Planning & Development Department, and United Nations Children's Fund, *Gilgit-Baltistan Multiple Indicator Cluster Survey 2016-17*, Government of Gilgit-Baltistan, Gilgit, 2017.

122 National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, *Pakistan Education Statistics, 2016–17*, Government of Pakistan, Islamabad, 2018, <<http://library.aepam.edu.pk/Books/Pakistan%20Education%20Statistics%202016-17.pdf>>, accessed 4 May 2020.

123 Punjab Bureau of Statistics and United Nations Children's Fund, *Multiple Indicator Cluster Survey 2014, Punjab*, Government of Punjab, Lahore, 2014; Punjab Bureau of Statistics and United Nations Children's Fund, *Multiple Indicator Cluster Survey 2017-2018, Punjab*, Government of Punjab, Lahore, 2018.

considerable in terms of pre-primary attendance. However, considerable differences are evident as a result of households' socioeconomic status. Only 20.9 percent of children from the province's poorest households attend any form of pre-primary education, compared to 45.4 percent of children from the richest households.¹²⁴ This rate increases to 63 percent for 5-year-olds at the beginning of the school year attending such classes. As ECE is one of the focus areas of the Punjab School Education Sector Plan 2013–2018, a key step taken was the issuance of an ECE Policy for Punjab. Sindh also launched its Early Childhood Care and Education (ECCE) Policy in 2015, followed by the development of an ECCE Curriculum (2017) and ECCE Standards (2019).

Data from Gilgit-Baltistan and Khyber Pakhtunkhwa¹²⁵ presents a grim picture for ECE and only 14.2 percent and 8.1 percent, respectively, of children under-five attend pre-primary education. Government of Khyber Pakhtunkhwa has recently approved a policy and standards for ECE programs and the government plans to invest by converting 10,000 classrooms into ECE.

PRIMARY EDUCATION (5–9 YEARS) AND MIDDLE AND SECONDARY EDUCATION (10–16 YEARS)

Pakistan's net enrolment rate (NER) at the primary education level (for children aged 5 to 9) is only 57 percent and is marked by considerable disparities based on gender, socioeconomic status and geography. While primary NER for boys is 60 percent, this falls to 53 percent for girls. Similarly, NER in urban areas is 66 percent, compared to 53 percent in rural parts of the country. It is even lower for girls in rural regions – only 48 percent.¹²⁶ The NER represents an often contested metric, partly because it underrepresents the total number of children enrolled in school. The National Education Management Information System (NEMIS) reports the adjusted net primary enrolment ratio (ANER) as the total number of children of primary school age enrolled in primary or secondary education, expressed as a percentage of the corresponding population. The NEMIS data for 2016–17¹²⁷ shows 77.4 percent of all children of primary school age in Pakistan are enrolled in schools, a marginal improvement from 77 percent in 2015–16. The ANER for girls (71.8 percent) is lower than for boys (82.5 percent). Islamabad Capital Territory continues to have the highest ANER in Pakistan of 96.5 percent, followed by Khyber Pakhtunkhwa at 86.9 percent – an improvement from 85 percent in 2015–16. Punjab's ANER of 83.1 percent has decreased from 86 percent in 2015–16. At the middle school level, Pakistan's overall ANER falls to 49 percent (45 percent for girls and 53 percent for boys), before falling to just 31 percent at the secondary school level (27 percent

124 Punjab Bureau of Statistics and United Nations Children's Fund, *Multiple Indicator Cluster Survey 2017-2018, Punjab*, Government of Punjab, Lahore, 2018.

125 Gilgit-Baltistan Planning & Development Department, and United Nations Children's Fund, *Gilgit-Baltistan Multiple Indicator Cluster Survey 2016-17*, Government of Gilgit-Baltistan, Gilgit, 2017; Khyber Pakhtunkhwa Bureau of Statistics, Planning & Development Department, and United Nations Children's Fund, *Khyber Pakhtunkhwa Multiple Indicator Cluster Survey 2016-17*, Government of Khyber Pakhtunkhwa, Peshawar, 2017.

126 Pakistan Bureau of Statistics, *Pakistan Social and Living Standards Measurement Survey 2014–15*, Government of Pakistan, Islamabad, 2016.

127 National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, United Nations Children's Fund and United Nations Educational, Scientific and Cultural Organization, *Pakistan Education Statistics, 2016–17*, NEMIS, AEPAM, Government of Pakistan, UNICEF and UNESCO, Islamabad, 2018, <<http://library.aepam.edu.pk/Books/Pakistan%20Education%20Statistics%202016-17.pdf>>, accessed 4 May 2020.

for girls and 35 percent for boys).¹²⁸ Overall, enrolment rates have stagnated overall and Punjab's performance has worsened on key enrolment indicators.¹²⁹ Reasons for low enrolment in Punjab have been cited in Punjab Education Sector Plan 2019-2024 as supply- side as well as demand side challenges, including distance from school and quality of education, financial constraints and low value placed on education by parents. Often, low and late enrolments and high dropouts in public schools are associated with the perception of low quality of public schooling in Punjab. Data and estimates on OOSC in Punjab are based on the administrative data that is available for the public schools only and the data for the private schools is estimated based on the Private Schools Census which underestimates the number of private schools and related enrolments given that there is a growing number of children who are attending non-registered private or religious schools, not captured in the data.¹³⁰

Pakistan's primary net attendance ratio (NAR) is 58.5 percent (55.4 percent among girls, compared to 61.4 percent for boys),¹³¹ with considerable regional disparities (54.2 percent in rural areas, far below 67.4 percent in urban centres). Primary NAR has declined from 59.9 percent in 2012–13 and 66 percent in 2006. This downward trend is witnessed across all provinces except for Sindh, where primary NAR improved to 55 percent, up from 50 percent in 2012 – although the ratio in 2012 itself represents a decline from 54 percent in 2006. Punjab's primary NAR is marked by a notable declining trend, falling to 64 percent from 67 percent in 2012 and 75 percent in 2006. Khyber Pakhtunkhwa's NAR remained at 57 percent in recent years, but declined from 62 percent in 2006. Balochistan has the country's lowest primary NAR of 39 percent, down from 42 percent in 2012 and 2006. Islamabad Capital Territory's primary NAR is the highest in the country (74 percent), followed by Azad Jammu and Kashmir (71 percent). While Gilgit-Baltistan's NAR is 58 percent, the NAR in the Merged Districts of Khyber Pakhtunkhwa (40 percent) is nearly as low as in Balochistan. Currently, there are 32,272 *Deeni Madaris* (formal madrasa schools)¹³² in Pakistan, with an estimated enrolment of 2.18 million (64 percent male and 36 percent female students). An overwhelming majority of enrolment in *Deeni Madaris* are in the private sector (estimated 97 percent) suggesting that these *Madaris* are generally sponsored and managed by local communities.¹³³

¹²⁸ National Education Management Information System, *Pakistan Education Statistics 2016–17*, Government of Pakistan, Islamabad, 2017.

¹²⁹ Figures include formal, non-formal Education and Deeni Madaris (formal madrasa schools).

¹³⁰ <https://peri.punjab.gov.pk/system/files/Research%20Brief%20-%20Abridged.pdf>

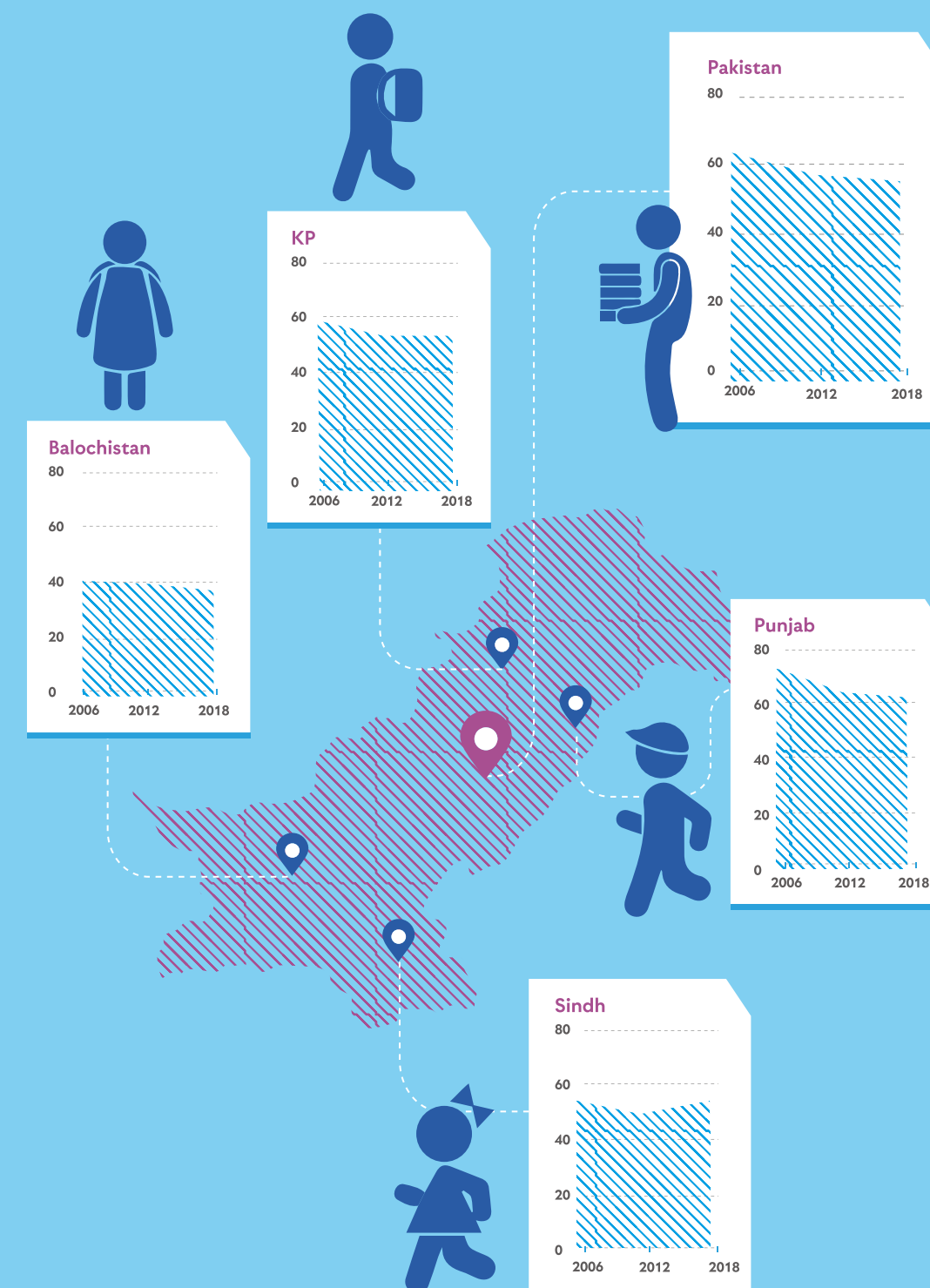
¹³¹ National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

¹³² Deeni Madaris are educational institutions in which formal religious education is provided. The degree provided by these institutions is equal to different levels of education provided by formal institutions.

¹³³ Deeni Madaris data is estimated based on past trend as stated in Pakistan Education Statistics 2016–17.



Figure 7: Primary school net attendance ratio (NAR) (2006–2018) - %

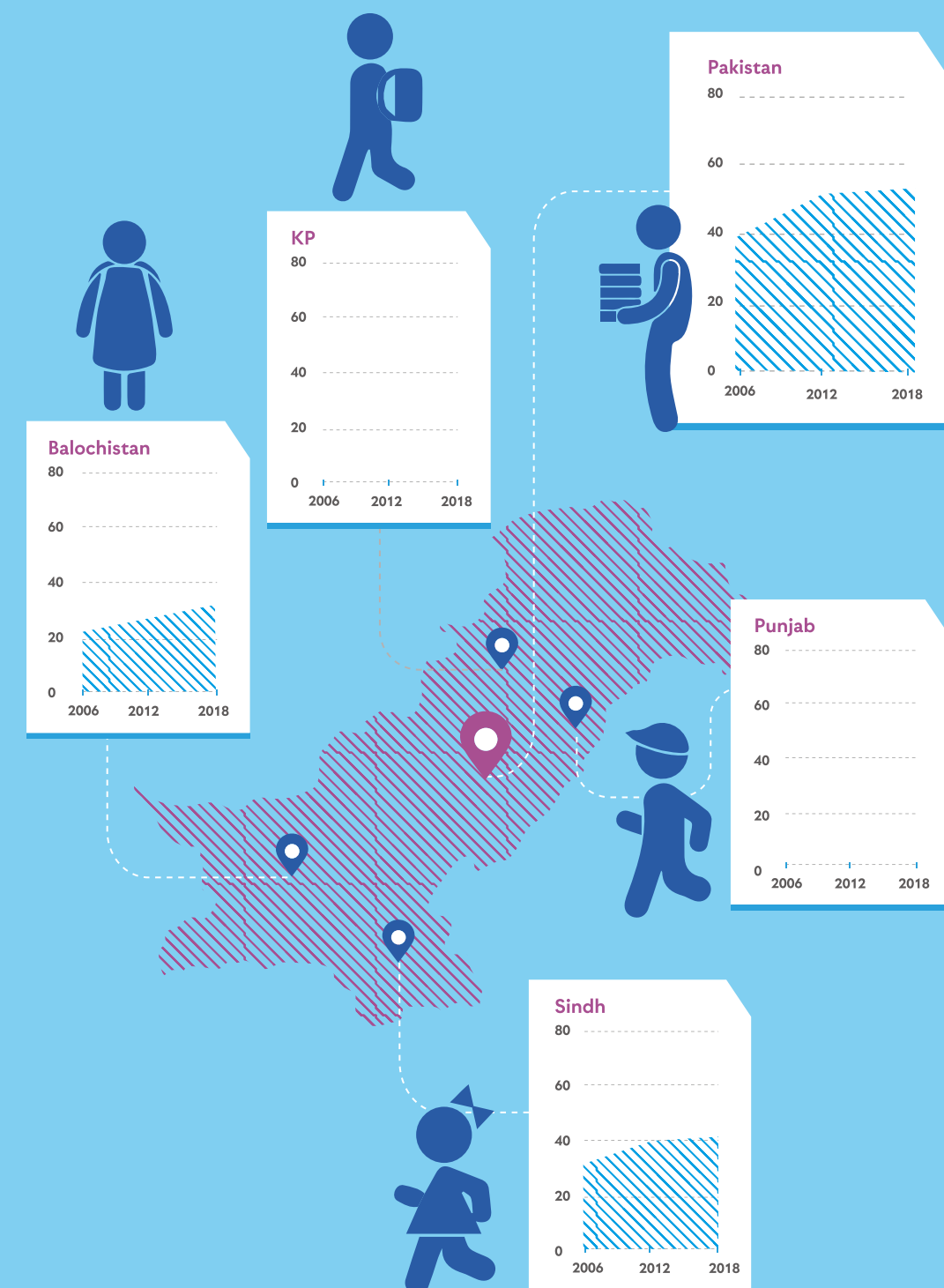


Source: Government of Pakistan, Pakistan Demographic and Health Survey (PDHS) 2006-07, 2012-13 and 2017-18



The NAR for middle and secondary education improved across all provinces and regions, except for Khyber Pakhtunkhwa. However, Pakistan's middle and secondary level NAR remains very low – 38 percent, a very slight improvement compared to 37 percent in 2012 and 27 percent in 2006. Ratios are notably higher in Punjab (45 percent), followed by Khyber Pakhtunkhwa (35 percent), Sindh (31 percent) and Balochistan (24 percent).

Figure 8: Middle and secondary Net Attendance Ratio (NAR) (2006-2018) - %



Source: Government of Pakistan, Pakistan Demographic and Health Survey (PDHS) 2006-07, 2012-13 and 2017-18

The continued gender gap in retention and enrolment at all three levels remains a major concern. This is due to fewer middle and secondary school places for girls and high population growth rate which puts burden on resources and any investment made is nullified with greater demand. This gap is particularly pronounced at the middle and high school levels. New MICS 2017–18 data reveals that primary NAR in Punjab (65.4 percent) declines sharply at the lower secondary level (36.7 percent) and falls further at the upper secondary level (28.9 percent). Girls from the province's poorest households are clearly greatly disadvantaged, as demonstrated by their extremely low NAR at the lower secondary (10.2 percent) and upper secondary levels (5.5 percent). Primary NAR in Khyber Pakhtunkhwa, according to MICS 2017 data, is 58 percent, while secondary NAR is 42.1 percent, and only 9.9 percent for girls from the poorest backgrounds. In Gilgit-Baltistan, primary NAR (49.4 percent) falls considerably at the secondary level (34.8 percent), and stands at only 15 percent for girls from the region's poorest households.

Pakistan's effective transition rate (ETR) between primary and middle school is 84 percent, marking an improvement from 78 percent in 2012–13.¹³⁴ However, it is less than 80 percent in Sindh, Balochistan and Khyber Pakhtunkhwa's Merged Districts – a concerning trend as the primary NAR in these regions is already low. Low NAR coupled with a low transition rate translates into a considerable proportion of the school age population being excluded from the formal education system at a very early stage. At present, primary schools outnumber cumulative middle and high/secondary schools by a ratio of 5 to 2. The disparity in numbers means that students are forced to travel further from their homes to access schooling beyond the primary level. This has repeatedly been documented in education research and identified as an important reason for both lower retention rates, as well as the persistent gender gap at the middle and high school levels.¹³⁵ Primary GPI currently stands at 0.92,¹³⁶ but declines further to 0.89 at the middle and secondary levels. The youth literacy GPI is only 0.81, suggesting – as with other education indicators discussed above – that access to education is more limited for girls than for boys. The GPI in Islamabad Capital Territory is over 1 at the primary, middle and secondary education levels – the best performance in the country. The Merged Districts of Khyber Pakhtunkhwa has the lowest GPI in the country, of 0.48 at the primary level, and 0.24 at the middle/secondary level. Rural Sindh's middle/secondary GPI is one of the lowest in the country (0.38).

To illustrate the challenge of post-primary retention in the public school system, the Punjab Education Sector Plan 2019–2024 calculates that for every 100 students enrolled in *Katchi*, only 28 students remain until Class 10 in government schools. The main reasons for low retention rates include a shortage of nearby schools, shortages of teachers and teachers' absenteeism, poor

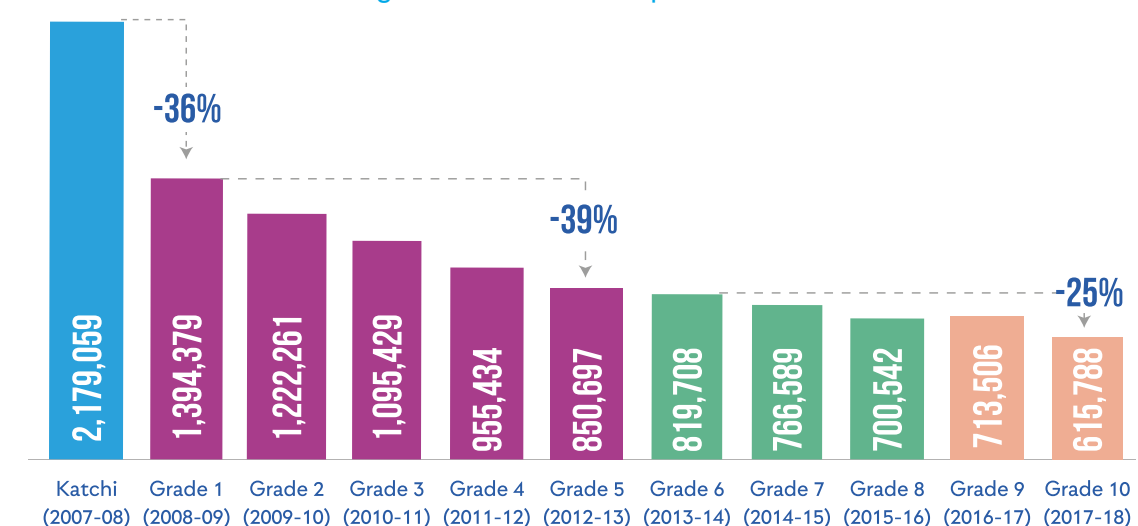
¹³⁴ National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, *Pakistan Education Statistics, 2016–17*, Government of Pakistan, Islamabad, 2018, <<http://library.aepam.edu.pk/Books/Pakistan%20Education%20Statistics%202016-17.pdf>>, accessed 4 May 2020.

¹³⁵ Asian Development Bank, *School Education in Pakistan: A Sector Assessment*, Mandaluyong, Metro Manila, ADB, 2019.

¹³⁶ Pakistan Bureau of Statistics, *Pakistan Social & Living Standards Measurement Survey 2018–19*, Ministry of Planning Development & Special Initiatives, Government of Pakistan, Islamabad, <http://www.pbs.gov.pk/sites/default/files/plsm/publications/plsm_hies_2018_19_provincial/key_findings_report_of_plsm_hies_2018_19.pdf> accessed 22 June 2020.

teaching quality, poor school environments, household poverty, insecurity and natural disasters. The retention crisis, reflecting in high dropout rates between primary and middle school, is also considerably affected by the relatively low supply of middle and secondary schools in Pakistan's public sector. Underpinning these trends are governance and management challenges within public sector education system. Additionally, low level of investment, particularly in the development budgets (not commensurate with actual needs for new schools); and poor utilization rate of capital budgets (poor capacity to spend, especially on infrastructure development) lead to a low supply of public education past primary levels.

Figure 9: Survival rates in public schools



Source: Punjab Education Sector Plan 2019/20 – 2023/2024

Challenges in the public system have fuelled a vibrant, growing private education sector. The country's private education market is dominated by low-cost private schools, which have emerged as an alternative to government schools, especially in urban areas by offering greater choice to low-income families. At present, there are over 73,000 private schools operating in Pakistan, in which some 16 million students are enrolled – roughly 36 percent of enrolment nationwide.¹³⁷ However, low cost private schools are not necessarily offering better quality education as compared to public schools due to poor quality of textbooks, untrained teachers and lack of quality teaching learning processes in classrooms (challenges which are associated generally with Pakistan's education system). Considerable proportion of these institutions are unregistered and, therefore, unsupervised by local education authorities.

¹³⁷ National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, *Pakistan Education Statistics, 2016–17*, Government of Pakistan, Islamabad, 2018, <<http://library.aepam.edu.pk/Books/Pakistan%20Education%20Statistics%202016-17.pdf>>, accessed 4 May 2020.

LEARNING OUTCOMES

While Pakistan has invested in improving learning, critical inputs are still lacking. Both low school participation rates and the high number of out-of-school children are directly linked with poor learning environments and poor learning outcomes in schools. Critical determinants for poor learning are linked to education system issues that are important for quality such as curriculum and instructional materials, teacher professional development, and assessment.

Different assessments of learning achievement in Pakistan¹³⁸ agree that the country is facing a 'learning crisis' which undermines sustainable development and poverty reduction. Available data shows that 75 percent of children of late primary school age are not proficient in reading, and 65 percent do not achieve MPL by the end of their primary education. Learning poverty – that is, being unable to read and understand a short, age-appropriate text by the age of 10 – is 16.3 percentage points worse in Pakistan than the South Asian average, and 19.5 percentage points worse than the average for lower middle-income countries.¹³⁹

Pakistan has not participated in any cross-national learning assessments in recent years. However, results from the annual citizen-led assessment, the Annual Status of Education Report (ASER),¹⁴⁰ show that 41 percent of Class 5 students cannot read a Class 2-level story in

¹³⁸ National Education Assessment System, *NAT Assessment Study 2014*, NEAS and Government of Pakistan, Islamabad, 2014; ASER Pakistan, *Annual Status of Education Report: ASER-Pakistan 2018: Urban Rural*, ASER Pakistan Secretariat, Lahore, 2019.

¹³⁹ World Bank, *Pakistan Learning Poverty Brief: EduAnalytics*, World Bank, Washington D.C., 2019.

¹⁴⁰ ASER Pakistan, *Annual Status of Education Report: ASER-Pakistan 2019*, ASER Pakistan Secretariat, Lahore, 2020, <http://aserpakistan.org/document/aser/2019/reports/national/ASER_National_2019.pdf>, accessed 4 May 2020.



Urdu, Sindhi or Pashto, a decline from 48 percent in 2016. Similarly, 45 percent of Class 5 students could not read a Class 2 level English sentence, decreasing from 54 percent in 2016. Some improvements are evident in mathematics, as 43 percent of Class 5 could not do a two-digit division, down from 52 percent in 2016.¹⁴¹ The survey also suggests that children enrolled in private schools are performing better in terms of learning outcomes than their public school counterparts. While 68 percent of children enrolled in Class 5 in private schools are able to read a story in Urdu, Sindhi or Pashto, this is true for only 58 percent of public school students. Similarly, 73 percent of private school students in Class 5 can at least read sentences and 69 percent can do division, compared to only 53 percent and 55 percent, respectively, of children enrolled in public schools.

NON-FORMAL BASIC EDUCATION (NFBE)

Alternative forms of learning are Alternate Learning Pathways (ALP) and Non Formal Education (NFE). It is generally agreed that the non-formal basic education (NFBE) sector in Pakistan can bridge current gaps, particularly in terms of addressing the learning needs of the huge number of out-of-school children. Despite its potential and though government has made attempts to promote it, non-formal education remains largely neglected in the country. Non-formal education in Pakistan includes Basic Education Community Schools (BECS), the National Commission for Human Development (NCHD), the Punjab Literacy & Non-Formal Basic Education Department, and NGOs. Approximately 1.24 million students are

¹⁴¹ ASER Pakistan, *Annual Status of Education Report: ASER-Pakistan 2016*, South Asian Forum for Education Development (SAFED), Lahore, 2017, <http://www.aserpakistan.org/documents/Report_Final_2016.pdf>, accessed 4 May 2020.

enrolled in over 31,000 non-formal education institutions in Pakistan. Some 6,000 institutions run by the National Commission for Human Development, and 12,300 by Basic Education Community Schools (BECS), are operated at the Federal Government. Over 11,000 are operated by Punjab's Literacy & NBFE Department, and 1,800 by various NGOs. Non-formal education tends to be delivered by community-based schools. In this context, accelerated learning programmes (ALP) are a major component, used as a common strategy to meet the needs of out-of-school children. NCHD runs 5,567 feeder schools, attended by 272,289 children, under its flagship Feeder Schools Programme.¹⁴² While literacy programmes predominantly focus on providing the adult population with basic education and skills development, non-formal education targets younger children and adolescents between the ages of 5 and 16. Alternative Learning programmes are offered to children who have missed out on months or years of formal schooling due to conflict or crisis. Catering to the specific needs of OOSC, alternative learning provides a degree of flexibility that national or provincial education systems simply cannot offer.

Institutions run by Punjab's Literacy & NBFE Department cater to more than 450,000 learners. These schools are all cohort-based and each is run by a single teacher, often in rooms and courtyards in their own homes. Slightly more students are girls (55 percent) than boys (45 percent), and the overwhelming majority of teachers (90 percent) are women. Once students complete Class 5, they take the PEC's Class 5 exam, required for students in formal schools to transition from primary to middle school. Once they pass the examination, these children are eligible to enrol in formal middle schools. In 2019, over 12,000 students from non-formal education institutions participated in the examination, with a 96 percent pass rate. However, girls find it especially difficult to join middle and secondary schools because of distance and mobility constraints. Therefore, Punjab's Literacy & NBFE Department has worked to upgrade primary non-formal education institutions to provide continued access for girls, and to give them the option of taking the Class 10 examination through the Allama Iqbal Open University. The department's recent needs assessment of all of its teachers aims to inform teacher support and training, delivered through a cluster-based training model.

In Sindh, Accelerated Learning Programme for age group 5 to 16 years provides access to education to marginalized overage OOSC children and adolescents for mainstreaming and reintegration at primary and elementary level in formal schools. For adolescents, the focus is on retaining girls and children that were never enrolled through accelerated learning programme consisting of a compressed curriculum covering five years of primary schooling (*katchi* to grade 5) in three years. The curriculum and textbooks for NFBE were developed by JICA in collaboration with Directorate of NFBE, School Education and Literacy Department Sindh. This also included teacher training modules on effective pedagogical skills.

While Khyber Pakhtunkhwa has initiated ALP and started making financial commitment, the government is spending 28 percent of its ADP on education which is the highest spending on

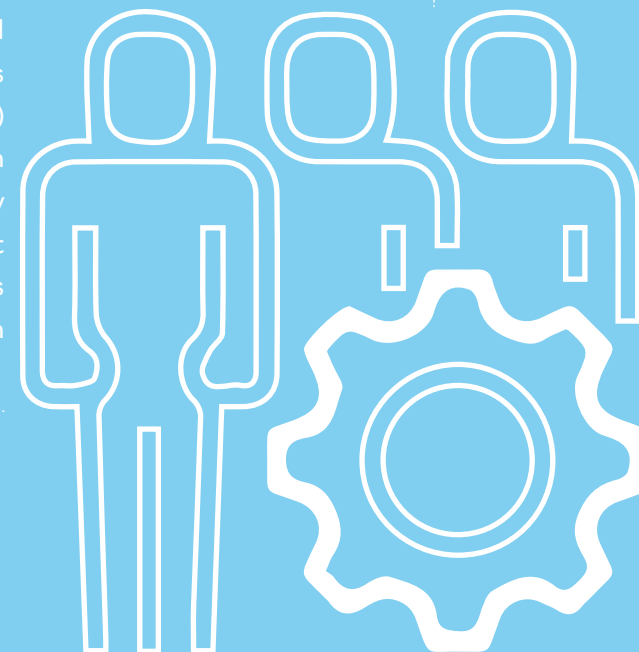
¹⁴² National Commission for Human Development, Ministry of Federal Education and Professional Training and PEHLA QADAM, *NCHD Annual Report 2017*, Government of Pakistan and PEHLA QADAM, Islamabad, 2017, <http://nchd.org.pk/ws/downloads/annual_reports/Annual_Report_2017.pdf>, accessed 4 May 2020.

education by any province in Pakistan. In Balochistan, ALP programs offer a compressed curriculum at primary and middle levels through Second Shift Centres in public and private schools as well as madrassas with maximum 35 students in a single class. Grants are provided for the uplift of the physical and learning environment of ALP centres. The community management committee along with the field teams develop plans to make the centres more conducive for the teaching and learning. Teacher training manual has been prepared by the Directorate of Schools in the Education Department. ALP kits are provided to ALP teachers in addition to training to enhance teaching techniques.

Policy promulgation is critical for creating a level of parity between the formal and non-formal education sectors. While the draft Punjab Non-Formal Education Policy 2019 will be the first non-formal education policy in the province, it is not the first in Pakistan. In 2017, Sindh launched its own provincial Non-Formal Education (NFE) Policy in 2017 and Khyber Pakhtunkhwa has also drafted its ALP policy expected to be launched in 2020. Training and professional development opportunities for teachers exist but are currently not adequate to prepare teachers to deliver quality non-formal education. Quality of learning in non-formal basic education centres remains challenges.

Box 4: Skills Training

A fundamental accelerator of transition from adolescence to adulthood is economic security, underpinned by socially approved gainful employment to sustain oneself and a family. Each year, hundreds of thousands of Pakistani young people between the ages of 15 and 29 enter the labour market, however, they finding it difficult to access decent jobs owing to poor educational outcomes, limited skills and a lack of confidence to enter the job market and succeed on the demand side and current macroeconomic environment affecting the private sector (responsible for creating 90 per cent of jobs in the country), overwhelming nature of informality in both rural and urban areas, demand for specific occupations and information gaps to reach potential employers on the supply side. Between 2015 and 2018, employment grew by 14 per cent for plant/machine operators, 10.6 per cent for technicians and associate professionals, and 5 per cent for professionals and for craft/trade-related workers. Three sectors - manufacturing, services and construction - generate the most demand annually for skilled graduates of technical and vocational education and training (TVET). The demand also comes from regional clusters (area specific and urban centers) where considerable economic activity is generated, an anticipated 800,000 jobs from CPEC in the nine priority Special Economic Zones (SEZs), international market that provided over 380,000 skilled and unskilled workers jobs in 2018 and demand for skills in the information communication technology sector.





Key stakeholders are the Ministry of Federal Education & Professional Training and the Ministry of Overseas Pakistanis and Human Resources Development, the National Vocational and Technical Training Commission (NAVTTTC), and the National Training Bureau (NTB) at the federal level; and Training and Vocational Education Authorities (TEVTAs) at the provincial level. In 2019, NAVTTTC developed the National Vocational Qualification Framework (NVQF) and initiated implementation of a competency-based training and assessment (CBT-A). There are over 3,500 TVET providers throughout the country, which enrolled over 433,000 students in 2018 under Skilling Pakistan initiative by NAVTTTC. Female enrolment in TVET is low in comparison to male enrolment and informal systems of apprenticeship tend to be more common in Pakistan.

Given the size of the youth cohort in Pakistan and potential demographic dividend it presents, there is an urgent need to find paths for skill development at scale by the government in partnership with the private sector and frontline implementation partners. An important aspect of this will be connecting secondary-age education and training to a complex and fast-changing world of work, matching young people with job opportunities, and fostering entrepreneurship. The government has prioritized action for youth, particularly youth employment and skilling, including the National Skills for All Strategy, and the Kamyab Jawan Program launched in 2019.

Generation Unlimited (GenU) was established in 2018 as a global multi-sector partnership to meet the urgent need for expanded education, training and employment opportunities for young people, ages 10-24. UNICEF has partnered with leading public and private organisations to launch this global partnership dedicated to expanding opportunity for young people. GenU is centred on finding new ways to help every young person to get into school, learning, training or employment by 2030 — with a focus on those in the greatest danger of being left behind, including girls, the poorest, those with disabilities, young people on the move and those affected by conflict and natural disasters. UNICEF and UNDP in partnership with the School of Leadership Foundation launched the Generation Unlimited Youth Challenge 2.0 in Pakistan.

Source: Developing skills in youth to succeed in an evolving south Asian economy - a case study on Pakistan, UNICEF and Overseas Development Institute (ODI), 2019.



INCLUSIVE EDUCATION

One major challenge Pakistan faces in addressing the needs of OOSC is a poor understanding of the situation and needs of children with disabilities. The country has yet to define a holistic mechanism to protect the rights of persons with disabilities at the constitutional level. It also lacks data on persons with disabilities (PWDs), which is vital for quantifying the magnitude of support they require. At present, there is no standardized instrument for collecting data on disability. Discrepancies in information on the prevalence and types of disabilities in Pakistan demonstrate that the purpose of surveys and instruments used to research disability different greatly. Therefore, their results cannot be relied upon to develop a comprehensive strategy on education for persons with disabilities.

In 2014, ASER Pakistan piloted an initiative to document the prevalence of disability prevalence in Pakistan through its household-level survey. The 2019 ASER report presents the proportion of schools that report children with disabilities within their student population, and the number of children with disabilities as a proportion of the total number of children enrolled in schools. Over 20 percent of schools reported that children with disabilities are present within their student bodies and that, on average, 0.29 percent of the students enrolled have some kind of disability. The highest is found in Khyber Pakhtunkhwa (0.53 percent) and the lowest in Sindh (0.11 percent). At the national level, 22.2 percent of public schools report that children with disabilities are among their students, compared to 16.6 percent of private schools. Punjab MICS 2018 data from

reveals that that 13.3 percent of children between 2 and 17 years old, and 6.4 percent of children between 2 and 4 years old, have functional difficulties in at least one domain.¹⁴³

The Teaching Effectively All Children (TEACH) survey finds that many children with ‘mild’ disabilities are enrolled in schools, whereas children with moderate or severe disabilities are more likely to be out of school. It also notes that children with physical disabilities are more likely to be out of school. This suggests that a lack of basic accessibility facilities in schools – such as ramps, aids and appliances, adapted teaching and learning materials – prevent children with disabilities from accessing schools and realizing their right to an education. The TEACH survey in central Punjab highlights intersectional evidence on disability, gender and poverty.¹⁴⁴ Children with disabilities are more likely to live in poorer households – 15 percent of children in the poorest quartile of the survey sample face ‘moderate to severe difficulties’, compared with 7 percent of those in the richest quartile. The survey also finds more boys with disabilities than girls in central Punjab. Most children with disabilities who attend private schools are boys with disabilities, while girls with disabilities are more likely to be out of school.

Although there are no dedicated studies in Pakistan that estimate the education completion rates of children with disabilities, a World Bank study found that among employed individuals with some form of disability (aged 18 to 65), only 27 percent have completed their primary education, compared to 42 percent of employed persons without disabilities.¹⁴⁵

An Institutional Plan for Punjab’s Special Education Department (SpED) 2017–2020 has been developed, laying out six core sector priorities and identifying the institutional capacities required to address associated challenges.¹⁴⁶ The department runs 259 schools for children with special needs, providing students with free education, textbooks and Braille books, alongside uniforms three times a year, free pick-up and drop-off facilities. Hearing aids, wheelchairs and other required supportive device are provided on a needs-based basis, while all students receive a stipend of PKR 800 per child, per month. Skills development opportunities and training courses are also provided in partnership with the Punjab Technical Education and Vocational Training Authority (TEVTA).

¹⁴³ Six domains for measuring disability: seeing, hearing, walking, self-care, communication and cognition.

¹⁴⁴ Bari, Faisal, Rabia Malik, Pauline Rose and Nidhi Singal, *Identifying disability in household surveys: Evidence on education access and learning for children with disabilities in Pakistan*, Research and Policy Paper 18/1, University of Cambridge Faculty of Education, Research for Equitable Access and Learning (REAL) and Institute of Development and Economic Alternatives (IDEAS), Cambridge, 2018, <https://www.educ.cam.ac.uk/centres/real/downloads/REAL%20Policy%20Doc%20Disability%20Pakistan%20A4%2013pp_FINAL.pdf>, accessed 4 May 2020.

¹⁴⁵ Filmer, Deon, *Disability, Poverty and Schooling in Developing Countries: Results from Eleven Household Surveys*, Social Protection Discussion Paper Series, World Bank, Washington D.C., 2005, <<http://documents.worldbank.org/curated/en/619451468315852012/Disability-poverty-and-schooling-in-developing-countries-results-from-eleven-household-surveys>>, accessed 4 May 2020.

¹⁴⁶ School Education Department, *Second Punjab Education Sector Programme (PESP II) (Draft) Inclusive Education (IE) Strategy (2019–2024)*, Government of the Punjab, Lahore, 2019.

DEVELOPMENTS IN PROVINCIAL EDUCATION SECTOR POLICIES

The *Punjab School Education Sector Plan (PSESP) 2013–2017* was developed as a guiding instrument on the Government of Punjab’s policy priorities for the education sector and strategic plans for addressing sectoral challenges. While some targets set by the PSESP were achieved during the course of its implementation, a number of targets were not. Achievements include improved school facilities, considerable improvements in teacher attendance, a marked increase in retention rates – especially for girls – and, above all, innovative partnerships with private sector players to improve the quality of education service delivery. Following on from the PSESP 2013–17, Punjab developed its *New Deal 2018–2023*, which outlines the Punjab School Education Department’s objectives in several areas to address issues of access, quality, equity and governance in the provincial education landscape. The New Deal’s ambitious goals include: transforming teachers’ effectiveness; reforming post-primary education; improving access, retention and equity; scaling-up pre-primary education; improving governance; and enhancing public-private engagement. The department also recently developed the Punjab Education Sector Plan 2019–24, whose approach is aligned with the New Deal. The plan focuses on three overarching strategic areas: quality and learning outcomes; access, retention and equity; and governance and management. Another important development is the establishment of the Punjab Education Initiatives Management Authority (PEIMA), under the *PEIMA Act 2018, to manage the Public Schools Support Programme (PSSP)*. Through the programme, the Punjab School Education Department has subcontracted the management and operation of 4,200 public schools to private sector partners. Some 600,000 children are currently covered by this programme.

Sindh’s School Education and Literacy Department (SELD) has prepared the *School Education Sector Plan and Roadmap for Sindh (2019–2024)*, which highlights eight priority programmes for achieving education objectives under three broad areas: equitable access, quality and learning, and governance and management. The eight priority programmes are: (i) out-of-school children and illiterate youth, under which non-formal education features as an important alternative learning pathway; (ii) the equitable and adequate provision of school infrastructure; (iii) equitable enrolment and retention; (iv) merit-based teacher recruitment, qualifications and professional development; (v) quality inputs and processes; (vi) professional education leadership and management; (vii) improved resource allocation and utilization; and (viii) effective strategic planning and monitoring and evaluation (M&E) of SELD interventions. Recent developments in Sindh also include the formulation of an *Early Childhood Care and Education (ECCE) Policy 2017*. Khyber Pakhtunkhwa Education Sector Plan (ESP) 2020–25 has been drafted and in final stages. The Education Sector Analysis (ESA) conducted for KP ESP structured its findings around three major priority areas i.e. improving access, retention and equity; enhancing quality and relevance of education; and improving governance and management.

Balochistan Education Sector Plan (BESP) 2020–25 has also been drafted along with Education Sector Analysis and awaits approval. BESP prioritizes learning, and access and participation as

two important policy areas; and identifies improved governance and management and better research and data as critical enablers for ensuring an efficient and effective education system.

SERVICE DELIVERY ANALYSIS

The two key implications of the 18th Constitutional Amendment for the education sector were: (i) introduction of Article 25-A obligates the state to provide free and compulsory education to all children from ages 5 to 16 years; and (ii) policy, planning, curriculum, and standards were fully devolved to provincial governments. Post-devolution reforms in the education sector have focused largely on improving access at the primary education level. Investing in at the secondary education sector and improving the quality of education and governance at all levels in Pakistan are essential to achieving education outcomes by 2030 for all education levels.

With the substantial numbers of out-of-school children of all ages, access continues to be a major challenge and large disparities are witnessed across provinces as discussed above. A high population growth rate makes raising enrolment rates particularly challenging as providing adequate numbers of good quality facilities and teachers for the growing population of children has been major issue in the country.

Inadequate quality of education has led to high dropout rates in recent years. Additionally, lack of access to middle schools, particularly for girls, has also undermined the efforts made at primary level and improving enrolments. Provinces suffer from inadequate availability of classrooms in most public schools leading to very crowded classrooms and widespread use of multigrade teaching. Although global evidence indicates that multigrade teaching is a reasonable option to provide schooling in remote and small communities, this is not the case in many schools that suffer the lack of good multigrade practices and irregular teacher attendance. Teacher deployment and gaps in the supply, quality and training of teachers in expanding areas of ECD and secondary level has led to low levels of learning and higher numbers of dropouts from such schools. According to an estimate, the total number of teachers will need to expand from 1.2 million to almost 1.7 million across education levels including public and private education sector by 2030.¹⁴⁷

It is argued that the main reason given by households for both boys and girls dropping out of school is the unwillingness of the children themselves to attend. It may be that this “unwillingness” masks several supply-side issues that households do not articulate, including poor facilities and low quality of education as well as the socio-economic situation of the household itself. Inequities in the education outcomes present themselves across genders, different socioeconomic groups, geographic areas, and within provinces and tend to compound with girls from poor rural families least likely to go to school. External factors, notably poverty, constraining social norms, and ethnic/language issues also contribute to drop-out and inequities in education and learning outcomes.

¹⁴⁷ Education Commission's Learning Generation vision model/scenario developed in 2018 based on 2015 data by UNICEF ROSA. For Pakistan to achieve universal pre-primary, primary, and secondary education access by 2030 on par with high income countries.

Low domestic investment in social spending calls for increase in spending and sustained higher domestic allocations to the education sector. For acceleration of reforms, interventions are needed to: (i) expand and invest in pre-primary as an effective way to increase enrolment, reduce drop out, improve learning outcomes, address equity and enhance overall basic education efficiency through strengthened sector management; (ii) effective investment in equity by scaling up efforts and developing an actionable strategy to expand school opportunities, target resources to the most disadvantaged – particularly girls, and promote accelerated learning opportunities; (iii) adopt innovative approaches to increase numbers and quality of teaching and non-teaching professionals; (iv) increase opportunities to address wider barriers to participation and cross-sectoral collaboration in education, such as water and sanitation, health, nutrition, tailored interventions for girls; and (v) expand provision at all levels offers an opportunity to seek innovative financing and partnerships with non-state actors – including exploring further opportunities for low-cost private schools combined with effective regulation.

IMPACT OF COVID-19 ON EDUCATION CONTINUITY AND LEARNING

COVID-19 has caused the worldwide closure of schools on an unprecedented scale. Globally, 188 countries have closed schools, affecting more than 1.5 billion children and youths,¹⁴⁸ including 42 million learners in Pakistan.¹⁴⁹ The experience of past shutdowns reveals a high risk of students, especially girls, dropping out of education – once they are out of school for extended periods of time, there is an acute risk that students will not return once schools reopen. Prolonged school closures and a loss of learning, therefore, will affect enrolment, learning, retention, and dropout rates in Pakistan. There is growing consensus that school closures will widen learning inequalities and disproportionately harm vulnerable children and youths.¹⁵⁰ While public sector investments in education sector are already considerably below desired levels across the country, the pandemic will cause further ‘shrinkage’ in the resources available for education service delivery.

COVID-19 is set to magnify and the exacerbate risks and vulnerabilities of Pakistan's already weak education system. It appears poised to cause higher rates of girls' absenteeism when schools reopen, as girls may struggle to balance schoolwork and the increased domestic responsibilities that befall them during the lockdown period. Geographically, rural areas and urban slums are the highest risk areas, with current enrolment of 70 percent of current enrolment and large pockets of out-of-school children not enrolled in any form of education.¹⁵¹ Intergenerational effects on educational outcomes are expected in the medium-term, as children of caregivers who are unable to home school them face the possibility of falling further behind on age-appropriate learning competencies.

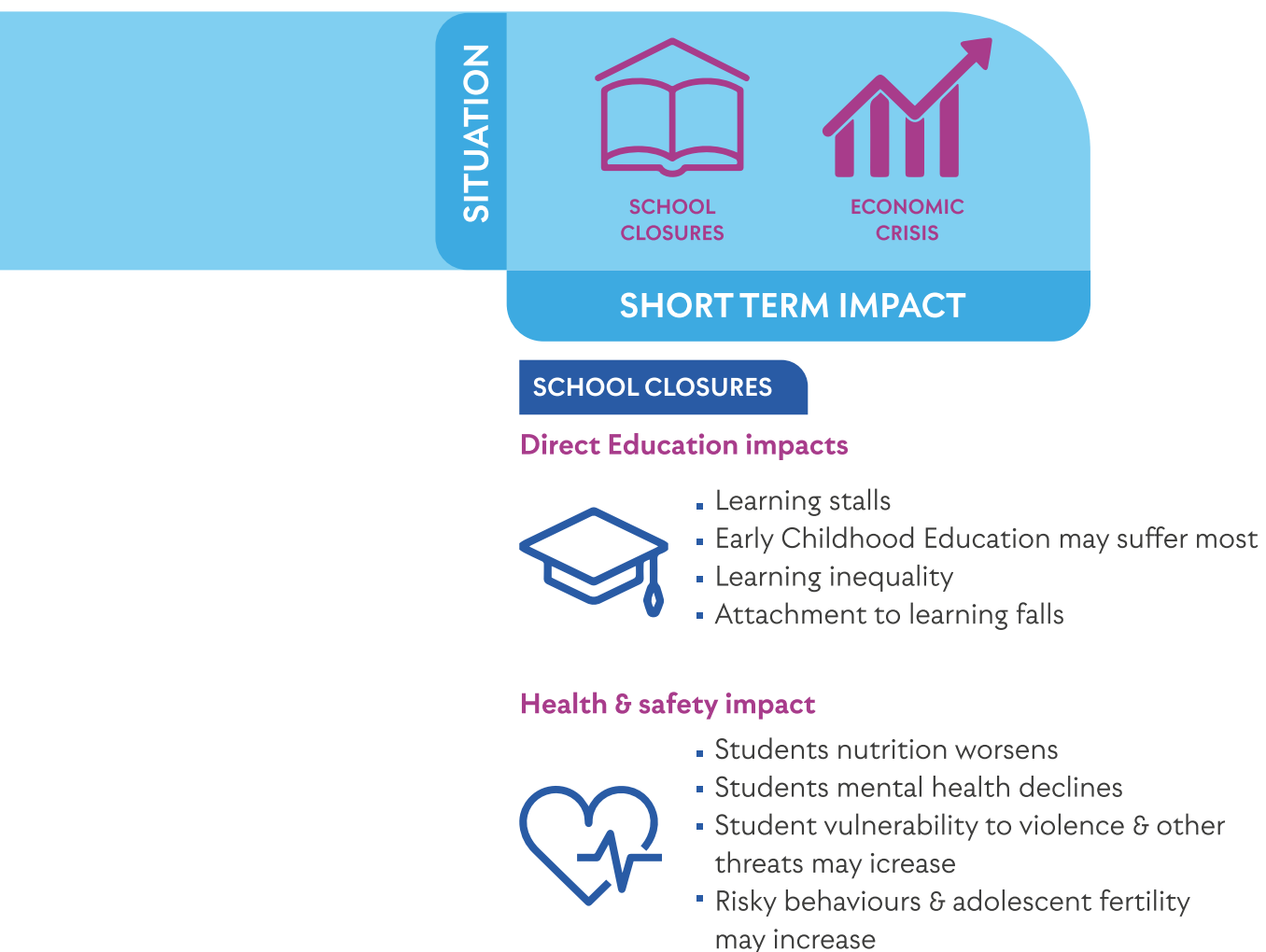
¹⁴⁸ United Nations, *Policy Brief: The Impact of COVID-19 on Children*, UN, New York, 15 April 2020, <https://www.un.org/sites/un2.un.org/files/policy_brief_on_covid_impact_on_children_16_april_2020.pdf>, accessed 4 May 2020.

¹⁴⁹ Pakistan Country Office, UNICEF.

¹⁵⁰ United Nations Educational, Scientific and Cultural Organization, *Global Education Coalition: COVID-19 Education Response*, UNESCO, Paris, 2020, <<https://en.unesco.org/covid19/educationresponse/globalcoalition>>, accessed 4 May 2020.

¹⁵¹ United Nations Children's Fund, *Covid-19 – Pakistan Socioeconomic Impact Assessment & Response Plan*, UNICEF, Islamabad, April 2020.

Figure 10: Impact of COVID-19 on education

**ECONOMIC CRISIS****Education demand-side**

- Drop-outs increase (especially for poor/ disadvantaged)
- Child labor, child marriage
- Educational investments by parents fall

Education supply-side

- Govt spending on education falls, education quality declines
- Teaching quality declines
- Private schools close

LONG TERM IMPACT

Poorer education outcomes



Lower human capital (dropout and learning decline, leading to a lifetime of lower productivity and earnings with macro effects)



Increased inequality of opportunities, especially for girls.



More unemployment; out-of-school children increase: estimates between 700,000 and 3 million new OOSC.

Source: UNICEF Pakistan, Education Team



The longer schools remain closed, the less likely children are to catch up on learning. It appears that loss of learning will have a twofold affect. Firstly, loss of learning compounds over years. Unless remedial education is organized to address the learning gap resulting from school closures, children often fall further behind as they struggle with new content being introduced, before they have had a chance to review and absorb the prerequisites. School closure of a few months, could result in a total learning loss of more than a full year by the time the child reaches Grade 10. This accounts for the difference between the number of years of schooling, and the learning-adjusted number of years of schooling. The second effect is the widening of inequalities. The most privileged students can be stimulated at home from educated parents and siblings, and be given access to multiple modalities of learning, such as digital learning, TV and radio, mobile phone apps etc. But the most disadvantaged children do not have access to any alternative modalities of learning and therefore fall further back, which further increases their risk of drop-out after schools reopen. Furthermore, special needs students who stand the most to benefit from face-to-face interactions with teachers for learning, are the ones who suffer the most, as alternative modalities of learning rarely cater for their special needs.

Response: To contain the spread of COVID-19, the Federal Government announced the closure of all the schools in Pakistan on 13 March, 2020. It is still uncertain whether the schools will reopen on the expected date of 15 July. Many schools nationwide have been designated as temporary isolation and quarantine facilities.¹⁵² Ministry of Federal Education and Professional Training launched the Pakistan National Education Response and Resilience Plan (K-12) in early May 2020. It provides a framework of strategies and interventions for Pakistan's education system to cope with the effects of COVID- 19 and prioritizes three main areas - continuation of learning, system strengthening and resilience, and addressing health, hygiene and safety. Provincial education departments will need to explore a mix of no tech/offline, low tech and high tech/online modalities across the technology spectrum to reach maximum number of children as well as respond to learning needs and of children at different levels.

Specific interventions by the Government include the launch of 'Teleschool', a dedicated national public television channel that will broadcast school lessons via satellite, terrestrial and cable television. Programmes are being broadcast between 8 a.m. and 5 p.m. every day, featuring content for Classes 1 to 12. At the provincial level, the Government of Punjab launched digitized lessons and an online teaching programme for public school students enrolled in Classes 1 to 8. This programme, *Taleem Ghar*, is accessible on cable television, mobile applications, and online on the programme's website. The initiative is led by Punjab's School Education Department, in collaboration with its Programme Monitoring and Implementation Unit (PMIU) and the Punjab IT Board (PITB). The department has also announced that Class 8 students will be promoted to the next grade without taking their Class 8 examinations. In Sindh, the School Education and Literacy Department has started developing content for tele-schooling.

¹⁵² Ministry of National Health Services, Regulation, and Coordination, *National Action Plan for Coronavirus disease (COVID-19) Pakistan*, Government of Pakistan, Islamabad, 2020.

Development partners have come forward to support Pakistan's federal and provincial governments to minimize the disruption of education and facilitate the continuity of learning. These include the World Bank, which has included education as part of its *Pandemic Response Effectiveness in Pakistan Project*, worth a total of US\$200 million. The project assigns US\$5 million to its subcomponent on mitigating COVID-19's impact on education by supporting remote learning opportunities for 50 million children whose schools are closed. This subcomponent aims to support a comprehensive communications campaign for schools and parents to engage in distance learning activities (including television and radio broadcasts, a virtual network of teachers and other means of delivering academic content at a distance). It also involves the development and implementation of plans to ensure the continuity of learning, such as remote learning options, at all levels of education.¹⁵³

UNESCO, through its *COVID-19 Global Education Coalition* initiative, is supporting efforts by governments worldwide to mitigate the immediate impact of school closures, particularly for more vulnerable and disadvantaged communities.

SHORT-TERM RECOMMENDATIONS:

- Ramp up existing distance learning programmes using available resources across the technology spectrum (no tech/offline, low tech and high tech/online modalities) to rapidly expand outreach, especially for girls and vulnerable children from low income or rural households with limited access to digital technology. Develop content that specifically targets girls and increase the accessibility of learning materials in partnership with civil society, NGOs and the private sector.
- Develop and implement a distance and blended learning competency standards and assessment framework. Ensure that all of Pakistan's provinces take the lead on developing and promoting localized, free and open digital tools for learning, to ensure large-scale nationwide remote learning.
- Create and develop content for different platforms and modalities according to levels, subjects and grades.
- Develop, print and distribute learning materials where there is limited access to online content or digital learning, to ensure the accessibility to learning materials for all. This is particularly important for children with disabilities whose needs are often overlooked in such initiatives.
- Strengthen and support the role of teachers, head teachers and schools by conducting teacher and head teacher trainings and orientation sessions for different learning modalities and provide them with necessary guidance and tools for community outreach. Address the need for social dialogue and engagement with unions to agree on working conditions for public teachers and set standards for the private sectors, given that the nature of the role of teachers has changed. This is important to get teacher buy-in in order to fulfil their new responsibilities.

¹⁵³ World Bank, *Pandemic Response Effectiveness in Pakistan (P173796)*, World Bank, Washington D.C., 2 April 2020, <<http://documents.worldbank.org/curated/en/651371585953227830/pdf/Pakistan-COVID-19-Pandemic-Response-Effectiveness-Project.pdf>>, accessed 4 May 2020.

- Provide practical support to the parents and caregivers of schoolchildren, especially by educating mothers on effective home-schooling, interacting with children in ways that promote learning, and engaging regularly with them during the lockdown. Mobilize all local level resources, such as parents, youth groups, religious leaders, active community groups, in favour of education. Schools-teachers should communicate clearly learning objectives to all parents to be able to become facilitators and support their children's learning - at least the process of it - through following guidance from schools.
- Ensure that government institutions partner with initiatives that support states to scale up learning while schools remain closed, such as the COVID-19 Global Education Coalition.
- Develop campaigns and advocacy efforts about risks from COVID-19 and procedures to prepare schools for safe reopening for children and staff. Upgrade WASH facilities at schools and focus on infection prevention and control (IPC). Support Clean Green School Programme - part of the Clean Green Pakistan Movement by the government.

MEDIUM-TERM RECOMMENDATIONS:

- Revisit provincial Education Sector Plans and Strategies to address weaknesses observed during the pandemic, such as the capacity of the education sector's delivery chain from top to bottom.
- Develop contingency plans for high stakes examination systems.
- Screen students once schools reopen and adopt special measures for education facilities that have been used as quarantine, isolation or care centres for COVID-19 patients. Establish linkages with health, nutrition and sanitation interventions to ensure holistic child development.
- Develop, implement and expand remedial learning/ALP/NFE programs for children who fall behind due to school closures.
- Expand demand-side interventions to bring girls back to school through targeted messages on participation and the use of incentives, such as monthly stipends, voucher subsidies and conditional cash transfers linked with high rates of attendance. Expand existing subsidy programmes at the national and provincial levels, including the Benazir Income Support Programme's (BISP) *Waseela-e-Taleem* initiative and provincial education voucher schemes.
- Arrange for schools to offer remedial classes to bridge learning gaps through accelerated education, especially with a view to minimizing the risk of high dropout rates.
- Design and rollout system-wide solutions to address missed examinations.
- Support government institutions' response coordination, service delivery and real-time monitoring to ensure that up-to-date information on the scale of education sector challenges is available beyond aggregate numbers. Monitor dropout rates post-COVID-19 – especially the participation rates of girls and children from impoverished or marginalized families through databases maintained by BISP and provincial education foundations – to inform realistic future programming. Integrate the learning outcomes of children with disabilities into the regular workstreams of provincial Education Departments, rather than addressing their needs in silos.

- Develop a comprehensive approach to mitigate the economic recession's spillover effects and safeguarding the education system. Revisit education budgets at the federal and provincial levels to ensure equitable allocations and the sustained funding of initiatives that support the continuity of learning. Address the uneven supply of education spaces across primary, middle and secondary education.
- Promote game-changing investments that support innovation and multiple pathways to education (for instance broadcasts, digital and online learning) supported by policy formulation and institutional arrangements. Bridge the digital divide and ensure that EdTech solutions can be available to all children including the most disadvantaged. Investments are necessary in school infrastructure development (electricity and internet, connected devices), teacher capacity (digital skills), curriculum adaptations (integration of digital skills early), and access to connected devices for all children. Given the magnitude of the investment to expand coverage and access, leveraging private sector investments and building public-private partnerships will be essential.
- Increase the coverage of early childhood education by increasing investment and focusing on scaling-up ECE.
- Identify and expand accelerated and alternate learning pathways in hard-to-reach areas where the pandemic's effects have been particularly severe. Prioritize extensive community engagement and social mobilization, alongside developing strong institutions, and relevant programmes that target older out-of-school children and those unlikely to get back into formal schools.
- Enhance the capacities of teachers to deliver accelerated education and remedial classes. Train teachers on conducting diagnostics assessments based on which to design remedial teaching strategies, and then blended approaches to teaching and learning. Enabling teachers to be able to use multiple modalities to reach children that are either in their physical classroom, or studying from home, and still be able to impart the same knowledge and skills.
- There is also a need for teacher management plans, which would result from the integration of pre-service and in-service trainings, in order to manage the upskilling of teachers on 21st century skills and digital skills. This is essential to enable teachers within a year or two to be able to still engage with their class students and guide their learning from a distance, if schools were to close again. This additional supply of education through alternative modalities can serve as a complement to classroom-based learning and can reinforce learning for school-going children, or offer alternatives for children who are not in school. The current crisis offers an opportunity to make education a free public good for all to access.
- Integrate health and hygiene as part of the school curriculum and syllabus.
- There is a need to have adequate WASH infrastructure in schools including hand washing stations and awareness on proper hand washing.



EVERY CHILD PROTECTED FROM VIOLENCE AND EXPLOITATION

All children have the right to live free from violence, fear, neglect, exploitation and abuse. The SDGs explicitly articulate protection goals for children. Ending violence and torture against children is the central aim of Goal 16 (target 16.2), while specific forms of violence and child harm are addressed by a range of other targets. These include targets 5.2 and 5.3 that proscribe all forms of violence against women and girls, including harmful traditional practices such as child, early and forced marriage and female genital mutilation, target 8.7 on eradicating child labour and the recruitment and the use of child soldiers, and target 16.9 on protecting children’s right to a legal identity through birth registration. The latter is particularly relevant as children who are not ‘covered’ by a legal registration process are invisible to governments – legislative and policy frameworks on child protection do not cover them, nor to social protection interventions.

BIRTH REGISTRATION

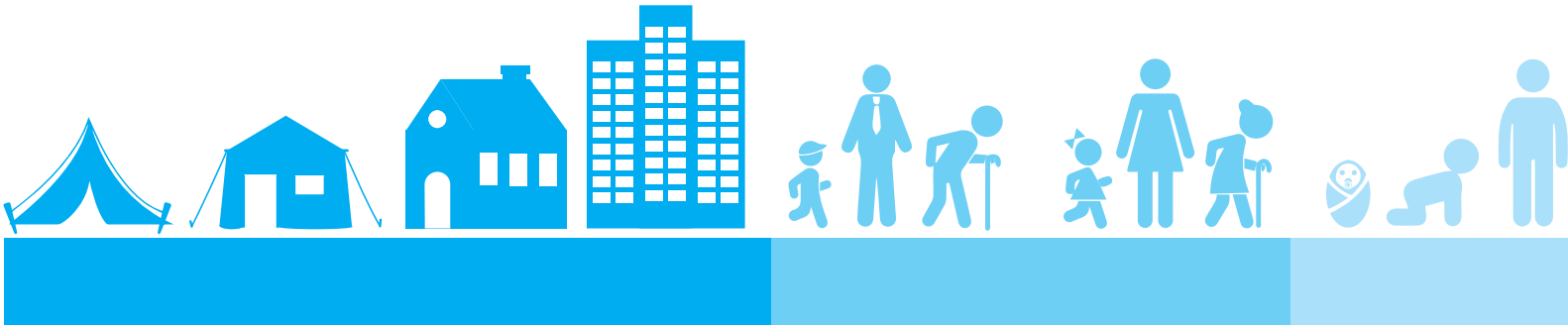
Being registered at birth is a human right. Birth registration of children under-five in Pakistan has improved from 34 percent in 2012–13¹⁵⁴ to 42.2 percent in 2017–18¹⁵⁵ (although this figure excludes Gilgit-Baltistan, and Azad Jammu and Kashmir). Major improvements are apparent in Balochistan, where birth registration rose from 8 percent to 38 percent in the same period, followed by Khyber Pakhtunkhwa, whose rates increased from 10 percent to 19 percent. Marked progress was also recorded in Sindh where registration rose from 25 percent to 28 percent and Punjab increasing from 46.1 percent to 57.8 percent. Nationwide, children under the age of two are less likely to be registered (39 percent) than children age between 2 and 4 years old (44 percent). The registration of older children is primarily driven by the practice of asking parents to produce a child’s birth certificate for school admission.

154 National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2012–13*, NIPS, Islamabad, 2013.
155 National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

Table 4: Birth registration across provinces and regions

%	Total	Urban	Rural
Punjab	57.8	70.5	51.7
Sindh	27.6	53.8	6.9
KP	18.8	30.4	16.3
Balochistan	37.8	46	34
ICT	82.4		
FATA	2.2		
GB	27.1		
AJK	29		

Source: PDHS 2017-18



Birth registration is considerably higher in urban areas (60 percent) than in rural settings (34 percent) with pronounced regional disparities. For instance, only 2 percent of children in Khyber Pakhtunkhwa’s Merged Districts and 19 percent of children in the province of Khyber Pakhtunkhwa are registered, compared to 82 percent of children in Islamabad Capital Territory. Stark disparities exist across wealth quintiles as only 9 percent of children are registered among families from the lowest wealth quintile in Pakistan (up from 5 percent in 2012–13), compared to 76 percent (up from 71 percent) of children from the highest wealth quintile. Marked discrepancies exist between MICS and

PDHS data due to differences in the surveys’ methodologies. MICS survey interviews are carried out exclusively with mothers and primary caregivers for the indicator on birth registration, while PDHS interviews are undertaken with any household respondent. New MICS data from Punjab and Khyber Pakhtunkhwa reveals mixed results compared to the last MICS. In Punjab, birth registration has improved from 73 percent to 75.3 percent and registration in rural areas is higher than in urban centres. However, registration has declined in urban parts of Punjab since 2014, falling from 83 percent to 74.8 percent in 2018, while rising in rural areas from 68 percent to 75.8 percent. MICS data from Khyber Pakhtunkhwa

found marginal improvements, identifying an overall birth registration rate of 20 percent among children under-five, which rises to 29 percent in urban areas (up from 27 percent, according to the province's last MICS). However, the survey found that birth registration in rural areas has stagnated at 18 percent.

Civil registration and vital statistics system in Pakistan: Civil registration and vital statistics (CRVS) systems are concerned with the legal registration and analysis of vital events in the population. Civil registration is defined as the continuous, permanent, compulsory and universal recording of the occurrence and characteristics of vital events (live birth, death, marriage, divorce, migration etc.) and other civil status events pertaining to the population in accordance with the law. Records of vital events from civil registration are the critical source of vital statistics. Pakistan has been identified as one of the six priority countries for CRVS strengthening by the United Nations Economic and Social Commission for Asia and the Pacific (UNESCAP) with the goal to “Get Every One in the Picture”. In this context, birth registration is not only the legal identity rather it is also the first step towards social inclusion; while a compulsory recording of marriages through marriage certificate may provide immense help in determining the age of marrying partners and hence prevent the early marriages. The cardinal importance of CRVS lies in providing population data as it the denominator for all the population-based SDGs indicators.

Pakistan has a decentralized civil registration system in which the local bodies/urban councils undertake the actual registration work at the local level. Authorities in charge of registering a birth are provincial local government departments - union councils. The National Database and Registration Authority (NADRA), which is the custodian of the data warehouse on national security and population and falls under federal government, automated and centralized computerized registration and certificate issuance of vital events at the local level. The health sector is also involved in the process of registration of births and deaths at the local level.

Currently, Pakistan does not produce disaggregated, vital statistics reports from civil registration data as the coverage of civil registration is too low. The Statistical Office is using data of the Population Census and other surveys such as Demographic Health Survey to produce vital statistics on a regular basis.

In 2013, the Ministry of Planning, Development and Reforms (MOPDR) conducted rapid and comprehensive assessments of the CRVS system in Pakistan, in close collaboration with provincial departments, NADRA and development partners. These assessments revealed serious weaknesses including exceptionally low national data for birth and death registration and almost no death registration mechanisms in place, and very few vital statistics generated from the civil registration systems. The assessments called for the promotion of CRVS in the country with formulation of strong coordination mechanisms with all relevant ministries, departments and partners at national and provincial levels for the development and adoption of a uniform CRVS law in place of current fragmented laws. Consequently, a high-level coordination mechanism, namely

the National CRVS Steering and Coordination Committee was established in September 2014. The Committee identified key thematic areas for the development of a National CRVS Strategy as: (i) legal framework; (ii) resources for civil registration; (iii) registration practices, coverage and completeness; (iv) death certification and cause of death; (v) ICD mortality coding practices; and (vi) data access, use and quality checks. Furthermore, a Technical Support Unit for CRVS was established in October 2017 to oversee and facilitate the CRVS development process.

In order to improve CRVS system in Pakistan, NADRA has taken initiatives such as a new project, “Digital Birth Registration” (DBR) project, was launched to improve the birth registration rate, using mobile technology for civil registration certificates. The project has been designed to ensure universal birth registration for children in target districts by 2024, primarily through improved governance structures and the use of specially designed information and communications technologies.¹⁵⁶

VIOLENCE AGAINST WOMEN AND CHILDREN

For the purpose of this analysis, ‘violence’ is used to denote all forms of harm to children,¹⁵⁷ whether physical, psychological or sexual. It is taken to encompass both physical and/or non-physical harm, as well as intentional and/or non-intentional harm, such as neglect and psychological maltreatment. It also includes exploitation and harmful practices.

Psychological aggression, physical punishment or violent behaviour: New MICS data from Punjab in 2018 show that 80.8 percent of children between 1 and 14 years old have experienced physical punishment and/or psychological aggression by caregivers in the month preceding the survey. When compared to Punjab MICS data from 2014, it suggests that prevalence of violent behaviour remains stagnant for the last 4-years when it was 81 percent. Experiences of such violence continue to be pervasive in Punjab, cutting across all socioeconomic cohorts and geographic locations – with similar rates evident across urban and rural areas, among girls and boys, and across wealth quintiles.

In Khyber Pakhtunkhwa, MICS 2017 reports rates of violence are widespread. Among children between 1 and 14 years old, 81 percent were subjected to at least one form of psychological or physical punishment (any violent discipline) by household members in the month before the survey. The highest percentage is reported for children between 3 and 9 years old (over 85 percent). While 77 percent of children in the province experienced psychological aggression, 64 percent were subjected to physical punishment, and 25 percent to severe forms of physical punishment – including being hit on the head, ears or face, being hit hard, and being hit repeatedly. Rural children in Khyber Pakhtunkhwa are subjected to more physical violence (65 percent) than urban children (58 percent).

¹⁵⁶ UNICEF. *Status of Civil Registration and Vital Statistics in South Asia Countries 2018*, UNICEF, 2019.

¹⁵⁷ As listed in Article 19, Paragraph 1 of the *Convention on the Rights of the Child (CRC)* in conformity with the terminology used in the 2006 United Nations study on violence against children. Nevertheless, other terms used to describe specific types of harm (injury, abuse, neglect or negligent treatment, maltreatment and exploitation) carry equal weight.

Tensions between neighbouring countries and the emergence of new groups and continued threats from militant groups has affected the security situation, including children's rights and protection in Pakistan. Balochistan, Khyber Pakhtunkhwa and the Merged Districts remained the main geographical areas of concern. The previous annual reports of the Secretary-General on children and armed conflict verified the recruitment and use of children attributed to armed groups in Pakistan as well as increasing number of child casualties and attacks on schools, including the targeting of girls' education and attacks relating to the Global Polio Eradication Initiative. The latest report¹⁵⁸ states that, although a decrease in the number of attacks against schools and in the number of child casualties have been noted, still a total of 23 children were reportedly killed (5) and injured (18) during armed clashes or by shelling or targeted fire across the line of control, by improvised explosive devices and explosive remnants of war in Azad Jammu and Kashmir, Punjab, Balochistan and Khyber Pakhtunkhwa Provinces; and three attacks against schools and hospitals and over 660 attacks or threats of attacks against polio eradication workers and facilities, mostly in Balochistan and Khyber Pakhtunkhwa have been reported.

Violence against women: According to PDHS 2017–18 data, 28 percent of women between the ages of 15 and 49 have experienced physical violence since they were 15 years old. Although this figure remains unacceptably high, it marks a decrease from 32 percent in 2012–13¹⁵⁹. Among ever-married women age 15–19, 26.3 percent experience physical violence and 33.4 percent experienced physical or sexual violence. The highest incidences of physical violence against women are recorded in the Merged Districts of Khyber Pakhtunkhwa (56 percent), followed by the provinces of Balochistan (48.4 percent) and Khyber Pakhtunkhwa (43 percent). Nationwide, young women between 15 and 19 years old are subjected more to physical violence (31.5 percent) than women who are older. Considerable improvements are evident in rural areas, fewer young women experiencing violence in 2017–18 (29.6 percent) than in 2012–13 (34 percent). In urban areas, violence against women aged 15 to 19 also declined during the same period (24.2 percent compared to 28 percent).

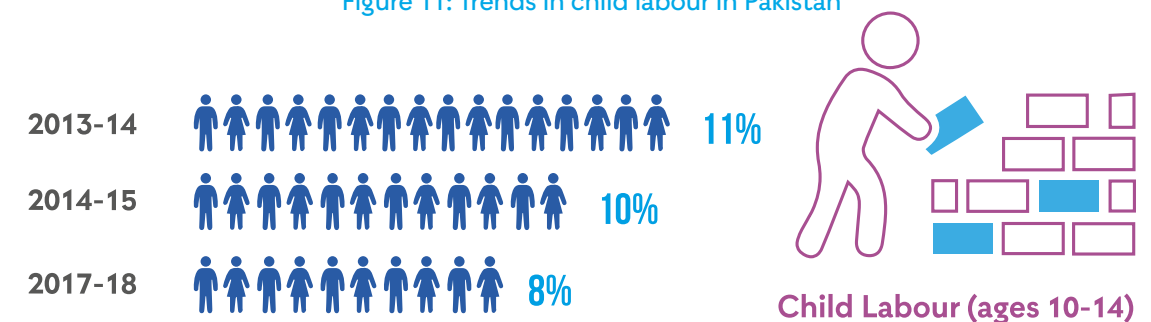
Exploitation: Fourteen is the minimum legal age for admission to hazardous work/employment in Pakistan. However, many younger children work in family establishments or non-hazardous occupations, resulting in a high prevalence of child labour nationwide. This is despite the fact that Article 25A of Constitution of Pakistan obligates the state to provide free and compulsory quality education to children of the age group 5 to 16 years. Work by children between the ages of 5 and 14 is not well-documented, nor is child labour data regularly collected/collated. The last national child labour survey was published in 1996. Pakistan's first national survey on child labour in over two decades is currently underway, led by the provincial labour departments and implemented by the provincial bureau of statistics with UNICEF's support.

¹⁵⁸ Report of the UN Secretary-General on children and armed conflict (A/74/845–S/2020/525) issued on 9 June 2020.

¹⁵⁹ National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2012–13*, NIPS, Islamabad, 2013.

The lack of recent data prevents federal and provincial governments from accurately assessing the scale of child labour. Some information, however, is recorded by the 2017 Census and the Pakistan Labour Force Survey (LFS) – the country's primary sources for labour statistics. According to the latest LFS, the current labour force participation rate for children aged 10 to 14 in Pakistan (8.23 percent) reveals a downward trend in recent years.¹⁶⁰ More boys (9.82 percent) than girls (6.4 percent) between girls 10 and 14 years old are involved in child labour. Rates increase manifold for children aged 15 to 19, whose labour force participation rate was 32.57 percent in 2018. Provincial analysis of children between 10 and 14 years old who are engaged in economic activities reveals that Punjab has the highest proportion of child labourers (10.32 percent), followed by Sindh (7.16 percent), Khyber Pakhtunkhwa (4.57 percent) and Balochistan (4.33 percent).¹⁶¹

Figure 11: Trends in child labour in Pakistan



Source: LFS 2013-14, 2014-15 and 2017-18

Further information on child labour is provided by MICS data 2017, available for Punjab and Khyber Pakhtunkhwa. In Punjab, the survey identified 13.4 percent of children aged 5 to 17 engaged in child labour (down from 16 percent in 2014). This includes 17.5 percent of children in this age group in rural areas, and 6.5 percent in urban settings. Alongside geographic disparities, stark disparities exist across wealth quintiles – while 28.1 percent of children from the province's poorest households are engaged in labour, this is true for only 3.1 percent of children from the richest families.¹⁶²

In Khyber Pakhtunkhwa (MICS 2017), 14.4 percent of 5 to 17-year-olds are engaged in child labour, (15.7 percent in rural areas and 6.9 percent in urban centres). Disparities across wealth quintiles are similarly pronounced, with one-quarter of children aged 5 to 17 from the poorest households are child labourers, compared to 4.1 percent from the richest. Approximately one-quarter of children between 15 and 17 years old in Khyber Pakhtunkhwa are engaged in economic activities. The ongoing child labour survey which is at different stages in different provinces will provide additional and more up to-date information on the status of child labour in the country.

¹⁶⁰ Pakistan Bureau of Statistics, *Labour Force Survey 2017–18*, Government of Pakistan, Islamabad, 2018.

¹⁶¹ Ibid.

¹⁶² Punjab Bureau of Statistics and United Nations Children's Fund, *Multiple Indicator Cluster Survey 2017-2018, Punjab*, Government of Punjab, Lahore, 2018.

Harmful Practices: The incidence of child marriage in Pakistan is high. Rates of child marriage (women age 15–19) have remained at 14 percent since 2013.¹⁶³ Over 7 percent of women aged 25 to 49 reported that they married by the age of 15, while a significant proportion (35.2 percent) were married by the time they turned 18. Over 48 percent of women aged 20 to 24 are married according to PDHS 2017–18. Women with no education marry more than six years earlier than women with higher education (18.7 years versus 24.9 years).

New MICS 2018 data from Punjab reveals that that 14.6 percent of women between 20 and 24 years old were married before the age of 18, while 5.7 percent married before turning 16. Moreover, 10.5 percent of women aged 15 to 19 in Punjab are currently married. The incidence of child marriage is especially high in rural areas, where one in every five women aged 20 to 49 married before their 18th birthday, while 30.8 percent of women age 20–49 from poorest wealth quintile were married before 18. In Khyber Pakhtunkhwa, MICS 2017 data finds that 18.8 percent of young women between 15 and 19 years old are married. Proportions in urban centres, rural areas and across wealth quintiles hover around the same levels, indicating that the practice of child marriage is pervasive and an accepted norm in the province. Among women aged 15 to 49 in Khyber Pakhtunkhwa, 30 percent were married before the age of 18, and 7.7 percent before the age of 15.

In Pakistan, *watta satta* (exchange marriages) are widespread and represent a large proportion of child grooms, especially in rural settings, since such marriages do not require dowry. There is also evidence of consanguineous marriages in Pakistan, which are more common among 15–24 year old women, rural women, women belonging to low socio-economic status, and husbands and wives who were in the unskilled working category. Alongside poverty and wealth, family structure and female agency give some clues as to the drivers of child marriage and indicate that the relationship between wealth/poverty and child marriage is influenced by a variety of other factors in addition to poverty/wealth indicators. Where women are the heads of household, there are significantly fewer child marriages than households with male heads of household and that women's decision-making power and indicators of women's empowerment (such as attitudes towards domestic violence, actual experience of domestic violence, women's decision-making power over household earnings and use of contraceptives) are related to lower rates of child marriage.¹⁶⁴

CHILDREN WITHOUT PARENTAL CARE

Alternative Care is defined as the care provided to a child who cannot live with her or his parents. Possible types of alternative care include: kinship or non-kinship care, foster care placement, other types of family-like or family-based care, placement in a suitable residential care facility

¹⁶³ National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2012–13*, NIPS, Islamabad, 2013; National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

¹⁶⁴ United Nations Children's Fund and United Nations Population Fund, *Child Marriage in South Asia: An evidence review*, UNICEF, Kathmandu, 2019.

(sometimes referred to as 'institutions'), or supported living arrangements for older children – but always, where possible, with the aim of family reunification, and always with the best interests of the child placed at the central objective of service provision.¹⁶⁵ In its Concluding Observations and Recommendations for Pakistan in 2016, the Committee on the Rights of the Child was concerned about the insufficiency of the assistance provided to families with children living in poverty and the absence of psychosocial support and guidance for families in need, leads to the abandonment and institutionalization of children. Drawing the State party's attention to the Guidelines for the Alternative Care of Children (2009), the Committee emphasizes that financial and material poverty should never be the sole justification for removing a child from parental care, for receiving a child into alternative care or for preventing a child's social reintegration. It is concerned that many children without parental care reside in private orphanages, institutions, including religious institutions (madrasas), and shelters that are sometimes registered with the national or provincial governments, but are not provided with any benchmarks for quality of care or monitored by the State party.¹⁶⁶ Though no official statistics are available, conservative estimates suggest that Pakistan hosts over four million orphans with many of them in care facilities¹⁶⁷

DEMAND AND SUPPLY ANALYSIS

Pakistan's low birth registration rates are caused by a number of intertwined social and economic factors, coupled with the uneven performance of the birth registration system. Children who are not 'covered' by a legal registration process largely live in rural areas; their families, especially their mothers, tend to be uneducated; most are from lower-income households; belong to minorities; are immigrants; or are frequently abandoned children. Higher levels of education among mothers relates positively with improved rates of birth registration. The exclusion of key groups of children from the birth registration system, such as refugees and those without caregivers or living in alternative care, is another leading cause of low birth registration rates. Identities are defined by ethnicity and other sub-groups, rather than state citizenship, resulting in low birth registration rates in rural areas. Parents register their children only when there is a tangible need to do so, for instance to secure school admission.

Those whose births go unregistered are especially likely to experience adverse socioeconomic conditions. For instance, at the macro level, children who remain officially 'unacknowledged' are less likely to be included in state development policies and planning for social service provision. Child rights and service delivery accountability mechanisms are also disparate, with limited outreach to communities, especially rural communities. The National Database and Registration Authority (NADRA) is the central repository of civil registration certificates, however, responsibility for the delivery of registration services rests with the local union councils at the sub-national level (union council is the lowest administrative tier).

¹⁶⁵ UNICEF, 2012. MoRES Toolkit: Indicator Selection Guidance –Child Protection

¹⁶⁶ CRC/C/PAK/CO/5, 3 June 2016

¹⁶⁷ <https://intpolicydigest.org/2015/08/26/implications-of-four-million-orphans-in-pakistan/>

Violence at the household and community levels is generally accepted, as is violence by authority figures. Prevailing social attitudes tend to view violence as a form of acceptable discipline. Harmful exploitative practices such as early marriage, child marriage and child labour are similarly socially acceptable. The available data suggests that great efforts will have to be made to achieve SDG targets on child protection, particularly SDGs 16.2, 5.2 and 5.3. Poverty and vulnerability are the two key reasons for limited compliance with child protection rights. Communities may often appear to be insensitive to the needs of children, especially to issues of protection and exploitation. Denial, stigma and social taboos exist around physical, psychological and sexual violence, while discretion or concealment is considered to be in the best interest of child. Cultural and religious attitudes make it difficult for victims to seek help. Protection concerns are compounded by the fact that legal assistance and defence are not guaranteed for all children, as well as by the absence of a public child protection case management and referral system. Harmful practices such as child marriage are widely accepted in rural areas due to cultural norms, indicating the acceptability of social norms over legislative initiatives. In addition, challenges related to the effective implementation of applicable laws for the protection of children are compounded by a lack of data to support accurate and evidence-based planning, budgeting, coordination and accountability among relevant actors.

DEVELOPMENTS IN INSTITUTIONAL, LEGISLATIVE AND POLICY FRAMEWORK

- The National Assembly of Pakistan passed the *National Commission on the Rights of the Child Act in 2017*, which mandates the Federal Government to establish a commission on the care and protection of children. The commission has recently been notified in 2020 and its responsibilities will include coordinating with provincial commissions on child rights, examining legislation and policies on child rights, and inquiring into child rights violations. The research process for this SitAn Update was unable to determine whether existing coordination bodies were active between 2017 and 2020.
- A National CRVS Policy has been approved by the Ministry of Planning Development and Special Initiatives in consultation with the provinces and territories. Health, education and other public services, as well as the media, social workers and civil society will be mobilized in providing information about the value of CRVS and encouraging the public to register vital events under this new policy. Under the CRVS Policy Identity management, civil registries, health information systems and statistical production will be linked, as stand-alone systems have considerable drawbacks in terms of inclusiveness, sustainability, use for governance functions and vital statistics production. To prevent fragmented and parallel identity management systems, efforts will be geared towards integrating and linking systems where possible, with due consideration for critical issues such as data security, privacy and confidentiality. This may include issuing unique and random personal identification numbers at birth. A National Mapping of the CRVS legislation has been completed and provincial CRVS by-laws are under revision in the provinces.

- The National Database and Registration Authority recently has begun issuing Juvenile Cards. These chip-based, 'smart' identity cards for children under the age of 18 are distinguished from Child Registration Certificates (CRC), also known as 'bay forms' (family tree forms), as Juvenile Cards are a form of 'entitled document' with multiple facilities, such as recording data on children's health, vaccination and education records.¹⁶⁸
- At the federal level, a Technical Support Unit was established in 2017 within Ministry of Planning Development and Special Initiatives for the promotion and strengthening of CRVS. National and provincial CRVS Steering Committees have been activated and meetings have been held (2018-2019). Pakistan's first CRVS International Summit was held in 2018. The national CRVS legislative mapping has been completed along with provincial studies on existing birth registration systems. Key initiatives include, CRVS Vulnerability Assessment, Multi Donor Forum formed and led by the Parliamentary Secretary at the Ministry, PC1 for CRVS ICT Model District to be approved shortly, and birth registration National Consortium formed for knowledge sharing in 2018.
- New laws have been passed to ban the use of corporal punishment to discipline children. The Gilgit-Baltistan Prohibition of Corporal Punishment Act 2015, the Prohibition of Corporal Punishment Act in 2017 in Islamabad Capital Territory, and the Sindh Prohibition of Corporal Punishment Act 2016 prohibit all forms of corporal punishment and the humiliating or degrading treatment of children in day care settings.¹⁶⁹ The Islamabad High Court (IHC) has suspended, until further notice, section 89 of the Pakistan Penal Code (PPC) that permits the use of corporal punishment by parents, guardians and teachers "in good faith for the benefit [of a child]".¹⁷⁰
- Sindh enacted the Sindh Prohibition of Employment of Children Act 2017, which establishes 15 as the minimum age for employment and 19 as the minimum age for employment in hazardous work.
- The Federal Government enacted the Islamabad Capital Territory Child Protection Act 2018 to establish a minimum age for work and hazardous work. It outlines a list of hazardous work and rules the worst forms of child labour a criminal offence in the federal capital.
- The Prevention of Trafficking in Persons Act 2018, enacted by the Federal Government, seeks to prevent and punish human trafficking, including the trafficking of women and children.
- In 2019, the Senate passed the Child Marriage Restraint (Amendment) Bill which proposes that the legal minimum age of marriage in the country be set at 18 for both girls and boys. However, the National Assembly rejected this milestone bill that would make child marriage a criminal offence. Age of marriage is still being contested in all provinces except Sindh, where the marriageable age for a girl has been increased to 18 years from 16 years.

¹⁶⁸ <<https://nation.com.pk/03-Aug-2013/nadra-all-set-to-issue-smart-cards-to-children>>

¹⁶⁹ Global Initiative to End All Corporal Punishment of Children, *Corporal punishment of children in Pakistan*, Global Initiative to End All Corporal Punishment of Children, London, March 2020, <<http://www.endcorporalpunishment.org/wp-content/uploads/country-reports/Pakistan.pdf>>, accessed 4 May 2020.

¹⁷⁰ <https://www.dawn.com/news/11534226>



- Government's focus is more on child welfare and national datasets on children are weak and even weaker on child abuse. Insufficient data is available to inform policy development, planning and programmes.
- The Juvenile Justice System Act (JJSA) 2018 has introduced several new provisions that were previously missing from the Juvenile Justice System Ordinance 2000. The Act defines a child according to the definition of UNCRC as 'a person who has not attained the age of eighteen years'. Salient features of the JJSA 2018 include the right to legal assistance at the expense of the state; observation homes (separate from police stations) where a juvenile is kept temporarily after being apprehended by police; juvenile rehabilitation centres; compulsory to make an enquiry to determine the age of the alleged offender; an alternative process of determining the responsibility and treatment of a juvenile for disposal of cases on the basis of his social, cultural, economic, psychological and educational background, without resorting to formal judicial proceedings; a juvenile shall not be charged with and tried for an offence together with an adult person; and female juveniles shall not in any circumstances be apprehended or investigated by a male police officer or released on probation under supervision of a male officer.

IMPACT OF COVID-19 ON CHILD PROTECTION

The restriction of movement, the loss of income, isolation, overcrowding and high levels of stress and anxiety are increasing the likelihood that children will experience physical, psychological and sexual abuse at home – particularly children already living in violent or dysfunctional families. The extended lockdown and restriction of access to public spaces particularly affect adolescents, friends' circles, and habitual connections which may also increase vulnerabilities. While online education has become central to the COVID-19 response, it is not only increasing the risk of cyberbullying, sexual abuse and exploitation but also increasing disparities among children who are digitally-enabled and who are not. An increase in gender-based violence, sexual exploitation and abuse are also expected, based on the experience of previous epidemics. In emergencies, undocumented children with little recourse to legal protection are at particular risk if they are separated from their parents or caregivers. Lockdown measures come with heightened risks of children witnessing or suffering violence and abuse, as violence by caregivers is the most common form of violence that children experience.¹⁷¹

One in every four women in Pakistan between the ages of 15 and 49 has experienced physical violence since they turned 15 – this already high incidence of gender-based violence is expected to increase in a crisis situation. Women may be exposed to a higher risk of domestic violence due to heightened tensions, stress and anxiety in households, especially when families are quarantined and suffering from adverse economic circumstances or unemployment.¹⁷² Domestic

¹⁷¹ United Nations, *Policy Brief: The Impact of COVID-19 on Children*, UN, New York, 15 April 2020, <https://www.un.org/sites/un2.un.org/files/policy_brief_on_covid_impact_on_children_16_april_2020.pdf>, accessed 4 May 2020.

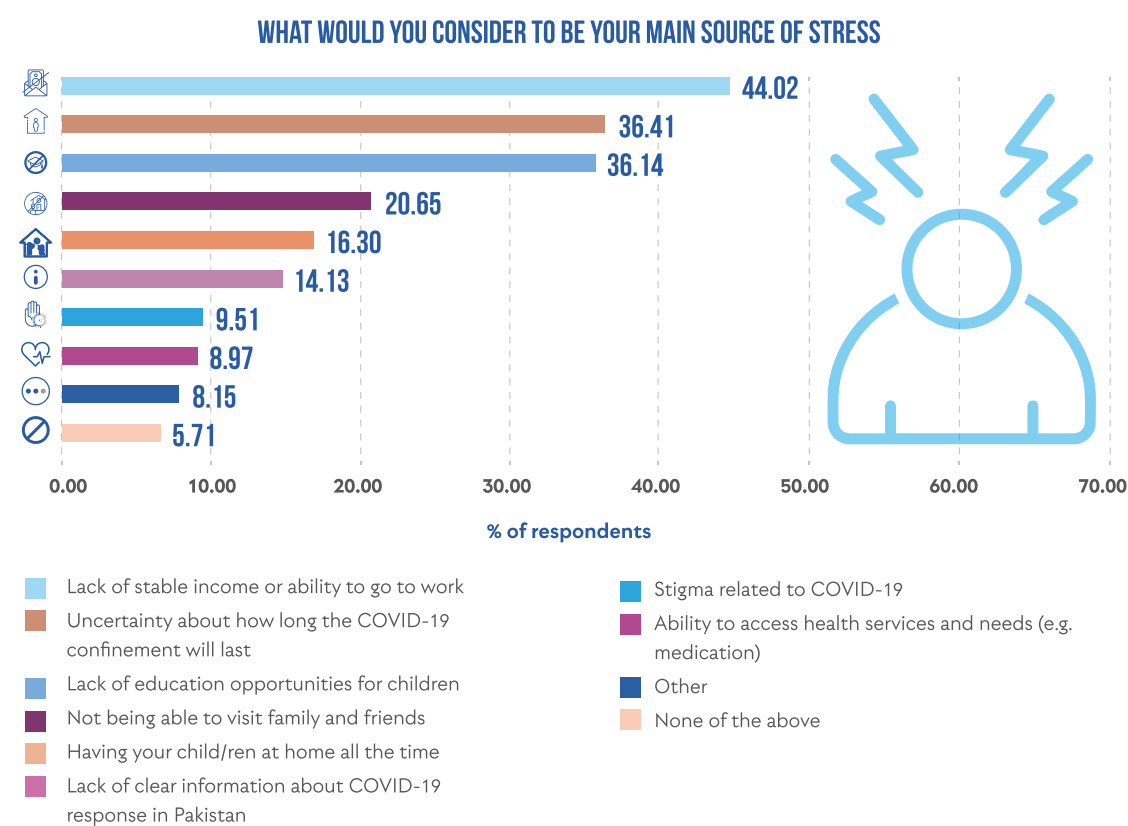
¹⁷² United Nations, *A UN framework for the immediate socio-economic response to COVID-19*, UN, New York, April 2020, <<https://unsdg.un.org/resources/un-framework-immediate-socio-economic-response-covid-19>>, accessed 4 May 2020.

violence against women appears to have increased in many countries, as detailed in the policy brief on the impact of COVID-19 on women.¹⁷³

Just as the combined effect of school closures and economic distress is likely to force more children to drop out of school in Pakistan, the same factors may force children into child labour and child marriage. Similarly, a combination of the above situations including limited opportunities for play will put more children under psychological distress with effects on their mental and psychosocial health.

A rapid study¹⁷⁴ was conducted to understand how confinement impacts children's experiences of violent discipline at home and perceptions around use of violence. Results indicate that the increase in perceived violence is high as unequal access to education opportunities and uncertainty in income levels are source of stress for caregivers.

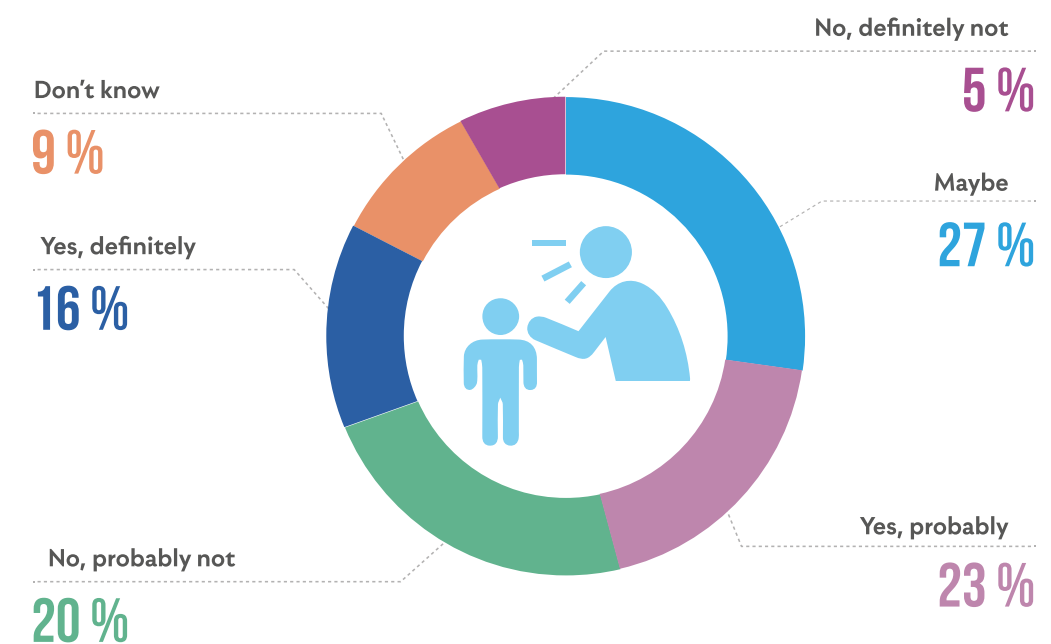
Figure 12: Main sources of stress for caregivers and perception that other people are screaming at/hitting/slapping children more now than before COVID-19



¹⁷³ United Nations, *Policy Brief: The Impact of COVID-19 on Women*, UN, New York, April 2020, <<https://www.unwomen.org/en/digital-library/publications/2020/04/policy-brief-the-impact-of-covid-19-on-women>>, accessed 4 May 2020.

¹⁷⁴ MAGENTA, COVID-19 Task force, *Impact of Confinement Study on Violence Against Children in Pakistan, Rapid Response Survey (Wave 1- June 2020)*. The survey was conducted for UNICEF.

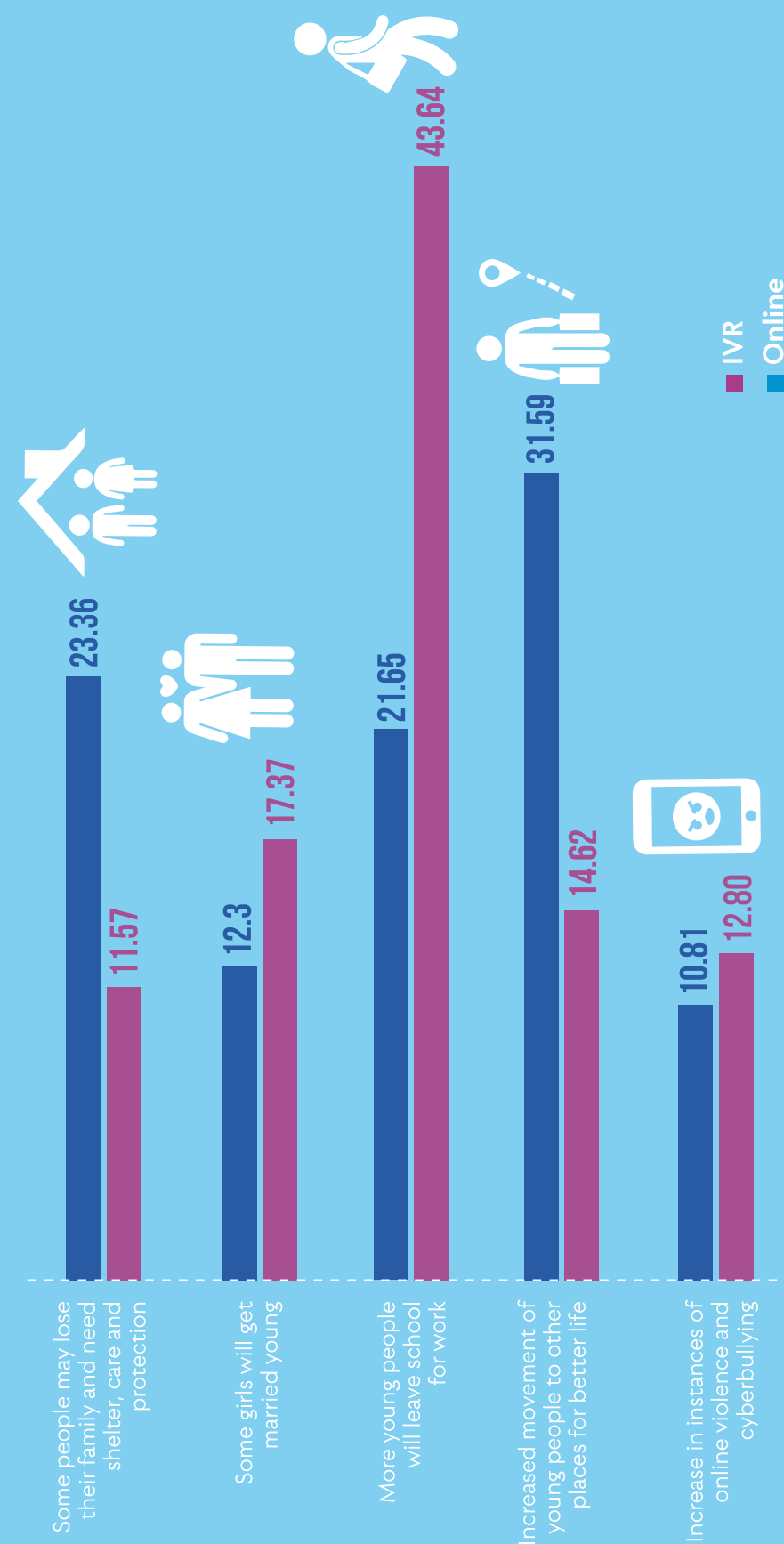
DO YOU THINK THAT OTHER PEOPLE ARE SCREAMING AT/HITTING/SLAPPING THEIR CHILDREN MORE NOW THAN BEFORE COVID-19



A Youth Perception Study (YPS)¹⁷⁵ was conducted to gain a better understanding of perceptions among youth (ages 14-29) in Pakistan with respect to COVID-19 and its associated restrictions. Over 10,000 young people were surveyed through an online or Interactive Voice Response (IVR) survey. Their responses on perceived risk to the protection of young people include losing family, shelter and protection; girls getting married young; leaving school to work; and increase in instances of online violence such as cyberbullying.

¹⁷⁵ UNICEF, UNDP, UNFPA, Viamo and Accountability Lab Pakistan, *Understanding Youth Perceptions of COVID-19*, 2020.

Figure 13: Perception of protection of young people once the coronavirus situation is under control



Response: Pakistan's governments and development partners have largely focused on ensuring infection prevention, risk communication while social protection has been emphasised as a means of addressing socio-economic needs in the context of COVID-19, through cash transfers under the *Ehsaas* programme for vulnerable groups (discussed above). However, the social protection interventions are not necessarily child sensitive.

UNICEF is working on providing mental health and psychosocial support to families and children. UNICEF is also working with youth and children's clubs and similar spaces as supporting these has critical implications for children's ability to voice their concerns and decrease vulnerability.

Global initiatives launched include the UN Development Systems (UNDS) efforts to work with national and local social services to ensure the continuity of first line response for children, women, and families at risk of violence, abuse, exploitation, neglect and family separation. The UN Trust Fund to End Violence Against Women (UNTF EVAW) will act as a rapid assessment mechanism to gather data on increased risks and the impact of violence against women due to COVID-19 in 69 countries.

There is a clear need for immediate measures to address children's risks of exposure to heightened violence, exploitation and neglect in the context of COVID-19. Measures should focus on investing in strengthening child protection systems making them robust and adaptive structures within a changing environment, and provide "multiple and creative pathways for action,"¹⁷⁶

There is a clear need for immediate measures to address children's risks of exposure to heightened violence, exploitation and neglect in the context of COVID-19, paired with long-term efforts to strengthen child protection in Pakistan. Our focus needs to be on immediate mental health and psychosocial care, curbing violence against children, and supporting vital registration for tracking and linking children to social protection.

SHORT-TERM RECOMMENDATIONS:

- Engagement of health sector for notification of births and deaths through hospital/ health facilities and other means (CRVS Model Districts).
- Protect children from the increased risk of violence, exploitation and abuse as part of the broader response to COVID-19. Support governments to ensure that COVID-19 prevention and response plans integrate age-appropriate, gender-sensitive and vulnerability measures to protect all children from violence, neglect, abuse and exploitation.
- Ensure that children and women have sustained access to social service workforce as essential during pandemic, including protection and psychosocial support services and legal services. Ensure that national budgetary allocations are not diverted from key children's services to fund the response or mitigate its wider economic impact.
- Provide child protection case management and emergency alternative care arrangements.

¹⁷⁶ Chapin Hall (2010)

- Support family based systems to ensure children are cared for in family settings and care institutions are a place of last resort.
- Ensure social protection for the most vulnerable children and households in Pakistan.
- Communicate with, and engage, parents, caregivers and children themselves with evidence-based information and advice on child protection and preventing abuse.
- Set up nationwide helplines, school counsellors and other child-friendly reporting mechanisms to encourage children in distress to reach out for help when they need it.
- Work towards online safety from abuse and that the digital learning and the provision of learning materials are undertaken through safe e-education platforms.
- Conduct a rapid assessment of data on the COVID-19 outbreak and emergency response (disaggregated by sex and age) to understand trends in exposure to violence, exploitation, abuse and neglect. Use this assessment to develop responsive prevention and redress measures. This should include the analysis of quantitative data on pre- and intra-COVID-19 trends on all relevant indicators on violence and exploitation of women and children (such as children covered by social protection, children aged 5 to 17 who are engaged in child labour, children who have experienced any physical punishment and/or psychological aggression by caregivers, children under-five whose births are registered, and early marriage among women).
- Develop and launch a nationwide campaign that raises awareness about the negative effects of the pandemic on women and children. This campaign should especially target men, including men in low-income settings. It should also feature real-life religious leaders as influencers.
- Prioritise social protection hotlines and services for girls and women as “essential”, and sensitise law enforcement on the need to be responsive to calls from victims of domestic violence.
- Work with youth and children’s clubs and similar spaces as supporting these has critical implications for children’s ability to voice their concerns and decrease vulnerability.

MEDIUM-TERM RECOMMENDATIONS:

- Support the Government to strengthen its Civil Registration and Vital Statistics (CRVS) capacities, particularly on births and deaths, to be able to take stock of the pandemic’s impact and increase rates of birth and death registration emphasizing continuity of civil registration as an essential service.
- Set up civil registration offices and/or services in main health facilities, where most births and deaths occur.
- Provide capacity development for social workers, teachers and caregivers.
- Linking vulnerable children and their families to strengthened social protection schemes to prevent exploitative and harmful practices.
- Undertake risk assessment studies post-COVID-19 to gauge the scale of its direct and indirect impact, and the efficacy of Pakistan’s response, including on child protection issues.

- Support government departments to improve areas where response was weak during the pandemic, including due to limited capacities and resources.
- Establish mechanisms to ensure that women and children who need medical and education services can access rehabilitation and social reintegration initiatives after experiencing COVID-19-related infection or trauma.

EVERY CHILD LIVES IN A SAFE AND CLEAN ENVIRONMENT

SDG 6 (‘clean water and sanitation’) focuses on ensuring the availability, sustainability and management of water and sanitation for all. To enhance water, sanitation and hygiene (WASH), target 6.1 centres on universal and equitable access to safe and affordable drinking water for all, and target 6.2 on access to adequate, equitable sanitation and ending open defecation, paying special attention to the needs of women, girls and vulnerable groups. Target 6.3 commits states to improving water quality by reducing pollution, eliminating dumping, minimizing the release of hazardous chemicals and materials, halving the proportion of untreated wastewater, and substantially increasing recycling and safe reuse.



Pakistan has yet to achieve universal and equitable access to safe and affordable drinking water, especially in rural areas where improper water treatment practices are rife. Nevertheless, the country has made considerable progress on WASH indicators – including the availability, accessibility and quality of improved drinking water, and the use of improved sanitation facilities. As of 2019, 92 percent of population have access to improved drinking water, and 70 percent had access to improved sanitation facilities.¹⁷⁷ Nevertheless, the use of improved water has declined marginally in urban settings, improved sanitation is far less common in rural than in urban settings, and open defecation remains fairly widespread, largely practiced by poor in rural areas.

Pakistan is committed to achieve localized targets on SDGs on safe drinking water services and sanitation services by 2030. In addition, all provinces have made the commitment of making Pakistan Open Defection Free by 2025.

Table 5: SDG Targets for 6.1 and 6.2 for Pakistan

	Safely Managed Water (%)	Safely Managed Sanitation (%)
Balochistan	100	100
KP	100	47
Punjab	100	100
Sindh	82.5	64
Pakistan	95	72

Source: Joint Sector Review, Drinking Water, Sanitation and Hygiene (WASH), Pakistan. January 2019

Safely-managed water

The right to water is not just about access. If water is not safe to drink, if it is far away and unreachable, and if it is not available when needed, then people’s basic right to water cannot be realized. The safely managed water under SDGs comprises of water accessible at premises, available when needed, and free from contamination. This is why three main factors must be assessed to gauge the deprivations that households, and their children, face to achieving their right to water: availability, accessibility and quality.

Around 92 percent of population in Pakistan has access to improved water source, 77 percent has it available when needed (available at premises), 56 percent have access to basic service and 35 percent of the population have access to safe water which is free from contamination. The

177 WHO & UNICEF, *Joint Monitoring Programme for Water Supply, Sanitation and Hygiene, Estimates on the use of water, sanitation and hygiene in Pakistan*, June 2019.

lowest figure of all three is safe water free from contamination i.e. 35 percent, which is the baseline for safely managed water in Pakistan in 2018.¹⁷⁸

Availability: Pakistan is well endowed with water – only 16 countries in the world have more water. But because Pakistan is the world’s sixth most populous country, water availability per person is comparatively low.¹⁷⁹ Less than 10 percent of the global population lives in countries with less water per person than Pakistan. Climate change is the biggest long-term and, currently, unmitigated external risk to Pakistan’s water sector. Climate change is not expected to greatly alter average water availability over the coming decades, but inflows will become more variable between and within years, increasing the severity of floods and droughts. As per the Pakistan Meteorological Department (PMD), severe drought-like conditions have emerged across much of southern Pakistan. These are expected to deteriorate further over the next four years. In 2018, rainfall decreased during the monsoon season (May to August) – with rainfall 69.5 percent below average in Sindh, and 45 percent below average in Balochistan – causing acute shortages of water, food and fodder. The Government estimates that 5 million people are affected by drought in 26 districts of Sindh and Balochistan.¹⁸⁰ Climate

178 WHO & UNICEF, *Joint Monitoring Programme for Water Supply, Sanitation and Hygiene, Estimates on the use of water, sanitation and hygiene in Pakistan*, June 2019.

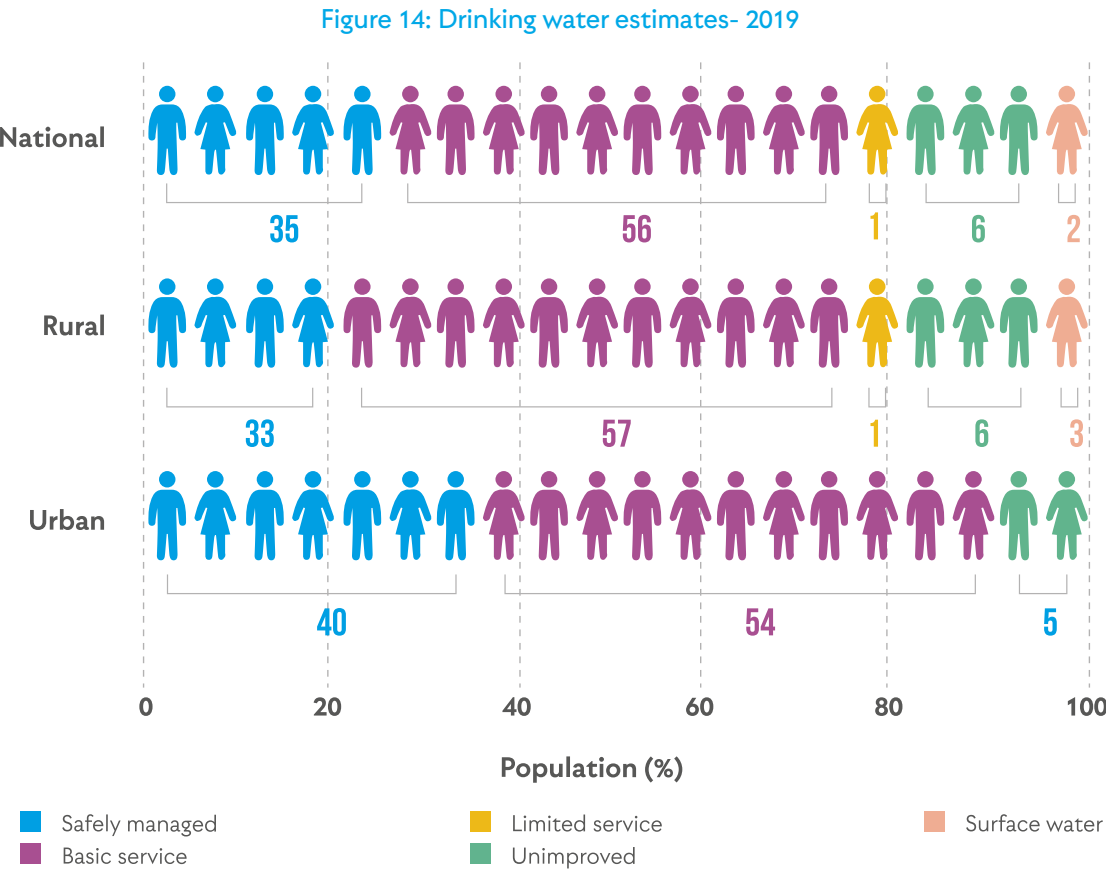
179 Young, William, et al., *Pakistan: Getting More from Water*, World Bank, Washington D.C., 2019, <<http://documents.worldbank.org/curated/en/251191548275645649/Pakistan-Getting-More-from-Water>>, accessed 4 May 2020.

180 United Nations Children’s Fund, Country Programme, *UNICEF Pakistan Country Programme 2018–2022*, UNICEF, Islamabad, 2019, <<https://www.unicef.org/pakistan/reports/country-programme-cooperation-between-government-pakistan-and-unicef-2018-2022>>, accessed 4 May 2020; United Nations Children’s Fund, *WASH Programme Strategy Note*, UNICEF, Islamabad.



change is expected to drive up water demands by between 5 and 15 percent by 2047, in addition to the demands of an increasing population and growing economy. Pakistan’s per capita water availability has fallen from 1,306 cubic meters per inhabitant in 2015 to just 1,100 cubic meters per inhabitant in 2019. This is forecast to decline further to approximately 500–600 cubic meters per inhabitant by 2040.¹⁸¹

Tube wells and boreholes remain the most common sources of drinking water in Pakistan (55 percent), followed by piped water (24 percent). The former is the most common source of water in both urban (37 percent) and rural areas (65 percent). The vast majority (87 percent) of households that use piped water or water from a tube well or borehole reported that water was available without interruption of at least one day. The availability of water without interruption is higher in rural areas (92 percent) than in urban centres (78 percent).¹⁸²



181 Young, William, et al., *Pakistan: Getting More from Water*, World Bank, Washington D.C., 2019, <<http://documents.worldbank.org/curated/en/251191548275645649/Pakistan-Getting-More-from-Water>>, accessed 4 May 2020.

182 National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

Accessibility: Access to water improved and 92 percent of Pakistan’s household now have access to improved drinking water compared to 91 percent in 2015. Access to improved water sources in rural areas is 91 percent compared to 95 percent in urban settings. 73 percent have drinking water on their premises, while only 10 percent have to spend 30 minutes or more to access water, and 17 percent spend less than 30 minutes to reach their nearest source of clean water.¹⁸³

Quality: Water is provided to households through different water channels, with tube wells and boreholes still being the highest source of water provision. Post 18th amendment, water and sanitation management has become a provincial subject, with each province having an autonomous water and sanitation authority. In certain parts of the city this job is also managed by the local governments. Despite 92 percent of population having access to improved water sources, only 35 percent has access to water that is free from bacterial contamination.¹⁸⁴

In Punjab, 94.4 percent of households use improved sources of drinking water, considerably more so in rural areas (97 percent) than in urban centres (89 percent). Major cities have the lowest proportion of households that use improved water sources (87 percent). More than half (59.6 percent) of households are at risk of faecal contamination in the water that they drink, while 36.2 percent are at risk from their source of drinking water.¹⁸⁵ In Khyber Pakhtunkhwa, water access is almost as high as in Punjab, with 91.3 percent of households using improved sources of drinking water.¹⁸⁶

Table 6: Provincial and regional statistics on safely managed water

	Improved and	Available on	Free from
Pakistan	87	77	36
Khyber Pakhtunkhwa	69	73	53
Punjab	95	82	35
Sindh	86	75	19
Balochistan	58	37	19
Gilgit Baltistan	79	63	
Azad Jammu and Kashmir	52	22	

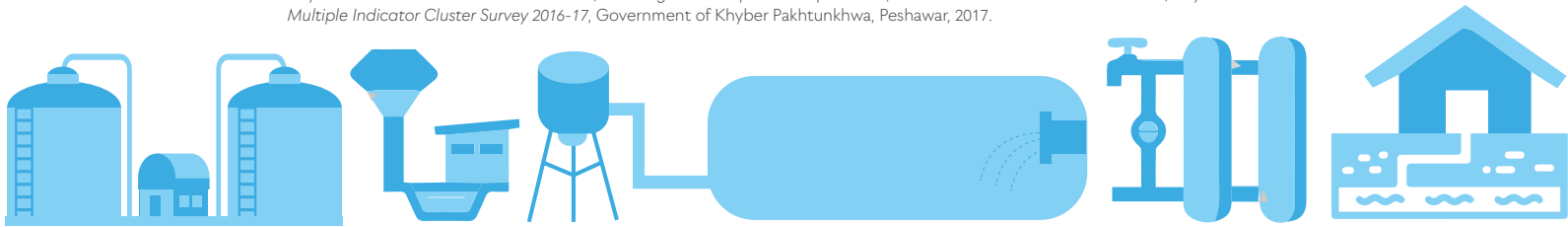
Source: WHO/UNICEF JMP (2019)

183 National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

184 WHO & UNICEF, *Joint Monitoring Programme for Water Supply, Sanitation and Hygiene, Estimates on the use of water, sanitation and hygiene in Pakistan*, June 2019.

185 Punjab Bureau of Statistics and United Nations Children’s Fund, *Multiple Indicator Cluster Survey 2017–2018*, Punjab, Government of Punjab, Lahore, 2018.

186 Khyber Pakhtunkhwa Bureau of Statistics, Planning & Development Department, and United Nations Children’s Fund, *Khyber Pakhtunkhwa Multiple Indicator Cluster Survey 2016–17*, Government of Khyber Pakhtunkhwa, Peshawar, 2017.



SAFELY-MANAGED SANITATION SERVICES

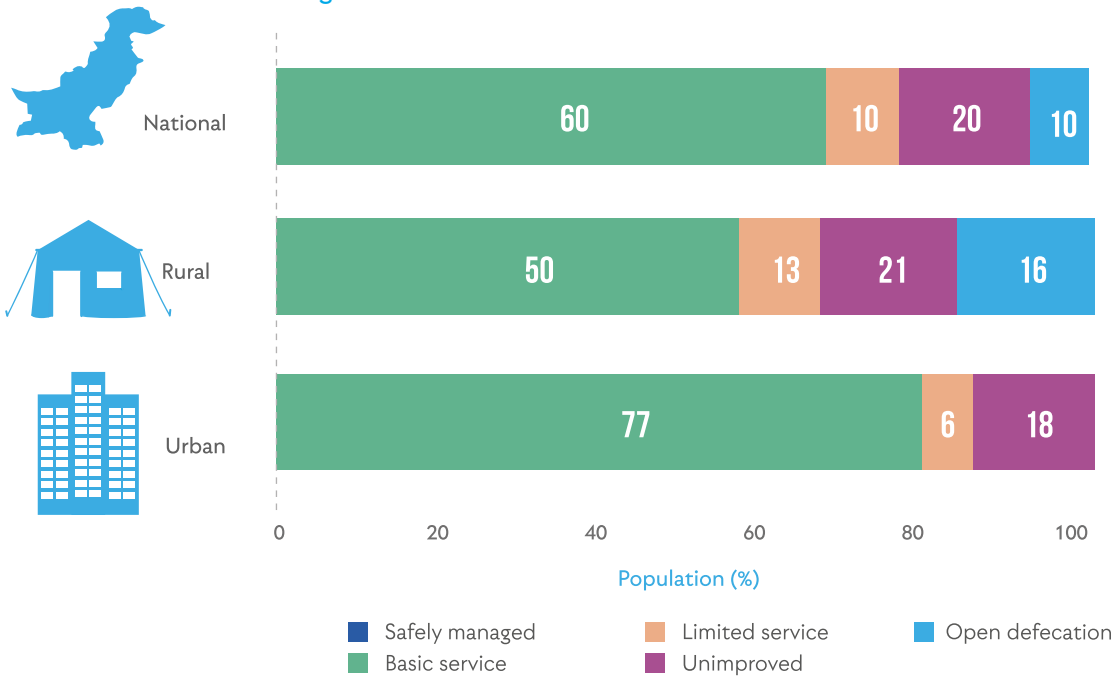
Sanitation remains a core focus area for development partners and the Government of Pakistan. Both aim to reduce the practice of open defecation as much as possible, and to provide improved sanitation facilities to areas where these are lacking. Pakistan’s commitment to SDG in terms of open defecation is to completely eradicate the practice by 2030. The safely managed sanitation services include: improved sanitation (latrines), on-site and off-site disposal/treatment along with hand washing with water & soap.

According to JMP estimates in 2019, about 60 percent of the population in Pakistan have basic sanitation, 10 percent with limited service, 20 percent fall under unimproved and around 10 percent practice open defecation. Half of the rural population in Pakistan use basic sanitation services compared to 77 percent in urban areas. Sharp contrast is also visible between income groups as only 16 percent of poor have basic sanitation services, while 63 percent practice open defecation compared to 90 percent of the richest with basic sanitation service and zero open defecation practice.¹⁸⁷ A considerable number of latrines are either connected to open drains or unlined drains leading to nearby fields or pit/dry raised latrines. Similarly, latrines are not regularly cleaned especially in rural areas due to many reasons including lack of sufficient water, etc. There is need for necessary legislative reforms for water and sanitation especially regarding connection of latrines with appropriate systems that provides either on-site or off-site treatment options.

¹⁸⁷ WHO & UNICEF, Joint Monitoring Programme for Water Supply, Sanitation and Hygiene, Estimates on the use of water, sanitation and hygiene in Pakistan, June 2019.



Figure 15: Sanitation services estimates- 2019



Source: WHO/UNICEF JMP (2019)

At provincial level, data updated since 2017 is only available for Punjab and Khyber Pakhtunkhwa. In Punjab, 80 percent of households use an improved sanitation facility which is not shared, as do 78 percent of those in Khyber Pakhtunkhwa. Open defecation remains a major challenge in both provinces, with 13 percent of Punjab's households (54.4 percent of its poorest households) and 8.7 percent of those in Khyber Pakhtunkhwa (35.5 percent of its poorest families) practicing open defecation. However, progress on ending open defecation appears to be gaining pace, improving by 12.3 percentage point in Khyber Pakhtunkhwa and 4.5 percentage point in Punjab compared to the last MICS data for both provinces. A gender analysis of the use of sanitation facilities could not be conducted due to a lack of data.

Handwashing facility was observed with 92.7 percent of households, while 68.6 percent of the households in which place for handwashing was observed had water and soap for hand washing.¹⁸⁸

WASH IN SCHOOLS

73 percent of primary schools in Pakistan have drinking water availability, this increases with the level of education as middle, secondary and higher secondary level schools have 85 percent, 92 percent, and 97 percent availability of drinking water. Availability of toilets for students is in 73 percent of primary schools, 88 percent of middle schools, 93 percent of secondary schools and 97 percent in higher secondary schools.¹⁸⁹

DEMAND ANALYSIS

Awareness around water use, safety, maintenance and conservation is generally lacking in Pakistan and water which appears clean is considered to be safe. Additionally, the notion that water is free and infinite resource leads to no accountability on the part of its users and is due to the fact that water is not generally metered or charged for. At the institutional level, no accountability mechanism exists to curb inefficient water use. Key bottlenecks at the household and community levels include limited availability of water and soap for hand washing, which is highly related with poverty. It is not uncommon for people in rural areas to spend considerable time fetching water as they do not have water facilities close to their homes. This task is typically performed by women and girls, often forced to walk for over 30 minutes through difficult terrain to obtain water for their households.

The practice of open defecation represents both a considerable health risk and a protection risk. It enables diseases to thrive, while exposing women and children to the prospect of violence. In poverty stricken rural areas, open defecation is a cultural norm and accepted social behaviour, and not having toilet facilities is common. Most of the people in Pakistan who do not have access to a toilet live in poor rural households or insecure, informal urban settlements are the hardest

¹⁸⁸ National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

¹⁸⁹ National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, *Pakistan Education Statistics, 2016–17*, Government of Pakistan, Islamabad, 2018, <<http://library.aepam.edu.pk/Books/Pakistan%20Education%20Statistics%202016-17.pdf>>, accessed 4 May 2020.

populations to reach. The concept of hygienic latrines is also absent in many cases, especially in terms of the construction of septic tanks in urban and peri-urban areas, where most latrines are connected to either covered or open drains. The absence of adequate drinking water and WASH facilities in schools undermines children's right to access education. This is particularly pronounced for girls, as a lack of safe, separate toilet facilities may prevent families from sending girls to school. The absence of menstrual hygiene management (MHM) facilities, or information on MHM practices is sorely needed to enhance girls' enrolment, attendance and retention. There is limited participation of religious leaders, media, youth and celebrities in awareness on WASH. The perspective and requirements of WASH financial investment have changed with Pakistan's endorsements for indicators of safely-managed drinking water and sanitation under SDGs. It is estimated at PKR 450 billion annually to meet SDG targets by 2030.

WASH SYSTEMS AND DELIVERY

Ministry of Climate Change (MoCC) at federal level is responsible for policy formulation, standards settings, reporting and coordination for regional and international commitments for drinking water and sanitation. In the provinces, local government and public health engineering departments are leading on the identification, planning, implementation and monitoring of drinking water and sanitation initiatives. However, there are multiple layers and ambiguities in the role of key departments in the service delivery and O&M at the provincial and district levels. The prime responsibility for service provision lies with elected local councils and local government department, however, in practice, the provincial departments are playing a leading role in resource allocations, project identification and service delivery including O&M of large schemes. The operation and maintenance of water supply and sanitation schemes is assigned to local councils in urban areas, while this responsibility in rural areas is entrusted to local community groups organized as community based organizations in majority of the provinces. In short, the service delivery for water and sanitation in Pakistan involves numerous actors working for a common goal but apparently in different directions.

There is a need of providing further clarity and facilitation in development of guidelines for local councils for WASH services so informed capacities are added and developed at appropriate levels. Social mobilization through engagement of local community based organizations has been partially successful to the extent of ending open defecation and limited component sharing but has not yet resulted in creating safely managed sanitation services across the country.

In October 2018, the Government launched the Clean and Green Pakistan Movement, in part to create an enabling environment for water and sanitation, including institutional strengthening for effective service delivery, and behavioural change to ensure the sustainability and continuity of available, accessible safe water and sanitation. MoCC also created a Water, Sanitation and Hygiene (WASH) Strategic Unit in the same year. The Clean Green Pakistan Movement has strongly emphasized on institutional strengthening and behavioral change for rolling out safe

water, sanitation and plantation in the country. The slow progress on the front of safe water and sanitation is linked with capacity issues on part of the service providers. This has been highlighted as a key challenge in provincial WASH Sector Development Plans and Joint Sector Reviews held in all the four provinces.

RECENT INSTITUTIONAL, LEGISLATIVE AND POLICY DEVELOPMENTS

Pakistan's *National Water Policy* was approved in 2018 after years in the making. Anchored in the paradigm of integrated water resource management, the policy outlines a framework for interventions by federal and provincial governments to address issues that are driving the declining supply and deteriorating quality of water. The policy also calls for a substantial increase in public investments in the water sector by the Federal Government – advocating for a rise from 3.7 percent in

2017–18 to at least 10 percent by 2018–19 and 20 percent by 2030 – and the establishment of an apex body to approve legislation, policies and strategies for water resource development and management.

Regional developments include the introduction of the *Punjab Water Act 2019*. Under this act, the provincial government has formed a commission responsible for conserving, redistributing or otherwise augmenting water resources; allocating water resources for domestic, agricultural, ecological, industrial or other purposes; securing the proper use of water resources; and establishing an Advisory Committee to advise the maintenance and development of water resources. A similar Water and Sanitation Commission was also established in Sindh. However, it was dissolved at the order of the Sindh High Court in early 2020.



Since 2011, Punjab, Khyber Pakhtunkhwa, Sindh and Balochistan have approved provincial drinking water policies. Provincial sector plans have been developed: Punjab WASH Sector Plan 2014-2024, Balochistan WASH Sector Plan 2015-2025, Sindh WASH Sector Plan 2016-2026 and KP WASH Sector Plan which initiated in 2018.

The process of WASH Joint Sector Reviews (JSRs) were rolled out in Pakistan in 2016. MoCC in collaboration with UNICEF and sector partners issued guidelines to the provincial departments for rolling out and cascading WASH-JSRs in their respective provinces. In 2017-2018, all four provinces conducted JSRs with the intention to integrate JSRs as essential component of country planning while strengthening the government leadership. In KP province, the recommendations of JSR have been integrated into 'Peshawar Declaration' of November 2017. Similarly, the provincial governments endorsed WASH-JSR Report 2017, along with targets set for SDG 6.1 and 6.2 for the province up to 2030.

IMPACT OF COVID-19 ON WASH

WASH is central to infection prevention and control and provision of safe water, sanitation and hygienic conditions is essential to protecting human health during all infectious disease outbreaks, including the COVID-19 outbreak.

The lockdown is likely to exacerbate WASH challenges in Pakistan, especially as 10 percent of its population already practices open defecation. As more members of such households are obliged to stay at home, instead of going out to work, the question of

WASH facilities has become urgent. Funds earmarked for WASH development activities may be diverted towards managing COVID-19, as interventions on COVID-19 predominantly target health care facilities, isolation centres, quarantine sites and at-risk communities. This will affect the most vulnerable people in Pakistan, putting them at a higher risk of other diseases in the continued absence of WASH facilities. In addition, as household incomes plummet during the lockdown – especially among households dependent on daily wages to make ends meet – access to WASH facilities and adequate WASH practices are likely to decrease.

A study by Xiao et al., found evidence of the COVID-19 virus in the stools of over 50 percent of patients. The study further observed that in more than 20 percent of severe acute respiratory syndrome coronavirus (SARS CoV-2) patients, the viral RNA remained positive in faeces even after the negative conversion of the viral RNA in the respiratory tract, indicating that viral gastrointestinal infection and potential faecal-oral transmission can last even after viral clearance in the respiratory tract. These findings make the role of having sound WASH facilities and strategies especially important for fighting the outbreak.

Response: The stimulus package of PKR 1.2 trillion presented by the Federal Government targets the economic survival of the most vulnerable members of society. However, it pays little or no attention to WASH. However, MoCC has developed WASH Sector COVID-19 Preparedness and Response Strategy Apr-Dec 2020. WASH Sector response is aligned with the overall Government of Pakistan Action

Plan for COVID-19 response plan and Global Humanitarian Response Plan (GHRP). WASH sector response focuses on the immediate measures that must be undertaken to ensure preparedness to address the COVID-19 outbreak. It also builds on one of the pillars of Pakistan's COVID-19 Strategic Preparedness and Response plan i.e. Infection Prevention and Control (IPC). Service delivery will be prioritized in institutions and community places (HCF managing COVID19 cases, isolation centers and quarantine sites, schools-if used as quarantine centres, mosques during Ramadhan) and at-risk communities, especially in urban poor and congested communities, to ensure the functional WASH services such as safe drinking water, gender segregated toilets, hand washing stations, training of health care facility staff on disinfection and waste management/ segregation and safe disposal.

WASH Sector response on IPC will mainly focus on all components of Clean Green Pakistan programme. There are five pillars under Clean and Green Pakistan with a focus on behavioral change and institutional strengthening. These five pillars are: Plantation, Solid Waste Management, Liquid Waste/ Hygiene, Total Sanitation and Safe Water. Hence, this reassures and shows high level of political commitment for the cause of clean and green Pakistan, where water and sanitation are integral component.

Development partners have also come forward to spearhead WASH interventions in Pakistan in the context of the pandemic. WASH Sector partners are working on a nationwide awareness raising campaign in regional and local languages on the importance of handwashing with soap and social distancing, with a view to preventing the spread of COVID-19. By using FM radio, SMS and local cable networks, the campaign will reach out to 22.6 million people across Pakistan.¹⁹⁰

Due to the rapid rise in COVID-19 cases in Pakistan, and the limited testing facilities available, WASH management will play a vital role in stemming the spread of virus. Governments and their partners must raise awareness of WASH practices, provide essential WASH services, facilities and equipment. Several measures can improve water safety, starting with protecting the source water; treating water at the point of distribution, collection or consumption; and ensuring that treated water is safely stored at home in regularly cleaned and covered containers. Such measures can be effectively planned, implemented and monitored using water safety plans. Additionally, IPC through WASH support to health facilities, quarantine and isolation centres and in the communities, would be instrumental in containing the outbreak. WASH sector coordination mechanism both at federal and provincial level, including public sector as well as implementing partners, are key to plan and implement the COVID-19 response.

SHORT-TERM RECOMMENDATIONS:

- Strengthen public awareness raising campaigns to explain how COVID-19 spreads and how WASH can help to prevent this. Ensure that campaigns highlight why and how to practice physical social distancing, frequent handwashing, and good hygiene practices, as well as

¹⁹⁰ WaterAid, 'COVID19 and hygiene', WaterAid, London, 2020, <<https://www.wateraid.org/pk/covid19-and-wateraid>>, accessed 4 May 2020.

explaining how to conserve water while washing one's hands to prevent water shortages. Follow all SOPs while engaging with communities and use of digital forum.

- Safe WASH services in health care facilities (HCFs) to deliver quality health services; protect patients, health workers, and staff; and to prevent further transmission.
- Set up handwashing facilities near public places, such as markets, train and bus stations, and hospitals.
- Disseminate vital equipment – such a portable water tanks and soap – especially in health care facilities and across urban areas as well as remote rural areas where open defecation is practiced, to ensure that faecal contamination does not result in the spread of COVID-19.
- Sensitize first responders – such as frontline health worker or sanitary worker – on the faecal-oral route of transmission of COVID-19, particularly in areas where open defecation is practiced.
- Scale up the WASH intervention in schools and hand washing initiatives for safe re-opening of schools after lockdown is lifted.

MEDIUM-TERM RECOMMENDATIONS:

- MoCC is the primary duty bearer for WASH at federal level. Integrate climate resilience and WASH as integral components of future interventions.
- Provision of capacity development of decentralized tiers in terms of both, human resources and management systems.
- Ensure that municipalities allocate funds for improved water supply and waste disposal infrastructure, the regular monitoring of water supplies, and ensuring that water supplies are contamination-free.
- Support municipalities to ensure the proper treatment and management of municipal wastewater, as per national and provincial environmental regulations.
- Devise a plan of action to manage faecal waste in rural areas in order to prevent the contamination of water supplies, as well as to promote integrated, responsive water and sanitation decision-making nationwide, at all levels.
- Develop a revised WASH implementation plan once the pandemic has subsided. Ensure that the plan examines setbacks caused by the redirection of WASH-related funds and, based on this analysis, devises a strategy to bridge the gap between the current scenario and original targets.



CHAPTER 4:
CROSS-CUTTING THEMES
- EVERY CHILD HAS AN
EQUITABLE CHANCE IN LIFE

CROSS-CUTTING THEMES - EVERY CHILD HAS AN EQUITABLE CHANCE IN LIFE

GENDER DISPARITIES AND INEQUALITY

Despite major progress on maternal and child health, women and children from disadvantaged groups will remain behind by 2030. Cultural norms in Pakistan perpetuate women’s marginalization, causing women to have less access and control over health care resources, and to be disadvantaged in decision-making on their own health. As a result, women in Pakistan tend to have poor health outcomes. These range from a high maternal mortality ratio, to high rates of malnutrition, communicable and non-communicable diseases. According to UNDP’s Human Development Report 2019, as discussed above, Pakistan’s ranks 152 of 189 countries, ranking low both on the Gender Development Index (GDI) with an index value of 0.747 (0.742 in 2016) and the Gender Inequality Index (GII), with a value 0.547 (0.546 in 2016). These results reflect entrenched gender inequalities in Pakistan, and only marginal improvements on key gender-related indicators in recent few years. Pakistan’s GII ranking has worsened, falling to 136 in 2019, from 133 in 2017. Pakistan still has low GDI and GII compared to other countries in the region.

Table 7: HDI, GDI and GII comparison with countries in the region

Country	GDI	HDI Female	HDI Male	GII	GII Rank	MMR per 100,000 live births
Pakistan	0.747	0.464	0.622	0.547	136	178
Bangladesh	0.895	0.575	0.642	0.536	129	176
India	0.829	0.574	0.692	0.501	122	174

Source: HDR 2019

On the Inequality-adjusted Human Development Index (IHDI),¹⁹¹ Pakistan’s performance in 2019 was assigned an index value of 0.386, far lower than its overall HDI value of 0.560. This indicates gross gender inequality in human development in the countries.¹⁹² Gender inequalities in the allocation of resources – alongside income, education, health care, nutrition and a political voice – are the key factors associated with poor the health and reduced well-being of women around the world.

191 The 2010 HDR introduced the IHDI, which takes into account inequality in all three dimensions of the HDI (Long and healthy life, knowledge and a decent standard of living) by ‘discounting’ each dimension’s average value according to its level of inequality. The IHDI is basically the HDI discounted for inequalities.

192 United Nations Development Programme, Briefing note for countries on the 2019 Human Development Report: Pakistan, UNDP, New York, 2019, <http://hdr.undp.org/sites/all/themes/hdr_theme/country-notes/PAK.pdf>, accessed 4 May 2020.

According to the Institute for Health Metrics and Evaluation, which analyses data on ‘years of life lost’ (YLL), maternal deaths, neonatal deaths and diseases caused by poor nutrition topped the list of causes for premature deaths in 2017.¹⁹³ Dietary iron deficiency was among the top-five leading causes of disability adjusted life years (DALYs) among women of reproductive age in 2017.¹⁹⁴

Gender inequalities affect girls and boys differently. Deprivations that are experienced more keenly by girls than boys are largely grounded on gender norms that put girls at a disadvantage, including cultural practices, family attitudes and unequal power between women and men in society. Overall, girls are more likely than boys to face greater levels of deprivations. Only 63 percent of girls between 12 and 23 months old receive all their basic vaccinations, compared to 68 percent of boys. Gender disparities abound in education, with more girls out of school (12.16 million) than boys (10.68 million) from the primary to the higher secondary levels. Enrolment and attendance rates are consistency lower among girls, as reflected in pre-primary GER (78 percent for girls compared to 89 percent for boys) to primary NAR (55.4 percent for girls vs 61.4 percent for boys). According to MICS data, girls from Punjab’s poorest households are at a particular disadvantage, with an NAR of just 10.2 percent and 5.5 percent at the lower and upper secondary levels, respectively. Girls

193 Institute of Health Metrics and Evaluations, ‘Pakistan’, IHME, University of Washington, 2018, <http://www.healthdata.org/pakistan>, accessed 4 May 2020.

194 Murray, Christopher J.L., ‘Global, regional, and national disability-adjusted life-years (DALYs) for 359 diseases and injuries and healthy life expectancy (HALE) for 195 countries and territories, 1990–2017: a systematic analysis for the Global Burden of Disease Study 2017’, *The Lancet Europe PMC Funders Author Manuscripts*, vol. 392, no. 10159, 2018, pp. 1859–1922, <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC6252083>, accessed 4 May 2020.



are more likely than boys to drop out of education – nationwide, 21.8 percent of girls in Class 6 drop out of school, compared to 18.4 percent of boys. Primary GPI currently stands at 0.92,¹⁹⁵ but declines to 0.89 at the middle and secondary school levels. Youth literacy GPI is a mere 0.81, reinforcing the finding that access to education is more limited for girls than boys. Reasons for this include the limited availability of girls' middle and secondary schools at the community level, alongside household challenges – particularly financial constraints – and the expectation that girls must contribute heavily to domestic and care work.¹⁹⁶

On a number of indicators, boys are more deprived than girls. For instance, boys appear to be more affected than girls by all form of malnutrition. 41 percent of boys are stunted compared to 39 percent of girls. While 28.6 percent of children under-5 suffer from iron deficiency anaemia, a slightly higher proportion of boys (29.1 percent) suffer from this micronutrient deficiency than girls. Boys are also more likely to be exploited as child labourers outside the home. Among children aged 10 to 14, 9.82 percent of boys are currently engaged in child labour, compared to 6.4 percent of girls. This may be tied to norms that expect boys to be breadwinners, prompting households to oblige boys to earn an income as soon as possible.

A number of other gendered inequalities are worth highlighting. Pakistan's high maternal mortality rate (MMR) is a particular concern. Although this has improved – declining from 276 maternal deaths per 100,000 live births in the past, to 178 deaths per 100,000 live births – it remains a long way from the aim of Pakistan's *Vision 2025*: of less than 40 maternal deaths for every 100,000 live births. As discussed above, most maternal deaths are due to preventable causes. Poverty is another key concern for women, who comprise a disproportionate portion of the poor in Pakistan. The highest fertility rates (4.9 children per woman) are among women from the poorest wealth quintiles in the country. Moreover, rates of early pregnancy and childbearing have remained unchanged since 2012–13, with 8 percent of adolescent women aged 15 to 19 years bearing their first child, largely within the country's lowest wealth quintiles.¹⁹⁷ Despite government efforts to address inequity and injustice in access to essential health care services, the key reasons for high MMR, IMR and U5MR are embedded in bio-demographic risk factors (the sex of the child, the age of the mother at birth, birth intervals and birth spacing) as well as socio-demographic risk factors (urban-rural location, the mother's level of education and the wealth quintile to which her household belongs).¹⁹⁸

Practices that expose Pakistan's children to violence and harm – such as child labour and child marriage – are rooted in deeply entrenched gender norms regarding the expected roles, and

195 Pakistan Bureau of Statistics, *Pakistan Social & Living Standards Measurement Survey 2018–19*, Ministry of Planning Development & Special Initiatives, Government of Pakistan, Islamabad, <http://www.pbs.gov.pk/sites/default/files/pplm/publications/pplm_hies_2018_19_provincial/key_findings_report_of_pplm_hies_2018_19.pdf> accessed 22 June 2020.

196 Kamran, Iram, Tahira Parveen, Maqsood Sadiq and Rehan M. Niazi, *Adolescent girls' voices on enhancing their own productivity in Pakistan*, Population Council, Islamabad, 2018, <https://knowledgecommons.popcouncil.org/departments_sbsr-pgy/794/>, accessed 4 May 2020.

197 National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

198 Ibid.

value accorded, to boys and girls. For instance, practices that grossly disadvantage girls persist, such as marriages to settle blood feuds (*vani/swara*) and so-called 'exchange marriages' (*watta satta*).

It is also important to note that women's participation in Pakistan's labour market is low and decreasing – falling from 15.8 percent in 2014–15 to 14.5 percent in 2017–18.¹⁹⁹ Women workers are concentrated in agriculture, representing 67 percent of Pakistan's agricultural workers.²⁰⁰ As agricultural employment is largely informal, workers in the sector tend not to be covered by social protection schemes and legislation. Recent developments to address this situation include the enactment of the *Sindh Home-Based Workers Act 2018* and the *Punjab Domestic Workers Act 2019* to address the needs of women employed in the informal sector. While violence against women is extremely widespread, as discussed above, the implementation of the *Acid Crime (Amended) Law* and efforts to monitor acid attacks has halved the incidence of acid crimes in Pakistan.²⁰¹ To accelerate progress on gender equality, a *Gender Management Information System* was launched in Punjab, while Sindh developed a *Gender Reforms Action Plan* (GRAP). The Vision 2025 aims to improve women's labour force participation to 45 percent. To this end, a "one woman, one bank account" plan was launched, whereby 5.7 million women will be able to open savings accounts.²⁰² Furthermore, a *Gender Equality & Women Empowerment Policy* (2019–2023) has been approved.

IMPACT OF COVID-19 ON GENDER DISPARITIES AND INEQUALITY

As a result of gender inequalities, girls and women in Pakistan are likely to be particularly affected by the COVID-19 pandemic in multiple ways. The experience of past public health emergencies reveals that resources tend to be diverted from routine health care services, and redirected towards containing and responding to outbreaks, often leading to limited access to already constrained services such as safe deliveries, contraception, and antenatal and post-natal health care.²⁰³ This will impact both maternal and child health. Moreover, sociocultural gender norms that accord less value to women and girls may cause them to be neglected in the response to the outbreak, preventing them from receiving timely care and medical attention if they become ill – whether they contract COVID-19, or other illnesses.

These norms also threaten to increase women and girls' burden of care, as they are primarily responsible for performing domestic and care work. Resulting household-level disease prevention and response efforts can expose women and girls to a greater risk of infection, and subject them

199 Ministry of Finance, *Pakistan Economic Survey 2018–19*, Government of Pakistan, Islamabad, 2019.

200 Pakistan Bureau of Statistics, *Labour Force Survey 2017–18*, Government of Pakistan, Islamabad, 2018.

201 Ministry of Finance, *Pakistan Economic Survey 2018–19*, Government of Pakistan, Islamabad, 2019.

205 Poverty Alleviation and Social Safety Division, *Ehsaas Financial Inclusion Policy*, Government of Pakistan, Islamabad, 2019, <<https://www.pass.gov.pk/userfiles1/files/Ehsaas%20Financial%20Inclusion%20policy%20for%20printing.pdf>>, accessed 4 May 2020.

203 Care International, *Gender Implications of COVID-19 Outbreaks in Development and Humanitarian Settings*, Care International, Geneva, 2020, <https://www.care-international.org/files/files/Gendered_Implications_of_COVID-19_Full_Paper.pdf>, accessed 4 May 2020.

to higher levels of emotional, physical and socioeconomic harm.²⁰⁴ Women employed in informal low-wage activities – the leading form of work among the majority of women in Pakistan – are highly affected by the economic consequences of public health emergencies. In addition, as frontline health workers, women are likely to experience greater risk of exposure, alongside pronounced stress and trauma related to the pandemic.

As discussed above, mass school closures are especially likely to cause girls to drop out from education. As they take on additional domestic responsibilities, it will be more difficult for them to balance these duties with an education once schools reopen. Domestic violence against women is poised to surge in the context of the pandemic, particularly as household livelihood opportunities shrink and tensions rise. An expected rise in food insecurity in the wake of an economic recession will negatively impact households that are already vulnerable – with a particular impact on women and girls, who tend to be the most vulnerable members of households. Economic distress among households is likely to keep girls out of school and force many into exploitative situations, such as child marriage. As boys are already more likely to be engaged in child labour outside the home, it is likely that growing financial pressure will prompt more families to send their sons out into the labour market at an early age.

CLIMATE CHANGE

It is those who have done least to contribute to man-made global warming who are carrying the greatest burden of climate change. People in the poorest countries are living on the brink of the climate crisis, and the poorest communities among them are worst affected, being least able to prepare and protect themselves and their environments. Pakistan is among the countries most affected by the impact of climate change. The *Global Climate Risk Index 2020* by the think-tank Germanwatch ranks Pakistan as the fifth most vulnerable country to climate change in the world – a situation which is deteriorating, as Pakistan ranked eighth on the list in 2019.²⁰⁵ The impacts of climate change and related natural disasters – heat waves, drought, floods and other extreme events – severely affect human health, mortality and well-being. The greatest burden falls on developing countries and, within them, on the poorest and most vulnerable segments of society, especially women and children. Women's roles as primary caregivers, and providers of food and fuel, make them especially vulnerable to the impacts of climate change-related catastrophes. UN figures indicate that, globally, 80 percent of people displaced by climate change are women. In Pakistan, over 70 percent of those displaced by severe flooding in 2010 were women and children.²⁰⁶

204 The 2010 cholera epidemic in Haiti and the 2014–2016 EVD outbreak in West Africa demonstrate how it places a three-fold caregiving burden on women and girls.

205 Germanwatch, *Global Climate Risk Index 2020*, Germanwatch, Bonn, 2019, <<https://germanwatch.org/en/17307>>, accessed 4 May 2020; Germanwatch, *Global Climate Risk Index 2019*, Germanwatch, Bonn, 2018.

206 Child-Centered Disaster Vulnerability. Provincial Disaster Management Authority (PDMA), Khyber Pakhtunkhwa (KP), PDMA and UNICEF, September 2018

Specific data on climate change's impact on women and children in Pakistan is scarce. However, climate-induced inequality is a social phenomenon in the country. **Climate sensitive health impacts** are evident in the increased frequency of water borne diseases (diarrhea and cholera), and vector borne diseases (malaria and dengue fever). Extreme weather events which include food insecurity, under-nutrition, as well as impacts on WASH. In 2015, WHO developed the framework for building climate resilient health systems, with the objective to provide guidance for health system and public health programming to increase capacity for protecting health in a changing climate environment. Although, some data has been collated regionally to analyse the impacts of climate change on children (Bangladesh Health-National Adaptation Plan HNAP; 2018), such data could be found to analyse the impacts of climate change on children in Pakistan in terms of increase in the frequency of diarrhea, malaria, or dengue. Rising temperatures, fluctuating rates of precipitation and prolonged drought compromise freshwater supplies, increasing the risk of waterborne diseases, diarrhoeal diseases, infections and vector-borne diseases that impair children's early development,²⁰⁷ while driving up the particular risk of heat-related mortality among children.²⁰⁸

Children are at greater risk of injury and especially susceptible to disease when continuity in water and sanitation services are threatened. **WASH and climate change are inextricably linked.** Disasters can destroy water supplies and toilet, or leaving behind contaminated water and putting the lives of millions of children at risk. Sewage systems are at risk of being flooded with increasing frequency, contaminating water sources and the local environment, while severe droughts force people to resort to even less safe sources of drinking water. Increase in equitable access to sustainable water sources and improved sanitation is critical in regions at risk of droughts and floods. Country and community level WASH hazards should be identified to develop an adaptive action into existing programmes.

Women around the world are more likely to experience poverty, especially in countries like Pakistan where HDI and GDI values are consistently low. This makes it especially difficult for women to recover from disasters which affect infrastructure, jobs and housing. **Climate change-induced injustices and protection issues** greatly affect girls' rights. In and after disasters, girls tend to be the first to drop out of school to take on more caregiving roles in the home. Girls are often married off at a much younger age than boys in households from the lowest wealth quintile, as a means of reducing the financial burdens that disasters cause. During and after disasters, women – especially adolescent girls – face an increased risk of sexual and physical violence, alongside as exploitation. As discussed above, emergencies such as climate-related disasters disrupt health care services, severely undermining women's reproductive health. A

207 American Academy of Pediatrics, *Global Climate Change and Children's Health: Technical Report*. American Academy of Pediatrics, Itasca I.L., 2015.

208 United Nations Children's Fund, *The Impact of Climate Change on Children*, UNICEF, New York, 2015, <sustainabledevelopment.un.org/content/documents/2161unicef.pdf>, accessed 4 May 2020.

focus on disaster risk reduction (DRR) and disaster risk management (DRM) is a core feature of Pakistan's partnership with the UN, as reflected in Outcome 6 ('resilience') of the United Nations Sustainable Development Framework for Pakistan 2018–2022 (UNSDF), also known as the One UN Programme III (OP III).

Climate change is also a driver of **food insecurity**. As climate-related catastrophes become ever more frequent and severe, food insecurity in the wake of emergencies has become a repeated pattern in Pakistan – evident in the rapid emergence of cases of malnutrition. As discussed, malnutrition is a major challenge among children and women in Pakistan and a major impediment to achieving the SDGs. In Sindh, where over 1.2 million people live in conditions of severe drought (2018–2019), acute malnutrition affects around 0.4 million children under-five.²⁰⁹ Women are socially conditioned to refrain from claiming from limited family resources and have a limited role in decision-making on the distribution of these resources. As climate change deals a blow to agriculture, informal women workers – the bulk of the country's agricultural workforce – stand to lose most and receive the least support since they are usually ineligible for compensation schemes that target formal workers. Since women are predominantly responsible for livestock management, the loss of livestock due to disasters diminishes their incomes, food security and nutrition. With dry seasons becoming more prolonged due to climate change, rural women in Pakistan are forced to work harder to feed and support their families.

Pakistan submitted its first National Determined Contribution (NDC) to the Climate Conventions (UNFCCC), which articulates Pakistan's goals, commitments, and aspirations to achieve the Paris Agreement. Under the adaptation commitments, Pakistan has stated that WASH and water management are national adaptation priorities – enhancing water resource management through integrated watershed management, water conservation, development and optimization of water resource allocation, implementation of strict water management regulations, and utilization of unconventional water resources such as recycling of used water and harvesting rain water and flood water. In the same category, education has been highlighted as an area where support is required to develop a multitude of professionals in the field of climate change through strengthened educational opportunities for individuals in the disciplines of geo-sciences, social sciences, management sciences, governance, policy formation and implementation.

The Government of Pakistan has put in place policy and institutional mechanisms to guide a national response to climate change. A *National Climate Change Policy* (NCCP) was developed in 2012, followed by an implementation framework in 2014. In 2017, MoCC was created and the *Pakistan Climate Change Act* passed to strengthen institutional capacity at the federal level. The act calls for the creation of three main instruments: a Climate Change Council, a Climate Change Authority, and a national Climate Change Fund. Khyber Pakhtunkhwa was the first province in Pakistan to draft a provincial *Climate Change Policy* in 2016, followed by Sindh's formulation of a provincial *Climate Change Policy* in 2018.

209 ReliefWeb, *Pakistan: Drought - 2018-2019*, United Nations Office for the Coordination of Humanitarian Affairs (OCHA), New York and Geneva, 2019, <<https://reliefweb.int/disaster/dr-2018-000428-pak>>, accessed 4 May 2020.

UNICEF is conducting *Climate Landscape Analysis for Children* (CLAC) in many countries to examine climate change's impact on children and vulnerable populations. Initiating this exercise in Pakistan is recommended to address the limited understanding of the impact climate change has on the country's children, especially by identifying specific climate change risks for children. The findings of such an analysis and its recommendations on climate, energy and environment (CEE) interventions should be integrated in national, sectoral and subnational development strategies and plans, including the priorities of UNICEF's next country programme for Pakistan.

IMPACT OF COVID-19 ON CLIMATE CHANGE

A reduction in global carbon dioxide (CO₂) emissions has been one clear, immediate effect of the COVID-19 pandemic.²¹⁰ The global reaction to the pandemic sheds light on how far the world still has to go to slow the pace of global warming, highlighting the scale of action needed to tackle climate change – as economies have ground to a virtual standstill, the use of fossil fuel has plummeted, as have air and water pollution. Lower-income and vulnerable populations are being disproportionately affected by both climate change and the pandemic.²¹¹ The fall in emissions prompted by COVID-19 will not stop climate change and the current drop in greenhouse gas emissions linked to the pandemic will likely be a short-term trend. Once the global economy begins to recover, emissions are expected to return to pre-pandemic levels, or to shoot up even further.²¹² Another negative impact of COVID-19 is a potential loss of momentum on efforts to address climate change. In the initial aftermath of the global financial crisis of 2008, global CO₂ emissions caused by fossil fuel combustion and cement production decreased by 1.4 percent, only to rise by 5.9 percent in 2010. The current crisis could have a longer-term impact on the environment at a far greater cost to human life, health and security, if it derails global efforts to address climate change. COVID-19 may already be derailing climate change efforts. For example, ahead of the UN's annual climate summit in November 2020, 196 countries were expected to introduce revamped plans to meet the emission reduction goals set by the 2015 Paris Agreement; however, due to the outbreak, this has been postponed until 2021. The pandemic also threatens local efforts to meet existing climate commitments.²¹³

The COVID-19 lockdown may have cut Pakistan's carbon emissions for now. But it is clear that the Government should go further by ensuring that economic recovery plans boost efforts to address the impacts of climate change. Pakistan's current federal and provincial economic recovery plans

210 Watkiss, Camila, 'Coronavirus: The immediate effect on climate change', ClimateActionClimateActionClimate ActionClimateAction, 30 March 2020, <www.climateaction.org/news/coronavirus-the-immediate-effect-on-climate-change>, accessed 4 May 2020.

211 Whieldon, Esther, 'COVID-19 highlights scale of action needed to tackle climate change', S&P Market Intelligence, New York, 30 April 2020, <<https://www.spglobal.com/marketintelligence/en/news-insights/latest-news-headlines/covid-19-highlights-scale-of-action-needed-to-tackle-climate-change-58333219>>, accessed 4 May 2020.

212 United Nations, 'Fall in COVID-linked carbon emissions won't halt climate change - UN weather agency chief', UN, New York, <<https://news.un.org/en/story/2020/04/1062332>>, accessed 4 May 2020.

213 The EU may shelve crucial climate initiatives, for instance, Poland intends to put its carbon trading programme on hold. China has already extended deadlines for companies to meet environmental standards, while the US Environmental Protection Agency may not penalize companies that fail to comply with federal monitoring or reporting requirements if they can attribute their non-compliance to the pandemic. Brazil's federal environmental agency announced that it is cutting back on its enforcement duties, which include protecting the Amazon rainforest from accelerating deforestation. This could lead to the release of massive amounts of greenhouse gas that are stored in one of the world's most important carbon sinks. See: <<https://www.hrw.org/news/2020/04/16/how-covid-19-could-impact-climate-crisis>>

are heavily focused on social protection and economic relief, diverting much development funding to fight the effects of the pandemic. Tackling climate change must be woven into the solutions adopted to address economic crisis caused by COVID-19.

SOCIAL PROTECTION

Social protection is regarded as a basic human right guaranteed in several international covenants and treaties. Social protection systems are essential to ensure that no one is left behind and are fundamental to prevent and reduce poverty across the life cycle, including cash transfers for children, mothers with newborns, for persons with disabilities, for those poor or without jobs, and for older persons. Goal 1 under SDGs, particularly target 1.3 requires countries to implement nationally appropriate social protection systems and measures for all, including floors, and by 2030 achieve substantial coverage of the poor and the vulnerable.

The Constitution of Pakistan exclusively stipulates the provision of social security for all citizens of the country. The country has number of social security schemes that are being carried out by provincial and federal governments. The National Social Protection Strategy (NSPS) of Pakistan which was adopted in 2007 is the broader policy framework for planning and delivery of social protection services in the country. Although, the Government of Pakistan is spending a huge amount on social security programs, they are rather fragmented and their coverage of children not easy to determine. Overall, spending on social protection in Pakistan is estimated to be less than 1% of GDP. This may be due to the classification of social protection functions within the government's public expenditure system as well as due to the challenges in assessing total funding.²¹⁴ Building a comprehensive approach to integrated social protection systems across the life course in Pakistan is extremely essential and expanding fiscal space for social protection would reach more families with children.

After the 2018 General Election, Pakistan Tehreek-e-Insaaf (PTI) formed a government on a manifesto that broadly focused on transforming governance through accountability, while empowering people at the grassroots level by revamping social services.²¹⁵ In 2019, the Poverty Alleviation and Social Safety (PASS) Division was established under the Cabinet Secretariat to implement the Government's flagship *Ehsaas* ('Compassion') programme. The programme aims to 'reduce inequality, invest in people, and lift lagging districts.'²¹⁶ Among other issues, it prioritizes the provision of social protection for informal workers and targets the extremely poor, orphans, widows, the homeless, persons with disabilities, the jobless, students from low-income backgrounds, poor women and the elderly. With a large mandate to bring all poverty alleviation programmes under one umbrella, the programme proposes 115 policy actions and

²¹⁴ Carol Watson, Tanya Lone and Valentina Barca (2017). Building on social protection systems for effective disaster response: the Pakistan experience. Policy Brief on Shock Responsive Social Protection Research – July 2017. <https://www.opml.co.uk/files/Publications/a0408-shock-responsive-social-protection-systems/srsp-policy-brief-pakistan.pdf?noredirect=1> accessed on 13th May 2020.

²¹⁵ Pakistan Tehreek-e-Insaaf, *The Road to Naya Pakistan: PTI Manifesto 2018*, PTI, 2018, <<https://www.insaf.pk/content/manifesto>>, accessed 4 May 2010.

²¹⁶ Poverty Alleviation and Social Safety Division, About Us, Islamabad, Government of Pakistan, <<https://www.pass.gov.pk/Overview.aspx>>, accessed 4 May 2010.

seven overarching goals for the multi-sectoral, multi-stakeholder *Ehsaas* Strategy. These cover four pillars: (i) address elite capture and make government system work for equality; (ii) safety net; (iii) human capital development; and (iv) jobs and livelihoods.

As millions of people struggle to maintain their livelihoods and income in the aftermath of COVID-19, there is a need to scale up social protection measures – providing social safety nets and cash transfers, protecting jobs, working with employers to support working parents, and prioritizing policies that connect families to life-saving health care, nutrition and education. The Government of Pakistan, under *Ehsaas*, has initiated an emergency cash transfer initiative for 12 million poor households during the COVID-19 pandemic. Pakistan's leading social safety mechanism – the Benazir Income Support Programme (BISP), which provides regular cash support for women from the country's lowest wealth quintile – is being used for to provide these emergency cash transfers. As of 1 May 2020, over PKR 83 billion had been disbursed to 6.8 million beneficiaries.²¹⁷

In response to COVID-19, the World Bank's Pandemic Response Effectiveness in Pakistan Project was launched. Of the US\$ 200 million committed under this project, US\$42 million has been assigned to existing government social safety mechanisms to help mitigate economic impacts on poor households. In addition, emergency cash transfer component worth US\$25 million has been assigned to finance cash transfers in order to augment government efforts, and US\$ 12 million has been designated for the provision of emergency food supplies for quarantined populations and people with limited mobility through the National Disaster Management Authority (NDMA) and Provincial Disaster Management Authorities (PDMAs).

²¹⁷ Poverty Alleviation and Social Safety Division, *Ehsaas Emergency Cash (All Categories)*, Islamabad, Government of Pakistan, <https://www.pass.gov.pk/ecs/uct_all.html>, accessed 4 May 2010.



CHAPTER 5: MAPPING DEVELOPMENT PROGRAMMES IN PAKISTAN

MAPPING DEVELOPMENT PROGRAMMES IN PAKISTAN



To identify where further interventions are needed for Pakistan's children across UNICEF's focal areas, this section looks at of existing programming by the Federal Government, provincial governments and developments partners. This mapping exercise is presented from two angles: from the perspective of donors, and from that of government authorities.

The mapping exercise used two sources of information:

- The Annual Development Plan (ADP) budgets of Pakistan's federal and provincial governments; and
- Donors' websites and other sources of data on key donors, including calls with sector-specific donor staff.

Filters were assigned to accurately present the existing scale, and thematic focus, of investments:

- The mapping exercise looks at interventions by donors in partnership with governments in the areas of health, nutrition, social protection, social policy, education and WASH.
- The timeframe was set to capture all active/ongoing projects and proposed projects by development partners.
- When performing the analysis on government development programmes, investments were considered where allocations on donor-assisted projects appeared in the current year's ADPs. Government ADPs were analysed to assess the interests of federal and provincial

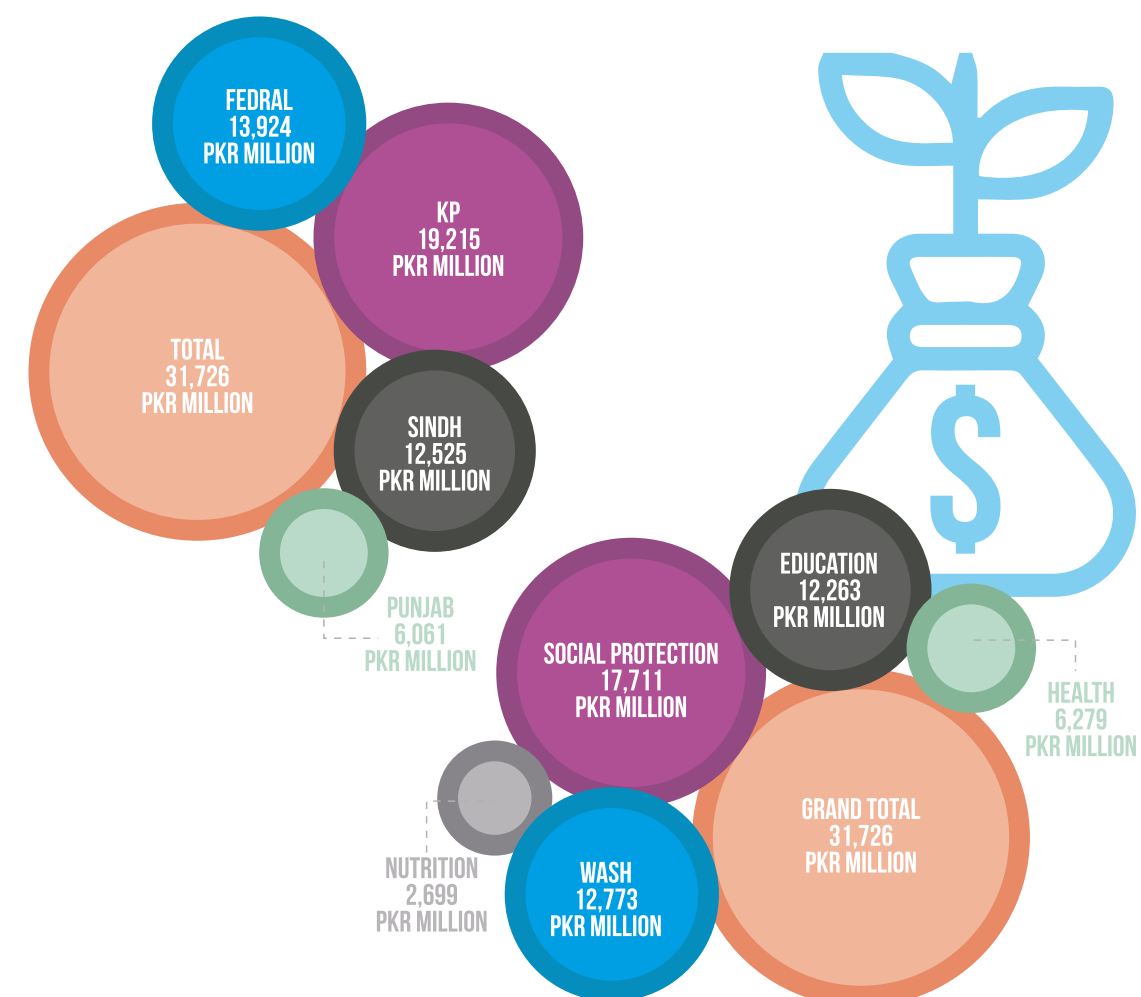
governments, and gain insights into public investments and development funding needs.

- All financial data was converted into USD using the average annual exchange rate. Where the exchange rate was mentioned in the project contract, that rate of exchange was used.
- Some projects were identified by UNICEF during an earlier donor mapping exercise. Those project that were active, but whose data is not publicly available or verified on donors' websites, were considered with data on an 'as is' basis.
- Where funding details were not available, the projects are considered but funding amounts are left blank.
- The analysis did not include UNICEF projects.

INVESTMENT UNDER ANNUAL DEVELOPMENT PLANS FOR 2019–2020

Donor funds allocated to projects in the current federal and provincial ADP budgets (2019–20) total PKR 51 billion in the areas on which UNICEF works. Most foreign assistance targets social protection followed by WASH and education. This suggests governments' interest in seeking development partner investments in these areas.

Figure 16: Donor funds allocated in ADPs

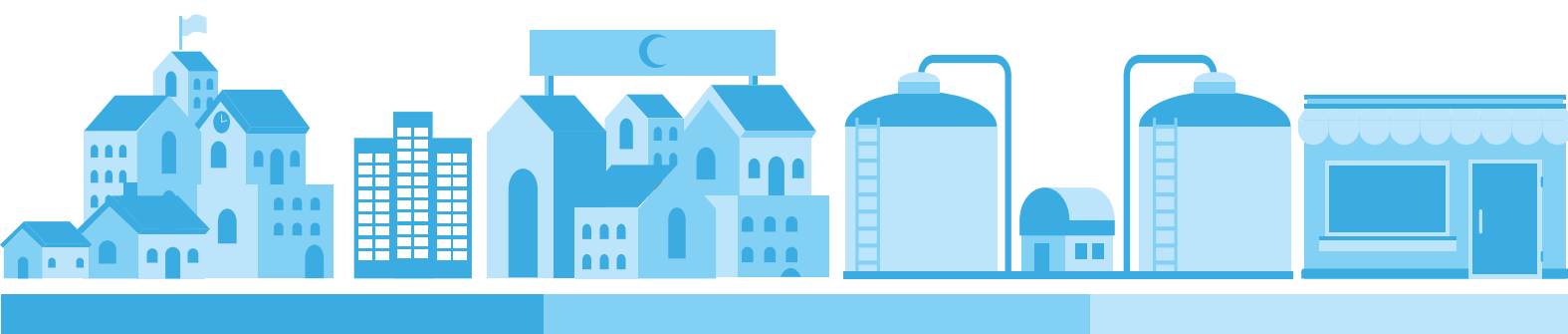


INVESTMENTS BY DEVELOPMENT PARTNERS

US\$4.8 billion has been invested by multiple donors in interventions on health, nutrition, education, social policy, social protection and WASH in Pakistan. This funding includes lead donors, as well as co-financing.

Table 8: Federal and provincial investments in key thematic areas

Lead donor plus co-financing (US\$ million)	Federal	Balochistan	Khyber Pakhtunkhwa	Punjab	Sindh	Total
Education	400	623	613	904	425	2,965
Health	271	–	15	–	–	286
Nutrition	68	–	35	3	152	258
Social policy	35	–	–	–	–	35
Social protection	745	–	110	–	–	855
WASH	9	–	71	215	80	375
Grand Total	1,528	623	844	1,122	657	4,774



The tables below list lead donors and implementing agencies who have partnered with the Government of Pakistan by providing technical assistance, grants and loans (discounting co-financing or funding). A total of 62 projects were mapped by this exercise.

Table 9: Number of projects by lead donors and implementing partners

No. of Projects	Education	Health	Nutrition	Social policy	Social protection	WASH	Total
Asian Development Bank (ADB)	2	2			2	5	11
United Kingdom Department for International Development (DFID)	2	1	1		4		8
European Commission (EC)	3			2			5
Directorate-General for European Civil Protection and Humanitarian Aid Operations (ECHO)			2				2
European Union (EU)	5		1				6
German Society for International Cooperation (GIZ)				1			1
Global Partnership for Education (GPE)*	2						2
IKEA Foundation					1		1
Japan International Cooperation Agency (JICA)		1					1
Korea International Cooperation Agency (KOICA)						1	1
Swiss Agency for Development and Cooperation (SDC)						1	1
Telenor					1		1
United States Agency for International Development (USAID)	1	4	1			1	7
World Bank	5	3	4		1	2	15
Total	20	11	9	3	9	10	62

*New GPE funding of US\$125 million not included in the data presented in tables. It is in the process of approval and details are provided below separately.

Table 10: Investments by lead donors and implementing partners

US\$ Million	Education	Health	Nutrition	Social policy	Social protection	WASH	Total
ADB	76				200	217	493
DFID	1,057	4	68		538		1,667
EC	21			35			56
ECHO			5				5
EU	105		68				173
GPE	642						642
IKEA Foundation					7		7
JICA		56					56
KOICA						7	7
SDC						4	4
Telenor					10		10
USAID	159	60				67	286
World Bank	734	73	91		100	40	1,038
Total	2,794	192	236	35	855	335	4,447

Development partners with large portfolio – such as DFID, the EU, GIZ and the World Bank – assist Pakistan either through general budgetary support, or sectoral budgetary support. Donors that provide smaller amounts of financial assistance usually work through local and international NGOs. Based on this mapping exercise, it is clear that development partners play an important role in Pakistan's education sector, providing considerable funding. While many development partners support education in the country, although the subsectors that they support vary depending on changing donor and government priorities. In Punjab, DFID and the World Bank are the largest donors that support the provincial government's education reform programme. Major contributors to Sindh's education sector are USAID, the EU, the World Bank and ADB.

Development partners have also come forward to support government efforts to minimize the disruption of education and facilitate the continuity of learning in the context of COVID-19. One notable investment is the World Bank's *Pandemic Response Effectiveness in Pakistan* Project, worth US\$200 million, to respond to the COVID-19 pandemic by strengthening the country's national health care systems and mitigating socioeconomic disruptions. As noted above, US\$5 million of this sum has been assigned to mitigate the pandemic's impact on education by supporting remote learning for 50 million children. It will also assist a comprehensive communications campaign to help schools and parents engage in distance learning activities (including television and radio broadcasts, a virtual network of teachers, and

other ways of delivering academic content at a distance). The World Bank's COVID-19 response focuses on three workstreams: (i) repurposing or restructuring existing projects worth US\$6 million; (ii) an emergency fast track response through additional funding under the *Pandemic Response Effectiveness in Pakistan Project* which involves supporting the Federal Government's 'Teleschool' initiative and new digital content; and (iii) a national pipeline project – the Equitable Education Project – that will specifically will focus on digital technology, broadcasts, distance learning, federal and provincial coordination, and grants for provinces. The Global Partnership for Education (GPE) – a global multi-stakeholder partnership and funding platform dedicated to ensuring education for children in the world's poorest countries, especially girls – is expected to provide US\$20 million in funding for a national education programme. These funds are within the larger investment of US\$125 million under Education Sector Program Implementation Grant for Pakistan that also includes US\$50.6 million for Punjab, US\$31 million for Sindh, US\$23.2 million for Khyber Pakhtunkhwa and US\$20 million for Balochistan. Additionally, the GPE funding will also include a Multiplier Grant of US\$25 million for Sindh.

Out of the \$200 million World Bank's *Pandemic Response Effectiveness in Pakistan Project* approximately US\$32 million has been allocated for CRVS for the four provinces, however, this is still in pipeline. The government has also recently approved World Bank's lending of US\$500 million including US\$10 million allocated for CRVS for the provinces (2 district for each province) to scale up the CRVS District Model where births and deaths will be notified and registered through engagement with health sector.

Donor commitment has been considerable for the Government's vision for social protection, which aims to develop an integrated, comprehensive social protection platform focused on the needs of the poorest and most vulnerable. As reflected in Pakistan's national development framework, Vision 2025, the Government seeks to enhance the effectiveness of social protection instruments through strengthened administration, and to expand the scope of the state's income support programme executed by the BISP. Donor commitments include the World Bank's National Social Protection Programme – worth US\$100 million – which strives to strengthen national social safety net systems for the poor to enhance their human capital and access to services, including the education stipend Waseela-e-Taleem. DFID supports social protection in Pakistan through its National Cash Transfers Programme, whose total value is equivalent to US\$373.4 million (GBP 300 million), while ADB does so through its US\$430 million Social Protection Development Project.

While support for education represents 62 percent of total donor funding mapped by this exercise – including large donor-funded projects at the federal and provincial levels – support for health only accounts for 6 percent of donor funding, while nutrition support represents 5 percent. Gaps in financing are seen in health, nutrition and social protection – particularly nutrition in Punjab and Balochistan, and health-related investments in Balochistan, Punjab and Sindh. WASH is a key area where investments are also needed. The Government has prioritized WASH in its ADP budget earmarking a considerable amount of support for water and sanitation. However, investments by development partners are not as many for WASH compared to those for other thematic areas.



**CHAPTER 5:
KEY FINDINGS,
CONCLUSIONS AND
RECOMMENDATIONS**

Findings	Risks	Recommendations
Contextual developments - Population growth continues to be over 2.4 per cent. - Half of Pakistan's population are between 0 and 19 years old. Children of school-going age (5 to 14 years old) account for over one-quarter of the country's population. 36.4 percent of the population lives in urban areas. The immense size of the urban population has important implications for public service delivery, particularly for children. - Performance on human development outcome remains poor, with Pakistan's HDI ranking falling to 152nd of 189 countries, although its HDI value has marginally improved to 0.560. - Pakistan still ranks low both on the Gender Development Index (GDI), with an index value of 0.747, and the Gender Inequality Index (GII), with an index value of 0.547. Its performance on these indexes has improved only marginally in recent years. Pakistan still has the lowest GDI and GI compared to other countries in the region. - Pakistan's GDP is projected to contract by at least -1.5 per cent, while GDP per capita is expected to shrink by 3.36 per cent in 2020 in the wake of the COVID-19 pandemic.	- In addition to creating a public health emergency, the COVID-19 pandemic has prompted an even greater economic emergency in Pakistan, with severe consequences for its poorest households and their most vulnerable members – women and children. - Budget allocations have been diverted to fight the impact of COVID-19, leaving limited funds for development budgets. This will push back national and provincial growth trajectories. If these trends are not urgently reversed, it may derail progress on several fronts. 18 million people in Pakistan could lose their jobs and between 20 and 70 million people may fall below the poverty line because of the lockdown implemented to stem the pandemic.	- Provide significant policy support for higher, sustained and inclusive growth, balanced regional development, and increased access to financial resources to support the livelihoods of the most vulnerable groups in Pakistan. - Support the Government to scale up social protection measures – providing social safety nets and cash transfers, protecting jobs, working with employers to support working parents, and prioritizing policies that connect families to life-saving health care, nutrition and education. - Implement measures to save lives by containing the spread of COVID-19, particularly interventions to protect the poorest and most vulnerable from negative shocks caused by its spread and resultant containment policy actions. - Safeguard essential services for women and children. - Counter the high risk of decreasing budgets for the most vulnerable, including children. - Recognize children as a unit of allocation and expenditure to incorporate them into annual budgeting and planning.

Findings	Risks	Recommendations
Multidimensional poverty and vulnerability - The COVID-19 Responsive Multidimensional Vulnerability Index (CRMVI) estimates Pakistan's multidimensional vulnerability score as 0.264, compared to its already high Multidimensional Poverty Index (MPI) score of 0.197. 56.6 per cent (126 million people, including 92 million in rural areas) are susceptible to falling into the category of 'vulnerable' persons as a result of the COVID-19 pandemic. - Sindh and Punjab may experience the highest increases in multidimensional vulnerability. - The largest contributor to vulnerability is having less than five years of schooling. 26 per cent of Punjab's population is multidimensionally poor with the MPI value of 0.124. Children are disproportionately heavily and more frequently affected by multidimensional poverty. The Child MPI is 0.220, incidence of multidimensional poverty for children is 50.7 per cent and intensity of poverty at 43.5 per cent.	- National MPI measurement does not accurately represent children's needs. - The rise in vulnerability in the wake of COVID-19 pandemic may lead to millions more people being pushed into multidimensional poverty. The pandemic may exacerbate existing forms of poverty, cause new forms of poverty, and render more people becoming poor.	- Institutionalise an accurate measure of child poverty, centring the child at the focus of the analysis over the household. - Include new children orientated indicators in the national MPI. Particularly, include nutritional variables and indicators. - Explore more accurate child poverty measurement methodologies and approaches that can provide an accurate picture of the realities around poverty. - Draw an accurate baseline and understand the extent, incidence and depth of intra-household deprivation/poverty. - Routine measurement of child poverty should be integrated into existing national and provincial surveys by ensuring that they capture relevant indicators of child poverty. - Explore opportunities for greater focus on child poverty and integrating it into national and provincial development/ growth strategies and sectoral plans.

Findings	Risks	Recommendations
Social protection The Government's flagship Ehsaas programme, led by the established PASS Division, prioritizes the provision of social protection for informal workers, targets the extreme poor, and aims to bring all of Pakistan's public poverty alleviation programmes under one umbrella.	Scaling up the Ehsaas programme will require a considerable financial outlay paired with sustained capacity building to monitor the impact of interventions.	Work with the Government to ensure the provision of social protection to at least 70 per cent of the population who live below the poverty line.
- Emergency cash transfers worth over PKR 83 billion have been disbursed to 6.8 million beneficiaries. The Government aims to reach a total of 12 million poor households during the COVID-19 pandemic. - Development partners have stepped up their support for the Government through specific interventions to help mitigate the economic impacts of COVID-19 on poor households.	The one-off cash transfer mechanism is expected to provide short term relief to the poorest. However, this may not be sufficient given the expected long-lasting impact of the pandemic on the livelihoods of the most vulnerable	- Building on existing greater interest of donors and the government in social protection, support government to make social protection more child sensitive and to develop integrated social protection systems to reduce the current fragmented system. These include targeting and integrated social registry systems. - Reporting on children who benefit from social protection programmes. - Assess the impact of support provided for social protection during the pandemic to adjust the approach used for future interventions, since one-time cash transfers may not be sufficient.

Findings	Risks	Recommendations
Health - One in every 14 Pakistani children does not survive to their fifth birthday and death of newborns within the first week of life accounts for half of all infant mortality. While child mortality rates have declined in the last five years, high child mortality remains a huge challenge for Pakistan. - Pakistan's NMR, IMR and U5M rates have declined, although not considerably, in the past five years. - Two in three children between 12 and 23 months old have received all eight basic vaccinations – an improvement since 2012–13. However, 4 per cent of children have not received any basic vaccinations. Despite improvement in immunization coverage, Pakistan is home to around 1.4 million un and under immunized children residing primarily in high risk urban, remote rural and conflict areas.	- Pakistan will not achieve SDG 3 target on child mortality without prioritizing newborn health and quality of perinatal care. - There is a risk that Pakistan's fragile health system will not be able to maintain the delivery of essential health services due the increased burden on services caused by COVID-19. Both direct mortality from the outbreak, and avertable mortality due to a lack of treatment for vaccine-preventable and treatable conditions may increase by threatening child health and survival.	- Newborn health and stillbirth reduction should be the top priority of health programmes through multiple approaches – addressing health system issues, HR, quality of services and community based newborn programme. - Collaboration between government and development partners for service utilization and evidence based advocacy on increasing the health budget with prioritization to PHC services. - Providing a comprehensive package of maternal and newborn care.

Findings	Risks	Recommendations
- Basic vaccination coverage is lowest in Balochistan and highest in Punjab. Low immunization coverage and the proportion of children who have missed all of their vaccines are associated with socio-economic factors, including poor housing and environmental standards, under-employment, low income status, and low literacy levels. 26 per cent of union councils in slums and underserved areas are without EPI facilities. - Polio remains endemic in Pakistan, one of only two countries in the world with new reported polio cases. - There is a declining trend in the prevalence of diarrhoea in children under-five in recent years. - Pakistan's current MMR is high at 178 deaths per 100,000 live births, although this marks a considerable improvement since 2006–07. - The percentage of women who receive antenatal care from a skilled provider has risen, although rates remain very low in Balochistan. Institutional deliveries have also increased, as has skilled birth assistance. - There is a positive relation between a mother's level of education and health care-seeking behaviour, as well as between household wealth and treatment for childhood illnesses.	- Access issues caused by poor infrastructure will make it hard to reach remote areas, severely affecting children's and women's ability to access essential reproductive, maternal, newborn and child health services. - Without increasing the health budget and social safety net the out of pocket expenditure will increase in coming years making the poor more vulnerable due to high cost of health care. - Without proper planning, costing along with investment case and engagement of Provincial Health Departments, UHC/BP action plan will not achieve desired results. - LHW programme is suffering from retention, financial, organization and management issues. - The health gains of the country are being masked by communicable disease and there is urgent need to address the public health system. - Increasing morbidity and mortality due to epidemics and natural disasters. - The frequent outbreak of VPDs (polio, measles, diphtheria), dengue and typhoid are showing the greatest challenge on public health system of Pakistan.	- Emphasize the broader effects of the pandemic on child and maternal health, alongside the direct impact of COVID-19 through resilient health system approaches. - Support the Government's priorities, as outlined in the National Action Plan for Preparedness and Response to Coronavirus Disease (NAP), to contain and respond to the outbreak in a timely and efficient manner. Prioritize financial resources for emergency preparedness and implement emergency preparedness actions by strengthening intersectoral collaboration with government sectors, the private sector and civil society at the provincial level. - Make concerted efforts to collect and analyse data on pre- and intra-COVID-19 trends on vaccination rates, antenatal care visits and hospital deliveries, among other indicators. - Deliver all immunization services and implement effective communication strategies to engage communities and ensure supply and demand for vaccination as it is essential to saving lives. Mass vaccination campaign should be undertaken after a risk-benefit assessment. - Emphasize on the public health response to COVID with focus on RCCE, IPC, protection of health workers and continuation of essential MNCH, Immunization and HIV services.

Findings	Risks	Recommendations
- One in eight adolescent girls and one in five adolescent boys is underweight. 56.6 per cent of adolescent girls in Pakistan are anaemic. Social-cultural barriers create bottlenecks that prevent young people from seeking support or services to meet their physical, mental, emotional and psychological needs. - Public expenditure on health only accounts for 1.1 per cent of GDP, and development expenditure represents 13.7 per cent of total public expenditure. - Urban-rural disparities suggest a lack of available health services, limited access to services due to difficult terrain and poor infrastructure, the poor quality of health services, and high rates of poverty in rural areas. - The government has prioritized Universal Health Coverage and Basic Package (UHC/BP) of health services to reach the most vulnerable population.		- Government to develop investment plan and contribute for UHC/BP implementation within the framework of Primary Health Care Acceleration. - Prioritize climate change and environmental health within the domain of public health system with effective cross sectoral collaboration. - Develop national and provincial health department for health emergency and humanitarian response strategy and implementation. - Strengthen interaction between public and private healthcare sector. Provision of private healthcare facilities with public financing can play a key role in provision of quality healthcare services to the poor and marginalized communities.

Findings	Risks	Recommendations
Nutrition - Two in every five children under-five in Pakistan are stunted, while 17.7 per cent suffer from wasting and 28.9 per cent are underweight. Nevertheless, improvements have been made on stunting and underweight rates compared to 2011. - Boys in Pakistan seem to be more affected than girls by all forms of malnutrition. - Stunting is most prevalent in the Merged Districts of Khyber Pakhtunkhwa, followed by Balochistan, Gilgit-Baltistan and Sindh.	- The state of food insecurity and malnutrition was already alarming before the COVID-19 pandemic. Pakistan's most food insecure households – who belong to the country's lowest economic strata – will likely face more acute food insecurity due to the current crisis. - The interruption of food and nutrition services has negatively affected groups with the highest nutrition needs – especially children under-five, pregnant and lactating women, and the elderly.	- Adopt a systems approach to nutrition that makes multiple systems accountable for nutrition results beyond sectoral objectives including the food, health, WASH, education and social protection systems. - Prioritize routine immunization of children in essential service delivery.

Findings	Risks	Recommendations
- The reasons for high stunting rates are manifold and linked to poor infant and young child feeding practices, lack of exclusive breastfeeding, insufficient awareness of good eating practices and the effects of poverty and geographic (urban/rural) location. - Nearly half of Pakistan's children are deficient in critical vitamins and minerals. More than half of Pakistani children are anaemic, of whom 5.7 per cent have severe anaemia. - Maternal anaemia affects 41.7 per cent of mothers and is a leading cause of pre-term delivery, low birth weight and perinatal mortality. - Average birth rate is 46 births per 1,000 women (aged 15-19), while 8 per cent of adolescent women, aged 15-19, are already mothers or pregnant with their first child. 36.9 per cent of Pakistan's households are food insecure, and 18.3 per cent are affected by severe food insecurity. However, the overall food insecurity situation has improved since 2011. - Balochistan and Sindh have the highest percentage of households affected by severe food insecurity. 94 per cent of children are breastfed, however, 48 per cent of children receive exclusive breastfeeding.	- The nutrition sector receives relatively small budgetary allocations, making it heavily dependent on donors. - Public sector interventions tend to be oriented towards treatment, rather than prevention.	- Conduct a rapid yet comprehensive mapping of those most at risk of being left behind. Formulate mechanisms during and after the pandemic, based on this mapping, to incorporate provisions on the right to food both in terms of quantity and nutritional quality. - Include nutrition-sensitive interventions that address malnutrition among children and women in planning and budgeting across all sectoral development schemes and all Annual Development Programmes. Strong institutional mechanisms and capacities, particularly at the sub-national levels needed. - Ensure the continuation of essential health services for the treatment of severe acute malnutrition. Continue advocacy and the dissemination of information on infant and young child feeding practices through the print and electronic media. - Invest in a robust, large-scale promotion and continuation of essential health services for the treatment of severe acute malnutrition.

Findings	Risks	Recommendations
- Indicators on minimum dietary diversity, minimum meal frequency and a minimum acceptable diet have all declined over time. - Structural factors that contribute to poor nutritional indicators include difficulties in tracking public expenditure on nutrition through financial reporting systems; inadequate systems for monitoring and evaluation; and delays in the implementation of provincial multi-sectoral nutrition strategies.		
Education - Despite investments by provincial governments and development partners to expand access to education, there are 22.8 million out-of-school children in Pakistan. They include 12.16 million girls, 5 million children aged 5 to 9, and 11.4 million adolescents between the ages of 10 and 14. - Balochistan has the highest percentage of OOSC (aged 5 to 16), followed by the Merged Districts of Khyber Pakhtunkhwa, Sindh, Gilgit-Baltistan, and Azad Jammu and Kashmir. - Dropout rate is high at 26 per cent in the transition between Katchi and grade-1. - More children in rural areas are likely to be out of school compared to their urban counterparts and girls are more likely to drop out than boys. 21 per cent of children from Pakistan's poorest households attend pre-primary education, compared to 45.4 per cent from the richest. - Primary ANER (77.4 per cent) has marginally improved since 2016.	- Low NAR coupled with low transition rate translate into a considerable proportion of school age population being excluded from the formal education system at a very early stage - The current countrywide school closure has affected 42 million learners in Pakistan. Previous shutdowns globally have put schoolchildren, and especially girls, who are out of school for extended periods of time, at a risk of not returning when classrooms reopen. Boys are likely to engage in labour and may also not return to school. - A high population growth rate makes raising enrolment rates particularly challenging leading to access issues.	- Address weaknesses observed during the pandemic, such as the capacity of the education delivery chain from top to bottom. - Support government efforts on response coordination, efficient delivery, and real-time monitoring to ensure up-to-date information on children, in order to understand the scale of education problem beyond aggregate numbers. - Provincial education departments to explore a mix of no tech/offline, low tech and high tech/online modalities across the technology spectrum to reach maximum number of children as well as respond to learning needs and of children at different levels. - Advocacy for increase in education budget allocations at the federal and provincial levels to ensure equitable allocations and the sustained funding of education initiatives.

Findings	Risks	Recommendations
- Primary NAR (58.5 per cent) has declined, particularly in Punjab. The downward trend is witnessed across all provinces except Sindh, where primary NAR has improved. - Middle and secondary NAR (38 per cent) remains very low, but has improved slightly since 2012 (37 per cent). Improvements are evident in all provinces except for Khyber Pakhtunkhwa. - The continued gender gap in retention and enrolment at all three levels remains a major concern. This is due to fewer middle and secondary school places for girls and high population growth rate which puts burden on resources and any investment made is nullified with greater demand. - Primary GPI (0.92) further declines at the middle and secondary education levels (0.89). Youth literacy GPI is only 0.81, suggesting limited access to education for girls. - Post-primary retention rates in the public education system are low – for every 100 students enrolled in the katchi (pre-primary) class, only 28 reach Class 10 in a public school. 36 per cent of enrolment nationwide in private schools with 16 million students enrolled. - Learning poverty in Pakistan is worse than the average for South Asia and lower middle-income countries. Three of four children of late primary school age are not proficient in reading in Pakistan, and one of every three do not achieve a minimum proficiency level by the end of their primary education.	- Inadequate quality of education has led to high dropout rates in recent years. - School closures will widen learning inequalities and hurt vulnerable children and youth disproportionately. - The longer schools remain closed, the larger the learning losses and the less likely children are to catch up on learning	- Development partners and private sector to support federal and provincial governments to minimize the disruption of education and facilitate the continuity of learning. - Support Government initiatives, such as 'Teleschool', Taleem Ghar and rapid response to ramp up existing digitized education programmes. - Develop and implement a distance and blended learning competency standards and assessment framework. - Create and develop content for different platforms and modalities according to levels, subjects and grades. - Support provinces to integrate post COVID-19 interventions in their annual development plans and post COVID-19 public investment strategies. - Develop, print and distribute learning materials where there is limited access to online content. - Expand demand side interventions, particularly efforts to bring girls back to school through targeted messages on participation and by using incentives. - Support the non-formal education sector by promulgating policies to create a level of parity between formal and non-formal education.

Findings	Risks	Recommendations
■ Critical determining factors for poor learning are linked to education system issues that are important for quality such as curriculum and instructional materials, teacher professional development, and assessment. ■ Children with disabilities are more likely to live in poorer households and less likely to be educated. Among employed persons with some form of disability (aged 18 to 65), only 27 per cent have completed primary school, compared to 42 per cent of persons without disabilities. ■ Public spending on education remains very low, accounting for 2.3 per cent of GDP, a marginal increase from 2.2 per cent in 2017. Development expenditure represents only 10 per cent of total expenditure on education.		■ Identify and expand accelerated and alternate learning pathways in hard-to-reach areas with maximum outbreaks, employing extensive community engagement and social mobilization. Develop, implement and expand remedial learning/ ALP/NFE programs for children who fall behind due to school closures. ■ Strengthen and support the role of teachers, head teachers and schools by conducting teacher and head teacher trainings and orientation sessions for different learning modalities and provide them with necessary guidance and tools for community outreach. ■ Develop campaigns and advocacy efforts about risks from COVID-19 and procedures to prepare schools for safe reopening for children and staff.
Child protection ■ Birth registration of children under-five has improved considerably in the last five years, particularly in Balochistan (8 per cent to 38 per cent) followed by Khyber Pakhtunkhwa (10 per cent to 19 per cent), Islamabad Capital Territory (74 per cent to 82 per cent), and Sindh (25 per cent to 28 per cent). ■ Children who are not 'covered' by a legal registration process largely live in rural areas; their families, especially their mothers, tend to be uneducated; most are from lower-income households; belong to minorities; are immigrants; or are frequently abandoned children.	■ Children who are already the most vulnerable to exploitation, abuse and neglect will be made even more vulnerable by the direct and indirect negative effects of COVID-19. ■ Movement restrictions, loss of income, isolation and high levels of stress are increasing the likelihood that children experience and observe physical, psychological and sexual abuse at home	■ Support government in strengthening its Civil Registration and Vital Statistics (CRVS) capacities. Emphasize on continuity of civil registration as an essential service. ■ Engagement of health sector for notification of births and deaths through hospital/ health facilities and other means (CRVS Model Districts).

Findings	Risks	Recommendations
■ 81 per cent of children aged 1 to 14 in Punjab have experienced physical punishment and/ or psychological aggression by caregivers, marking no improvement since 2014. The same is true for 81 per cent of the children in Khyber Pakhtunkhwa, with the highest percentage of violence among children aged 3 to 9. ■ Among ever-married women age 15-19, 26.3 per cent experience physical violence and 33.4 per cent experienced physical or sexual violence. Young women (aged 15 to 19) experience more physical violence than older women. The Merged Districts of Khyber Pakhtunkhwa have highest incidence of gender-based physical violence in the country, followed by the provinces of Khyber Pakhtunkhwa and Balochistan. ■ The labour force participation rate for children aged 10 to 14 (8.23 per cent) is on a downward trend, but remains considerable for children aged 15 to 19 (32.57 per cent). 13.4 per cent of children aged 5 to 17 in Punjab, and 14.4 per cent in Khyber Pakhtunkhwa, are engaged in child labour. ■ Child labour is high (17.5 per cent) in rural areas, and far lower (6.5 per cent) in urban settings. Disparities are also stark across wealth quintiles – while 28.1 per cent of children from Pakistan's poorest households are engaged in work, this is true for only 3.1 per cent from the richest.	■ While online education has become central to the COVID-19 response, it is also increasing the risk of cyberbullying, sexual abuse and exploitation. ■ An increase in gender-based violence and sexual exploitation and abuse are also expected based on previous experience of epidemic ■ Household economic distress can force children into child labour and child marriage in high-risk countries like Pakistan.	■ Support government to play central role to ensure that COVID-19 prevention and response plans integrate age appropriate and gender sensitive measures to protect all children from violence, neglect and abuse. ■ Ensure children and women have sustained access to social services, including protection, psychosocial support services and legal services. ■ Rapid assessment on data related to COVID-19 outbreak and the implementation of the emergency response on disaggregated data by sex and age on all relevant indicators on violence and exploitation of women and children ■ Prioritize child focused Social Protection Policy framework that captures preparedness, resources and mitigation in case of natural and made disasters.

Findings	Risks	Recommendations
The incidence of child marriage – determined by analysing how many women aged 15 to 19 are currently married – has remained high (14 per cent) and virtually unchanged since 2013. Over 48 per cent of women aged 20 to 24 are married. Women with no education marry more than six years earlier than women with higher education (18.7 years versus 24.9 years).		- Provide child protection case management and emergency alternative care arrangements. - Communicate with, and engage, communities, parents, caregivers and children themselves with evidence-based information and advice on child protection and preventing abuse. - Develop and launch a nationwide campaign that raises awareness about the violent effects of the pandemic on women and children.
Water, sanitation and hygiene - Around 92 per cent of population in Pakistan has access to improved water source, 77 per cent has it available when needed (available at premises), 56 per cent have access to basic service and 35 per cent of the population have access to safe water which is free from contamination. The lowest figure of all three is safe water free from contamination i.e. 35 per cent, which is the baseline for safely managed water in Pakistan in 2018. - Only 7 per cent of households follow appropriate water treatment practices prior to drinking water. 60 per cent of the population in Pakistan have basic sanitation, 10 per cent with limited service, 20 per cent fall under unimproved and around 10 per cent practice open defecation (without any toilets).	- The lockdown is likely to exacerbate WASH challenges in Pakistan, especially as 10 per cent of its population already practices open defecation. - Funds earmarked for WASH development activities may be diverted towards managing COVID-19, as interventions on COVID-19 predominantly target health care facilities, isolation centres, quarantine sites and at-risk communities.	- Strengthen public awareness raising campaigns to explain how COVID-19 spreads and how WASH can help to prevent this. - Safe WASH services in health care facilities (HCFs) to deliver quality health services; protect patients, health workers, and staff; and to prevent further transmission. - Scale up the WASH intervention in schools and hand washing initiatives for safe re-opening of schools after lockdown is lifted. - Integrate climate resilience and WASH as integral components of future interventions.

Findings	Risks	Recommendations
- Half of the rural population in Pakistan use basic sanitation services compared to 77 per cent in urban areas. - Only 16 per cent of poor have basic sanitation services, while 63 per cent practice open defecation compared to 90 per cent of the richest with basic sanitation service and zero open defecation practice. 73 per cent of primary schools in Pakistan have drinking water availability, this increases with the level of education as middle, secondary and higher secondary level schools have 85 per cent, 92 per cent, and 97 per cent availability of drinking water. - Availability of toilets for students is in 73 per cent of primary schools, 88 per cent of middle schools, 93 per cent of secondary schools and 97 per cent in higher secondary schools. - Awareness around water use, safety, maintenance and conservation is generally lacking in Pakistan and water which appears clean is considered to be safe. The notion that water is free and infinite resource leads to no accountability on the part of its users. - In poverty stricken rural areas, open defecation is a cultural norm and accepted social behaviour. - Public sector expenditure on WASH totalled PKR 62 billion, against Pakistan's calculated annual investment needs of PKR 450 billion for the WASH sector.	As household incomes plummet during the lockdown – especially among households dependent on daily wages to make ends meet – access to WASH facilities and adequate WASH practices are likely to decrease.	- Provision of capacity development of decentralized tiers. Ensure that municipalities allocate funds for improved water supply and waste disposal infrastructure, the regular monitoring of water supplies, and ensuring that water supplies are contamination-free. - Ensure better management of faecal waste to avoid the contamination of water supplies, particularly recalling the dangers of potential faecal-oral transmission of COVID-19. - Ensure the continuity and quality of water and sanitation services – which will be highly affected by a reduced workforce, disrupted supply chains, and funding challenges – through close collaboration with national and local WASH authorities. - Further strengthen public awareness campaigns on the importance of WASH – particularly handwashing with soap and clean water – to fight COVID-19 and highlight how to conserve water while washing one's hands.

REFERENCES

Agha, Sohail, 'A profile of women at the highest risk of maternal death in Pakistan', *Health Policy and Planning*, vol. 30, no. 7, 2015, pp. 830–836, <<https://doi.org/10.1093/heapol/czu066>> <<https://academic.oup.com/heapol/article/30/7/830/825238>>, accessed 4 May 2020.

Asian Development Bank, *School Education in Pakistan: A Sector Assessment*, Mandaluyong, Metro Manila, ADB, 2019.

American Academy of Pediatrics, *Global Climate Change and Children's Health: Technical Report*. American Academy of Pediatrics, Itasca I.L., 2015.

Bari, Faisal, Rabea Malik, Pauline Rose and Nidhi Singal, *Identifying disability in household surveys: Evidence on education access and learning for children with disabilities in Pakistan*, Research and Policy Paper 18/1, University of Cambridge Faculty of Education, Research for Equitable Access and Learning (REAL) and Institute of Development and Economic Alternatives (IDEAS), Cambridge, 2018, <https://www.educ.cam.ac.uk/centres/real/downloads/REAL%20Policy%20Doc%20Disability%20Pakistan%20A4%2013pp_FINAL.pdf>, accessed 4 May 2020.

Black, Robert E., et al., 'Maternal and Child Undernutrition and Overweight in Low-and Middle-Income Countries: Prevalence and Consequences', *The Lancet Launch Symposium*, London, 6 June 2013.

Care International, *Gender Implications of COVID-19 Outbreaks in Development and Humanitarian Settings*, Care International, Geneva, 2020, <https://www.care-international.org/files/files/Gendered_Implications_of_COVID-19_Full_Paper.pdf>, accessed 4 May 2020.

Filmer, Deon, *Disability, Poverty and Schooling in Developing Countries: Results from Eleven Household Surveys*, Social Protection Discussion Paper Series, World Bank, Washington D.C., 2005, <<http://documents.worldbank.org/curated/en/619451468315852012/Disability-poverty-and-schooling-in-developing-countries-results-from-eleven-household-surveys>>, accessed 4 May 2020.

Germanwatch, *Global Climate Risk Index 2020*, Germanwatch, Bonn, 2019, <<https://germanwatch.org/en/17307>>, accessed 4 May 2020; Germanwatch, *Global Climate Risk Index 2019*, Germanwatch, Bonn, 2018.

Gilgit-Baltistan Planning & Development Department, and United Nations Children's Fund, *Gilgit-Baltistan Multiple Indicator Cluster Survey 2016-17*, Government of Gilgit-Baltistan, Gilgit, 2017.

Government of Pakistan, *Census 2017*, Government of Pakistan, Islamabad, 2017.

Government of Pakistan, *Pakistan's Implementation of the 2030 Agenda for Sustainable*

Development: Voluntary National Review 2019, Government of Pakistan, Islamabad, 2019.

Global Initiative to End All Corporal Punishment of Children, *Corporal punishment of children in Pakistan*, Global Initiative to End All Corporal Punishment of Children, London, March 2020, <<http://www.endcorporalpunishment.org/wp-content/uploads/country-reports/Pakistan.pdf>>, accessed 4 May 2020.

International Monetary Fund, *World Economic Outlook Database*, IMF, Washington D.C., April 2020.

Pakistan Institute of Development Economics, *COVID-19 Bulletin*, No. 13, Islamabad, PIDE, 2020, <<https://www.pide.org.pk/pdf/PIDE-COVID-Bulletin-13.pdf>>, accessed 4 May 2020.

High Level Panel of Experts on Food Security and Nutrition, *Impact of COVID-19 on Food Security and Nutrition*, Committee on World Food Security (CFS) and HLPE, Rome, 19 March 2020, <http://www.fao.org/fileadmin/templates/cfs/Docs1920/Chair/HLPE_English.pdf>, accessed 4 May 2020.

Jalil F, et al, 'Early child health in Lahore, Pakistan: IX. Perinatal events', *Acta Paediatric Supplement*, vol. 82, no. 390, 1993, pp. 95–107.

Kamran, Iram, Tahira Parveen, Maqsood Sadiq and Rehan M. Niazi, *Adolescent girls' voices on enhancing their own productivity in Pakistan*, Population Council, Islamabad, 2018, <https://knowledgecommons.popcouncil.org/departments_sbsr-pgy/794/>, accessed 4 May 2020.

Khyber Pakhtunkhwa Bureau of Statistics, Planning & Development Department, and United Nations Children's Fund, *Khyber Pakhtunkhwa Multiple Indicator Cluster Survey 2016-17*, Government of Khyber Pakhtunkhwa, Peshawar, 2017.

Kim, Jiyeon, Ajin Lee and Maya Rossin-Slater, *What to Expect When It Gets Hotter: The Impacts of Prenatal Exposure to Extreme Heat on Maternal and Infant Health*, IZA Institute of Economics, Bonn, 2019, <<https://www.iza.org/publications/dp/12685/what-to-expect-when-it-gets-hotter-the-impacts-of-prenatal-exposure-to-extreme-heat-on-maternal-and-infant-health>>, accessed 4 May 2020.

Lancet global Health 2020. *Early estimates of the indirect effects of the COVID-19 pandemic on maternal and child mortality in low-income and middle-income countries: a modelling study*, <[https://doi.org/10.1016/S2214-109X\(20\)30229-1](https://doi.org/10.1016/S2214-109X(20)30229-1)>, accessed 12 May 2020.

MAGENTA, COVID-19 Task force, *Impact of Confinement Study on Violence Against Children in Pakistan, Rapid Response Survey (Wave 1- June 2020)*. The survey was conducted for UNICEF.

Ministry of Finance, *Pakistan Economic Survey 2018–19*, Government of Pakistan, Islamabad, 2019.

Ministry of National Health Services, Regulation, and Coordination, *National Action Plan for Coronavirus disease (COVID-19) Pakistan*, Government of Pakistan, Islamabad, 2020.

Ministry of National Health Services, Regulations & Coordination, *National Vision 2016-2025 for Coordinated Priority Actions to Address Challenges of Reproductive, Maternal, Newborn, Child, Adolescent Health and Nutrition*, Government of Pakistan.

Ministry of National Health Services, Regulation, and Coordination, Performance Evaluation, *Lady Health Worker Programme Pakistan, September 2019*.

Ministry of Finance, *Pakistan Economic Survey 2018–19*, Government of Pakistan, Islamabad, 2019.

Ministry of National Health Services, Regulation, and Coordination, *National Action Plan for Coronavirus disease (COVID-19) Pakistan*, Government of Pakistan, Islamabad, 2020.

Mir, Fatima, et al., 'HIV infection predominantly affecting children in Sindh, Pakistan, 2019: a cross-sectional study of an outbreak', *The Lancet*, vol. 20, no. 3, 2020, pp. 362–370, <[https://www.thelancet.com/journals/laninf/article/PIIS1473-3099\(19\)30743-1/fulltext](https://www.thelancet.com/journals/laninf/article/PIIS1473-3099(19)30743-1/fulltext)>, accessed 4 May 2020, <<https://www.ncbi.nlm.nih.gov/pubmed/31866326>>, accessed 4 May 2020.

Murray, Christopher J.L., 'Global, regional, and national disability-adjusted life-years (DALYs) for 359 diseases and injuries and healthy life expectancy (HALE) for 195 countries and territories, 1990–2017: a systematic analysis for the Global Burden of Disease Study 2017', *The Lancet Europe PMC Funders Author Manuscripts*, vol. 392, no. 10159, 2018, pp. 1859–1922, <<https://www.ncbi.nlm.nih.gov/pmc/articles/PMC6252083>>, accessed 4 May 2020.

National Commission for Human Development, Ministry of Federal Education and Professional Training and PEHLA QADAM, *NCHD Annual Report 2017*, Government of Pakistan and PEHLA QADAM, Islamabad, 2017, <http://nchd.org.pk/ws/downloads/annual_reports/Annual_Report_2017.pdf>, accessed 4 May 2020.

National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2017–18*, NIPS, Islamabad, 2018.

National Institute of Population Studies, *Pakistan Demographic and Health Survey (PDHS) 2012–13*, NIPS, Islamabad, 2013.

National Education Management Information System, Academy of Education Planning and Management, Ministry of Federal Education and Professional Training, *Pakistan Education Statistics, 2016–17*, Government of Pakistan, Islamabad, 2018.

Nutrition Wing, Ministry of National Health Services, Regulation and Coordination, United Nations Children's Fund, Aga Khan University, and UK Aid, *National Nutrition Survey 2018*, Government of Pakistan, Islamabad, 2019.

Nutrition Wing, Ministry of Health, Aga Khan University, Pakistan Medical Research Council, and United Nations Children's Fund, *National Nutrition Survey 2011*, Government of Pakistan, Islamabad, 2012.

Pakistan Bureau of Statistics, *Labour Force Survey 2017–18*, Government of Pakistan, Islamabad, 2018.

Pakistan Bureau of Statistics, *Labour Force Survey 2014–15*, Government of Pakistan, Islamabad, 2015.

Pakistan, *Annual Status of Education Report: ASER-Pakistan 2019*, South Asian Forum for Education Development (SAFED), Lahore, 2019.

Punjab Bureau of Statistics and United Nations Children's Fund, *Multiple Indicator Cluster Survey 2017-2018, Punjab*, Government of Punjab, Lahore, 2018.

Punjab Bureau of Statistics and United Nations Children's Fund, *Multiple Indicator Cluster Survey 2014, Punjab*, Government of Punjab, Lahore, 2014.

Pakistan Bureau of Statistics, *Pakistan Social and Living Standards Measurement Survey 2014–15*, Government of Pakistan, Islamabad, 2016.

Pakistan Polio Eradication Programme, 'District Wise Polio Cases', National Emergency Operation Centre, Islamabad, 2019, <<https://www.endpolio.com.pk/polioin-pakistan/polio-cases-district-wise-2019>>, accessed 4 May 2020.

Planning Commission, Ministry of Planning, Development & Reform, *Summary for the National Economic Council (NEC): Sustainable Development Goals (SDGs) National Framework, March 2018*, Government of Pakistan, Islamabad, 2018.

Poverty Alleviation and Social Safety Division, *Ehsaas Financial Inclusion Policy*, Government of Pakistan, Islamabad, 2019, <<https://www.pass.gov.pk/userfiles1/files/Ehsaas%20Financial%20inclusion%20policy%20for%20printing.pdf>>, accessed 4 May 2020.

Population Council Pakistan, *Adding It Up: Investing in Contraception and Maternal and Newborn Health in Pakistan*.

Rafiq, Sonia, Wajeeha Syed and Simi Fayyaz Ghaffar, 'Trends and causes of maternal mortality in a tertiary care hospital over five years: 2013-2017', *Pakistan Journal of Medical Sciences*, vol. 35, no. 4, 2019, pp. 1128–1131, <<https://www.ncbi.nlm.nih.gov/pmc/articles/PMC6659062>>, accessed 4 May 2020.

Randell, Heather and Clark Gray, 'Climate change and educational attainment in the global tropics', *Proceedings of the National Academy of Sciences of the United States of America*, vol. 116, no. 18, 2019, pp. 8840–8845, <<https://www.ncbi.nlm.nih.gov/pmc/articles/PMC6500158>>, accessed 4 May 2020.

School Education Department, *Second Punjab Education Sector Programme (PESP II) (Draft) Inclusive Education (IE) Strategy (2019–2024)*, Government of the Punjab, Lahore, 2019.

Sleet, Phoebe, 'Food Security in Pakistan: Surplus Food is not Enough to Create a Food Secure

Country', Future Directions International, Nedlands, 9 April 2019, <<http://www.futuredirections.org.au/publication/food-security-in-pakistan-surplus-food-is-not-enough-to-create-a-food-secure-country>>, accessed 4 May 2020.

Sub-committee of the National Coordination Committee for COVID-19 on Economic Analysis, *Preliminary Macroeconomic and Socioeconomic Assessments, April 2020*, Government of Pakistan, Islamabad, 2020.

United Nations Children's Fund, *The State of the World's Children 2019: Children, Food, and Nutrition – Growing well in a changing world*, UNICEF, New York, 2019.

United Nations Children's Fund, *Mid-Term Evaluation of Project for Accelerating Policy Change, Translation and Implementation for Pneumonia and Diarrhoea Commodities 2016–2020: Pakistan*, UNICEF, Islamabad, 2016.

United Nations Children's Fund, *Consolidated Report for the Assessment of the Mass Media Campaign of the Polio Eradication Initiative (PEI)*, Islamabad, UNICEF, 2019.

United Nations Children's Fund, *Study on Review of HIV Treatment, PPTCT & Paediatric Services*, UNICEF, Islamabad, 2019.

United Nations Children's Fund, *Covid-19 – Pakistan Socioeconomic Impact Assessment & Response Plan*, UNICEF, Islamabad, April 2020.

United Nations Children's Fund, *The Impact of Climate Change on Children*, UNICEF, New York, 2015, <sustainabledevelopment.un.org/content/documents/2161unicef.pdf>, accessed 4 May 2020.

United Nations Children's Fund, *Status of Civil Registration and Vital Statistics in South Asia Countries 2018*, UNICEF, 2019.

United Nations Children's Fund and United Nations Population Fund, *Child Marriage in South Asia: An evidence review*, UNICEF, Kathmandu, 2019.

United Nations Children's Fund, United Nations Development Programme, United Nations Population Fund, Viamo and Accountability Lab Pakistan, *Understanding Youth Perceptions of COVID-19*, 2020.

United Nations, Policy Brief: *The Impact of COVID-19 on Children*, UN, New York, 15 April 2020, <https://www.un.org/sites/un2.un.org/files/policy_brief_on_covid_impact_on_children_16_april_2020.pdf>, accessed 4 May 2020.

United Nations Department of Economic and Social Affairs, *World Population Prospects 2019*

United Nations Development Programme, *Human Development Report 2016*, UNDP, New York, 2016; United Nations Development Programme, *Human Development Report 2019*, UNDP, New

York, 2019.

United Nations Development Programme, Oxford Poverty and Human Development Initiative and Ministry of Planning, Development & Reform, *Multidimensional Poverty in Pakistan*, UNDP, Islamabad, 2015.

United Nations Development Programme, *Multidimensional Vulnerability Index*, UNDP, Islamabad, 2020.

United Nations Development Programme, *FATA Governance Project*, UNDP, Islamabad, 2020, <<https://www.pk.undp.org/content/pakistan/en/home/projects/fata-governance-project.html>>, accessed 4 May 2020.

United Nations, *Shared Responsibility, Global Solidarity: Responding to the socio-economic impacts of COVID-19. Report of the Secretary-General*, United Nations New York, March 2019, <<https://unsdg.un.org/sites/default/files/2020-03/SG-Report-Socio-Economic-Impact-of-Covid19.pdf>>, accessed 4 May 2020.

United Nations, *A UN framework for the immediate socio-economic response to COVID-19*, UN, New York, April 2020, <<https://unsdg.un.org/resources/un-framework-immediate-socio-economic-response-covid-19>>, accessed 4 May 2020.

United Nations Office for the Coordination of Humanitarian Affairs and Government of Pakistan, *Pakistan: COVID -19 Situation Report, 02 May 2020*, Islamabad, OCHA and Government of Pakistan, 2020.

United Nations, *Sustainable Development Goals: Goal 2 Zero Hunger*, UN, New York, 2015

Verity, Robert, et al., 'Estimates of the severity of COVID-19 disease', Imperial College London, Queen Mary University of London and Oxford University, 13 March 2020, <<https://www.medrxiv.org/content/10.1101/2020.03.09.20033357v1.full.pdf>>, accessed 4 May 2020.

World Bank, *South Asia Economic Focus, Spring 2020: The Cursed Blessing of Public Banks*, World Bank, Washington D.C., 2020, <<https://openknowledge.worldbank.org/handle/10986/33478>>, accessed 4 May 2020.

World Bank, 'Pakistan Maternal Mortality Rate 1990-2020', World Bank, Washington D.C., <<https://www.macrotrends.net/countries/PAK/pakistan/maternal-mortality-rate>>, accessed 4 May 2020.

World Bank, *Tracing the Flow of Public Money, Punjab: Expenditure and Quantity of Service Delivery Survey in Primary School Sector*, World Bank, Washington D.C., 2015.

World Bank, *Pakistan Learning Poverty Brief: EduAnalytics*, World Bank, Washington D.C., 2019.

World Bank, *Pandemic Response Effectiveness in Pakistan (P173796)*, World Bank, Washington D.C., 2 April 2020

World Health Organization, 'HIV cases–Pakistan', WHO, Islamabad, 3 July 2019, <<https://www.who.int/csr/don/03-july-2019-hiv-cases-pakistan/en>>, accessed 4 May 2020.

World Health Organization, 'HIV cases–Pakistan', WHO, Islamabad, 3 July 2019, <<https://www.who.int/csr/don/03-july-2019-hiv-cases-pakistan/en>>, accessed 4 May 2020.

World Health Organization, Alliance for Health Policy and Systems Research, and Bill & Melinda Gates Foundation, Primary Care Systems Profiles & Performance (PRIMASYS): Pakistan Case Study, WHO and Alliance HPSR, Geneva, 2016, <<https://www.who.int/alliance-hpsr/projects/AHPSR-Pakistan-061016.pdf>>, accessed 4 May 2020.

Watkiss, Camila, 'Coronavirus: The immediate effect on climate change', ClimateAction, 30 March 2020, <www.climateaction.org/news/coronavirus-the-immediate-effect-on-climate-change>, accessed 4 May 2020.

Whieldon, Esther, 'COVID-19 highlights scale of action needed to tackle climate change', S&P Market Intelligence, New York, 30 April 2020, <<https://www.spglobal.com/marketintelligence/en/news-insights/latest-news-headlines/covid-19-highlights-scale-of-action-needed-to-tackle-climate-change-58333219>>, accessed 4 May 2020.

Xiao, Fei & Tang, Meiwen & Zheng, Xiaobin & Liu, Ye & Li, Xiaofeng & Shan, Hong. (2020). Evidence for Gastrointestinal Infection of SARS-CoV-2. *Gastroenterology*. 158. 10.1053/j.gastro.2020.02.055.

Young, William, et al., *Pakistan: Getting More from Water*, World Bank, Washington D.C., 2019, <<http://documents.worldbank.org/curated/en/251191548275645649/Pakistan-Getting-More-from-Water>>, accessed 4 May 2020.

Sustainable Development Goals (SDGs)



EVERY CHILD SURVIVES AND THRIVES

Goal 1: End poverty in all its forms everywhere

Goal 2: End hunger and all forms of malnutrition (2.1 and 2.2)

Goal 3: Ensure healthy lives and promote well-being for all at all ages. End preventable deaths of newborns and children under 5 years of age (3.1 and 3.2)

Goal 6: Universal and equitable access to safe and affordable drinking water, sanitation and hygiene for all (6.1 and 6.2)

Goal 16: End abuse, exploitation, trafficking and all forms of violence against and torture of children (16.1 and 16.2)

EVERY CHILD LEARNS

Goal 4: Ensure inclusive and equitable quality education and promote lifelong learning opportunities for all, including providing for education facilities that are child-, disability- and gender-sensitive with safe, non-violent, inclusive and effective learning environments for all (4.1, 4.2, 4.3, 4.4, 4.5 and 4a)

Goal 5: End all forms of discrimination against all women and girls everywhere (5.1)

Goal 6: Safely managed water and sanitation (6.2)

Goal 10: Reduce inequality and empower and promote the social, economic and political inclusion of all, irrespective of age, sex, disability, race, ethnicity, origin, religion, or economic or other status. Promote peace, justice and strong institutions (10.2 and 10.3)

EVERY CHILD PROTECTED FROM VIOLENCE AND EXPLOITATION

Goal 1: End poverty in all its forms everywhere and implement nationally appropriate social protection systems and measures for all, with substantial coverage of the poor and the vulnerable (1.3).

Goal 16: Promote peaceful and inclusive societies for sustainable development, provide legal identity for all, including birth registration (16.9)

Goal 16: Significantly reduce all forms of violence and related death rates everywhere, end abuse, exploitation, trafficking and all forms of violence against and torture of children, promote and enforce non-discriminatory laws and policies and promote the rule of law at the national and international levels, and ensure equal access to justice for all (16.1, 16.2, 16.3 and 16.b)

Goal 5: End all forms of discrimination against all women and girls everywhere, eliminate all forms of violence against all women and girls in the public and private spheres, including trafficking and sexual and other types of exploitation, and eliminate all harmful practices, such as early and forced marriage and female genital mutilation (5.1, 5.2 and 5.3)

Goal 8: Take immediate and effective measures to eradicate forced labour, end modern slavery and human trafficking, and secure the prohibition and elimination of the worst forms of child labour, including recruitment and use of child soldiers (8.7)

EVERY CHILD LIVES IN A SAFE AND CLEAN ENVIRONMENT

Goal 6: Ensure availability and sustainable management of water and sanitation for all. Achieve universal and equitable access to safe and affordable drinking water for all, and access to adequate and equitable sanitation and hygiene for all and end open defecation, paying special attention to the needs of women and girls and those in vulnerable situations (6.1 and 6.2)

Source: UN, Sustainable Development Goals, Knowledge Platform <<https://sustainabledevelopment.un.org/sdgs>>
Source: Pakistan's Implementation of the 2030 Agenda for Sustainable Development, Voluntary National Review 2019.

Pakistan National Baseline and 2030 Targets on SDGs

Pakistan National Baseline and 2030 Targets	
	1.2.1 – Reduce proportion of population living below the national poverty line from 29.5 percent (2013/14) to 9 percent 1.2.2 – Reduce multi-dimensional poverty from 38.8 percent (2014/15) to 19 percent 1.a.1 – Increase resources allocated by the government directly to poverty reduction programmes from 33 percent of Federal Consolidated budget (2014/15) to 45 percent 1.a.2 – Increase total government spending on essential services from 18.1 per cent of Federal Consolidated budget (2014/15) to 25.3 percent
	2.2.1 – Reduce stunting among children under 5 years of age from 44.8. percent (2012/13) to 10 percent (moderate) and 11.9 percent (severe)
	3.2.1 – Reduce under-five mortality rate (per 1,000 live births) from 89 (2012/13) to 40 3.2.2 – Reduce neonatal mortality rate (per 1,000 live births) from 55 (2012/13) to 25 3.7.1 – Increase proportion of women of reproductive age (15–49 years) who have need for family planning satisfied with modern methods from 47 percent (2012/13) to 70 percent 3.7.2 – Reduce adolescent birth rate (15–19 years) from 44 percent (2012/13) to 22 percent
	4.1.1 – Increase the minimum level of reading and mathematics proficiency of children and young people from 57 percent in 2014/15 to 100 percent 4.5.1 – Increase education parity indices from 0.88 for primary and 0.87 for secondary (2014/15) to 1.0.
	5.5.2 – Increase the proportion of women in managerial positions from 4.8 percent (2017/18) to 5 percent Increase the proportion of women in professionals from 38.1 percent (2012-13) to 45 percent
	6.1.1 – Increase the proportion of population using safely managed drinking water services from 77 percent (2013-14) to 85 percent 6.2.1 – Increase the proportion of population using (a) safely managed sanitation services from 73 percent (2014/15) to 80 percent
	7.1.1 – Increase access to electricity from 93.45 percent (2014/15) to 96 percent 7.1.2 – Increase primary reliance on clean fuels and technology from 41.34 percent (2014/15) to 60 percent 7.2.1 – Increase share of renewable energy total final energy consumption from 11 percent (2014/15) to 25 percent
	8.1.1 – Increase annual growth rate of real GDP per capita from 1.03 percent (2014/15) to 5 percent 8.2.1 – Increase annual growth rate of real GDP per employed person from 1.9 percent (2014/15) to 3.5 percent 8.3.1 – Reduce informal employment in non-agriculture employment from 73.6 percent (2014/15) to 65 percent 8.5.1 – Increase average hourly earnings of employees from PKR77.97 per hour (2014/15) - to PKR 233.91 per hour 8.5.2 – Reduce unemployment rate from 5.94 percent (2014/15) – to 3.5 percent
	9.2.1 – Increase manufacturing value added from 13.56 percent of GDP & 18,489 Per Capita (2015/16) to 16 percent of GDP & 96,000 Per Capita 9.2.2 – Increase manufacturing employment from 15.33 percent (2014/15) to 18 percent 9.3.1 – Increase small-scale industries in total industry value added from 8.4 percent (2014/15) to 12 percent 9.5.1 – Increase research and development expenditure as a proportion of GDP from 0.2 percent of GDP (2015/16) to 2 percent of GDP
	10.1.1 – Improve growth rates of household expenditure or income per capita from national average of 9.26 percent by 1.5 times, and of bottom 40 percent of population= 7.41 percent (2014/15) by 2.5 times 10.2.1 – Reduce proportion of people living below 50 per cent of median income, from 16.6 percent (2014/15) to 10 percent
	15.1.1 – Increase forest area as a proportion of total land area from 5 percent (2014-15) to 8 percent
	17.1.1 – Increase total government revenue from 14.5 percent (2014-12) to 18 percent 17.1.2 – Increase proportion of domestic budget funded by domestic taxes from 56 percent (2017/18) to 65percent 17.3.2 – Increase remittances as a proportion of total GDP from 7.1 percent (2012/13) to 10 percent

Government of PakistanSource: Pakistan's Implementation of the 2030 Agenda for Sustainable Development, Voluntary National Review 2019. Government of Pakistan

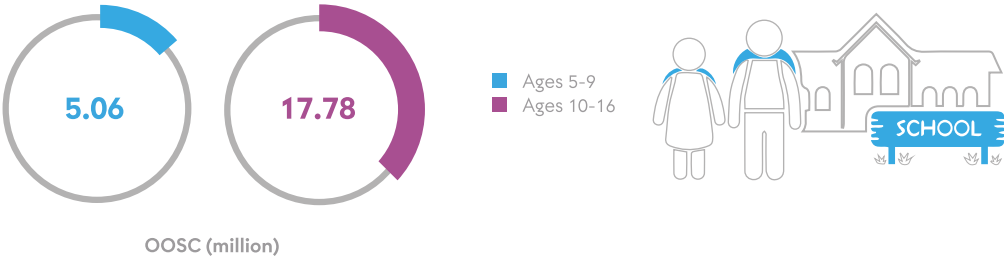
Key Indicators

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Indicators (%, unless stated otherwise)	Pakistan	Urban	Rural	Male	Female	Punjab	Sindh	KP	Balochistan	ICT	Ex-FATA	AJK	GB	Source
Children with Vitamin-A deficiency	51.5													
Children with Zinc deficiency	18.6													
Population Food Insecure	18.3					14.2	26.0	11.8	35.3	12.5	23.7	18.1	3.7	NNS 2018-19
Breastfeeding started within 1-hrs of birth	45.8					43.7	48.0	46.9	61.1	51.0	37.6	38.7	20.1	NNS 2018-19
Exclusive Breastfeeding under 6 months	48.4					44.3	52.3	60.8	43.9	57.6	59.0	42.1	54.9	NNS 2018-19
Under five Stunting (moderate and severe)	40.2					36.4	45.5	40.0	46.6	32.6	48.3	39.3	46.6	NNS 2018-19
Under five Stunting (severe)														NNS 2018-19
Under five Wasting (moderate and severe – minus two standard deviations)	17.7					15.3	23.3	15.0	18.9	12.2	23.2	16.0	9.4	NNS 2018-19
Under five Wasting (severe - minus three standard deviations)														NNS 2018-19
Under five Underweight (moderate and severe – minus two standard deviations)	28.9					23.5	41.3	23.1	31.0	19.2	33.7	21.9	21.3	NNS 2018-19
Under five Underweight (severe - minus three standard deviations)														NNS 2018-19
Nutritional status of adolescents (10-19) - Girls (underweight)	11.8					10.5	16.6	6.2	12.2	8.9	6.8	12.1	6.0	NNS 2018-19
Nutritional status of women (15-49) Underweight	14.4					12.1	22.6	8.3	14.5	10.7	5.4	12.9	10.1	NNS 2018-19

Indicators (%, unless stated otherwise)	Pakistan	Urban	Rural	Male	Female	Punjab	Sindh	KP	Balochistan	ICT	Ex-FATA	AJK	GB	Source
Adolescents girls with Anaemia	56.6													NNS 2018-19
Children with Vitamin-D deficiency	62.7													NNS 2018-19
Women age 15-49 with Anaemia	42.7													NNS 2018-19
Women age 15-49 with Iron deficiency	18.2													NNS 2018-19
Women age 15-49 with Vitamin-A deficiency	27.3													NNS 2018-19
Women age 15-49 with Zinc deficiency	22.1													NNS 2018-19
Women age 15-49 with Vitamin-D deficiency	79.7													NNS 2018-19
Health														
Safely managed drinking water	36.0													JMP 2019
Basic drinking water	92.0													JMP 2019
Basic managed sanitation	60.0													JMP 2019
Open defecation	10.0													JMP 2019
Drinking Water in Schools (primary)	72.7					99.3	54.7	89.0	50.9	97.4	38.7	21.4	45.7	PES 2016-17
Sanitation (Latrines) Facilities in Schools (primary)	72.7					99.0	60.9	95.7	19.0	96.9	34.7	27.4	39.2	PES 2016-17
WASH														

Number of OOSC in Pakistan by age cohort



Distribution of out-of-school children by Region

	OOSC (million)		OOSC (%)	
	2016	2017	2016	2017
Pakistan	22.6	22.9	44	44
Punjab	9.9	10.5	38	40
Sindh	6.7	6.4	55	52
KP	2.5	2.3	36	34
Balochistan	1.9	1.9	70	70
AJK	0.6	0.6	50	47
FATA	0.7	0.7	58	57
GB	0.2	0.2	50	47
ICT	0.04	0.04	12	12



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